

PHILIPPINE HEALTH INSURANCE CORPORATION

Dama ang Benepisyo Tungo sa Kalusugang Pangkalahatan



Vision

Bawat Filipino, Miyembro.
Bawat Miyembro, Protektado.
Kalusugan ng Lahat, Segurado.

Mission

Benepisyong Pangkalusugang
Sapat at De-kalidad para sa Lahat.

Core Values

Integridad, Inobasyon, Agarang Serbisyo,
Taos-pusong Paglilingkod, Pagmamalasakit,
Angkop na Benepisyo, Panlipunang Pagkakabuklod.

Dama ang Benepisyo Tungo sa Kalusugang Pangkalahatan

In 2013, the Philippine Health Insurance Corporation (PhilHealth) transitioned from a fee-for service model to a new payment mechanism known as All Case Rates (ACR), no significant increases or enhancements have been made to ACR since then.

The COVID-19 pandemic presented numerous challenges to improving health care services and enhancing access to health services, highlighting the need to revisit and rationalize existing benefit packages. In November 2023, the Corporation issued PhilHealth Circular (PC) No. 2023-0025, titled “Guiding Principles of the Rationalized Inpatient Case Rates” as a precursor and major steps in enhancing its benefit packages. This issuance essentially lays the framework for improving financial coverage in healthcare delivery as the Corporation transitions its provider payment mechanism to Diagnosis-Related Groups (DRGs).

As prescribed by the Universal Health Care (UHC) Law, PhilHealth strives to provide, a comprehensive, quality, and cost-effective set of promotive, preventive, curative, rehabilitative, and palliative health services without causing financial hardship.

PhilHealth’s initiatives to improve the benefit packages for conditions that may lead to catastrophic health spending are also guided by other existing laws (e.g. Senior Citizens Act, National Integrated Cancer Control Act, Mental Health Act, etc.). This improvement of the benefit packages is subject to periodic review.

Enhancements of existing benefits include rationalized packages for Pneumonia High Risk, Z Benefits for Colon and Rectal Cancer and Orthopedic Implants. Additionally, the development of new benefits such as Outpatient Mental Health Packages, KonSulTa with SDGs, and PhilHealth GAMOT have led to improvements in facilities and an increase in healthcare providers, as evidenced by the accomplishments detailed in this Annual Report.

With this, PhilHealth remains committed to providing adequate and quality health insurance for all.

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From the President of the Philippines

A Republic strengthens its foundations by ensuring the well-being of its people through institutions that remain faithful to their mandate, and deliver services with commitment, endurance, and purpose.

The Philippine Health Insurance Corporation (PhilHealth) was created to administer a national health insurance program that expands access to essential health services and provides financial protection for all Filipinos. Through this role, the PhilHealth manages our pooled public resources and promotes equitable healthcare services across sectors and communities.

The 2024 Annual Report stands as a strategic reflection of PhilHealth's evolution as a vital instrument of our national health financing architecture, documenting how actuarial discipline, administrative reform, and commitment to universal health care translate into tangible gains for the Filipino people. Indeed, this report articulates a cohesive vision for building a resilient healthcare system that fosters inclusive access and underpins our collective progress, inviting every stakeholder to invest in the future of a nation where health is the bedrock of our prosperity and where the dignity of every life is upheld.

The enduring service of the men and women of PhilHealth has my highest regard as they continue this vital work with responsiveness, excellence, and integrity. Let this annual report— and the milestones featured in it—renew your sense of purpose, strengthen your resolve, and sharpen your focus in the years ahead. In every claim honored, every system improved, and every life protected, you contribute to the promise of inclusive and compassionate governance in our Bagong Pilipinas. May your continued service remain a source of hope for the nation and a quiet triumph for every Filipino life made more secure.


Ferdinand R. Marcos, Jr.
President Republic of the Philippines





From the Office of the Secretary

Before we embark on another year in our pursuit of Universal Health Care in the Philippines, let us first reflect on the milestones, challenges, and successes of PhilHealth in 2024. This Annual Report highlights the significant steps taken in ensuring that every Filipino, in every corner of our archipelago, is protected from financial risk of availing needed health services. Our guiding principle remains clear: "Bawat Pilipino, Ramdam ang Kalusugan."

The Health Sector 8-Point Action Agenda guides us toward a stronger and more equitable health system. We have worked tirelessly to expand service and financial coverage, realizing that we should not only update existing packages but also develop new ones catered to the demands for both inpatient and outpatient services. These efforts emphasize our dedication in making healthcare a reality for every Filipino, regardless of their socioeconomic status.

For this year, PhilHealth implemented significant changes to its benefit packages to provide better financial support for healthcare expenses. Notable improvements include an initial 30% and subsequent 50% increase in most case rates, a 1300% increase in the breast cancer package, enhancements for neonatal sepsis and bronchial asthma, and rationalization of inpatient COVID-19 cases. The capitation rates for KonSulTa were increased to cover additional services, and the hemodialysis package was significantly enhanced.

PhilHealth also introduced new benefit packages for Severe Acute Malnutrition, Severe Dengue, Ischemic Heart Disease, and Peritoneal Dialysis. Payments for services in an outpatient setting were also approved, such as the Preventive Oral Health Services and Outpatient Emergency Care Benefit.

Looking ahead, PhilHealth remains committed to advancing these improvements. The goal is to continuously develop, collaborate, and enhance the benefits to support the country's vision of Universal Health Care. As the Chairperson of the PhilHealth Board of Directors, I assure all Filipinos that our mission to pay the health benefits of our members is unwavering, recognizing that PhilHealth's ultimate contribution to Universal Health Care is to pay the health benefits of its members—each and every Filipino.



Dahil sa Bagong Pilipinas, Bawat Buhay Mahalaga.


Teodoro J. Herbosa, MD
Secretary of Health



From the President and CEO

We remain focused on the future—we are committed to building on this momentum to further enhance our services and ensure the long-term sustainability of PhilHealth. Our goal is to continue to be a reliable partner in health for every Filipino.

PhilHealth made significant strides in 2024, focusing on tangible improvements in healthcare access for Filipinos. Expanding benefit packages, streamlining processes, and strengthening partnerships have enabled us to deliver on our promise of “Pinalawak at mga Bagong Benepisyo para sa Mamamayang Filipino.” These achievements translate to real and tangible benefits for our members, ensuring they receive adequate level of financial protection they deserve. I am particularly proud of the new benefits and enhancements we have introduced—exceeding our initial targets under our ambitious benefit development plan. This accomplishment reflects our commitment to listening to our members and responding to their evolving needs, ensuring PhilHealth remains a relevant and responsive partner in their healthcare journey.

Even as we celebrate our achievements, we remain mindful of the challenges ahead. Moving forward, PhilHealth will continue to develop more responsive benefits and extend our coverage, particularly to those in marginalized communities. I will never tire in saying that we remain committed to upholding our mission of providing all Filipinos with financial access to quality and affordable healthcare services.

I extend my sincerest gratitude to our dedicated employees, our valued partners, esteemed legislators, and most importantly, our members. Your trust and support are the driving force behind our efforts. Together, we are building a healthier future for the Philippines.

Mabuhay ang PhilHealth! Mabuhay ang Bagong Pilipinas!

Emmanuel R. Ledesma, Jr.
President and Chief Executive Officer



2024 Operational Highlights

Jun	Jul	Aug	Sep	Oct	Nov	Dec	

Dama ang Benepisyo Tungo sa Kalusugang Pangkalahatan

Health Care Financing

The share of health in total public spending has increased, with the Department of Health (DOH) including PhilHealth, accounting for a significant portion. In 2022, total health expenditure made up 5.5% of the Gross Domestic Product (GDP), while general government health spending accounted for 44.2 percent of current health expenditure (CHE). Even though the OOP share in CHE dropped to 41.9 percent in 2021 from 45.0 percent in 2020, its current share of 44.7 percent in 2022 suggests that this decline was offset by increased government funding for the COVID-19 pandemic response. Further, investments in primary care service delivery systems remain low in the Philippines, despite significant increases in overall health spending over the past decade. The majority of health expenditures go toward curative care and medical goods, while primary care—a focus of the UHC Act—constitutes a smaller share. Supply challenges, including limited access to Rural Health Units (RHUs), contribute to poor performance in primary healthcare.

National Health Insurance Program

The Philippine Health Insurance Corporation (PhilHealth) stands as a cornerstone of the nation's health sector, embodying a steadfast commitment to enhancing the well-being of every Filipino. Guided by its mandate under the National Health Insurance Act (NHIA), the Universal Health Care (UHC) Act, NEDA's Ambisyon 2040 vision and Philippine Development Plan, the President of the Philippines' 8-point socio-economic agenda and the Department of Health's (DOH) 8-point agenda, PhilHealth envisions a universal healthcare system that protects every Filipino particularly the poor and the underprivileged from the high cost of healthcare services.

Administering the National Health Insurance Program (NHIP), PhilHealth ensures that every Filipino has access to the full spectrum of essential, quality health services providing financial risk protection against the economic impacts of health emergencies. PhilHealth's programs have significantly reduced the financial burden on individuals, contributing to a healthier, more productive population—reflecting the organization's unwavering commitment to ensuring “Bawat Pilipino, Miyembro, Bawat Miyembro, Protektado, Kalusugan ng Lahat, Segurado”.

Universal Health Care

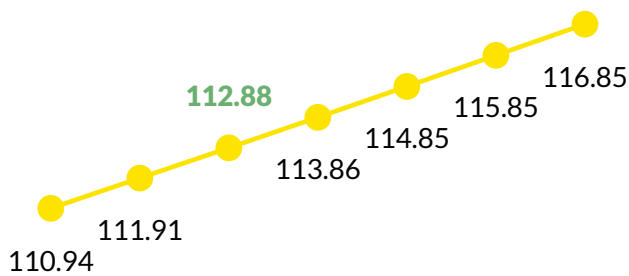
The realization of UHC is anchored on three dimensions, namely Population, Service, and Financial Coverage. As such, this report centers on updates on these three dimensions.

Population Coverage: Bawat Pilipino, Miyembro

With the signing of the UHC Act (Republic Act No.11223), all Filipinos are automatically included under the NHIP—making PhilHealth's coverage rate at 100%. Hence, the Philippine Statistics Authority's (PSA) projected population of 112.88M Filipinos in 2024 are all covered by PhilHealth.

Challenges still remain particularly in registering all Filipinos in the PhilHealth database. By the end of 2023, a total of 108.51M Filipinos are already registered by PhilHealth. This accounts for 97% of the 111.91M population projection by the PSA in 2023. Nonetheless, the said 111.91M Filipinos in 2023 which increased to 112.88M individuals in 2024 can avail of PhilHealth benefits at any point in time as needed.

PhilHealth-Covered Filipinos



2022 2023 2024 2025 2026 2027 2028
(in Millions; per PSA population projections)

With this, PhilHealth continues to roll-out various initiatives to enroll members and their qualified dependents as well as updating of records. These include partnership with other government agencies such as with the PSA on integration with the National ID and matching of Death records. PhilHealth also regularly conducts marketing activities in partnership with Local Government Units (LGUs) for the enrolment of their respective constituents. PhilHealth is also active in its social media platforms (e.g. Facebook, Twitter, Youtube, etc.) and other mainstream media (e.g. TV and Radio, etc.)

Service Coverage: Bawat Miyembro, Protektado

Every Filipino shall be granted immediate eligibility and access to preventive, promotive, curative, rehabilitative, and palliative care. It is therefore essential that accredited facilities are readily available and can provide such health care services as necessary.

Health Care Providers

In 2022, a total of 10,897 health facilities were accredited by PhilHealth. In 2023, this increased to 11,904. However, as of April 2024, accredited facilities decreased to 10,909. This is due primarily to facilities still undergoing renewal of their accreditation. The list of health care facilities accredited by PhilHealth include:

- Hospitals Infirmaries/Dispensaries
- Animal Bite Centers
- Ambulatory Surgical Clinics
- Drug-Abuse Treatment and Rehabilitation Centers
- Free-Standing Dialysis Clinics
- Mental Health Facility
- Family Planning Providers
- KonSulTa Package Providers

- Maternity Care Package Providers
- Outpatient HIV/AIDS Centers
- Outpatient Malaria Providers
- TB-DOTS Package Providers
- Covid-19 Facilities (Testing Laboratories)
- Community Isolation Units, Home Isolation)

As for Health Care Professionals, 50,951 accredited practitioners are ready to provide health care services as of April 2024. In 2022 and 2023 there were only 47,251 and 49,179 accredited professionals respectively. These include physicians, and midwives, among others.

Provider-Payment Mechanisms and Benefit Packages

PhilHealth's roadmap to improve its benefit packages is guided primarily by the UHC Act. Other existing laws also serve as bases for new benefits to be developed (e.g. Senior Citizen Act, National Integrated Cancer Control Act, Mental Health Act, etc.). Conditions that lead to catastrophic health spending are highly considered in the plan. With this, PhilHealth released enhancements in its benefit packages and improvements in its provider-payment mechanism (PPM) and patient classification system 2022 to 2024:

- 30% Increase for Case Rates - accounts for health inflation in the last decade.
- Global Budget (GB) - Funding mechanism that places a set amount of funding for a given time period. It is used to control the total expenditure on a particular product, service or institution, and motivate providers to improve service delivery through incentives that reward quality, efficiency and timely clinical practice.
- Bacterial Sepsis of Newborns (Enhancement) - from PHP11,700 to PHP25,793 (120.5% increase)
- Bronchial Asthma (in Acute Exacerbation; Enhancement)—from PHP9,000 to PHP22,488 (150% increase)
- KonSulTa Expansion - increased the capitation from PHP500 in government facilities and PHP750 in private facilities to PHP1,700 and covered additional diagnostic tests such as breast ultrasound and mammogram, among others.
- COVID-19 Inpatient Benefit Package (Enhancement)
- Outpatient Therapeutic Care Services for Severe Acute Malnutrition (SAM) for children 5 years and Below—package amounts to PHP7,500 to PHP17,000
- Supplemental Health Insurance Benefit Package (PhilHealth Plus)
- Preventive Oral Health Services - amounts to PHP200 to PHP1,500 (on a per tooth or visit basis)
- Severe Dengue - from PHP16,000 to PHP47,000 (194% increase)
- Ischemic Heart Disease with Acute Myocardial Infarction (Enhancement)
- Outpatient Emergency Care Benefit (OECB)
- Benefits: Breast Cancer - from PHP100,000 to PHP1.4 million (1300% increase)
- Z Benefits: Kidney Transplantation - from PHP600,000 to as much as PHP2,146,000 (up to 258% increase)
- Z Benefits: Peritoneal Dialysis—from PHP270,000 to as much as PHP1,269,000 (up to 370% increase)

Financial Coverage: Kalusugan ng Lahat, Segurodo

PhilHealth stands as a pivotal institution in the country's pursuit of accessible and affordable healthcare services. PhilHealth aims to shield Filipinos from the financial burden associated with seeking medical care. By offering insurance coverage, it ensures that individuals can access necessary healthcare services without facing exorbitant out-of-pocket expenses.

One of PhilHealth's notable programs is the No Copayment/No Balance-Billing (NBB) policy. Under this initiative, members experience zero copayments when admitted in basic accommodations in any PhilHealth-accredited health facility (HF). To advance this initiative, PhilHealth regularly conducts dialogues with health facilities. Further, PhilHealth has partnered with other government agencies, namely the DOH and the Department of Interior and Local Government (DILG), through initiatives such as the "Healthy Communities" and the Seal of Good Local Governance (SGLG) recognition programs in advancing the No Copayment/NBB.

With the implementation of the UHC Act, PhilHealth is transitioning to an improved provider payment mechanism in Global Budget and adopting a new payment classification system with DRGs. However, while these new systems are still under development, PhilHealth has already adjusted its case rates in January 2024. As aforementioned, PhilHealth introduced a thirty percent (30%) increase in the existing rates of its benefit packages - accounting for health inflation in the last decade.

In closing, PhilHealth stands as a beacon of unwavering commitment to the well-being of every Filipino. With a resolute focus on financial risk protection, PhilHealth ensures that no citizen is left vulnerable to the daunting costs of healthcare. By expanding coverage, implementing innovative policies and benefit packages, and advocating for equitable access, PhilHealth continues to pave the way toward a healthier and more secure future for all. Its tireless efforts resonate with the promise that no Filipino should bear the burden of medical expenses alone, reinforcing the collective strength of a nation that cares for its people — ultimately all for the realization of universal health care.



Corporate Governance Statement

“Damang-dama ang Benepisyo”

2024 Board Accomplishment Report

The year 2024 marks a significant chapter in PhilHealth's journey towards achieving Universal Health Care. Guided by its mandate, the Corporation reassures the nation its commitment to provide accessible, affordable, and quality health care for every Filipino. This year, we dedicated our efforts to expanding our roster of benefit packages and improving existing benefits thus making health care more inclusive and more responsive to the needs of our members.

We conducted a total of 16 meetings and passed 109 Resolutions aimed at guiding and supporting the PhilHealth Management, 30 of which were with particular focus on benefits. These new packages, alongside improvements in various programs, reflect PhilHealth's commitment to universal healthcare and its mission to provide stronger financial protection for every Filipino.

Expanding Access, Elevating Care

As a testament to our commitment of continuous improvement, PhilHealth adopted a 30% inflation adjustment factor for select case rates. Case Rates such as that for appendectomy was adjusted from Php 24,000 to Php 31,200, Cholecystectomy from Php 31,000 to Php 40,300, and Cesarean Delivery from Php 19,000 to Php 24,700 to name a few. We also enhanced several packages such as Z Benefits for Breast Carcinoma, Kidney Transplantation, Open-Heart Surgery, and Hemodialysis, Package rates for Bronchial Asthma and Newborn Sepsis, Case Rates for Severe Dengue, Cataract Extraction, and Case rates for specific heart diseases.

To further strengthen financial protection for members, we also expanded our coverage through the introduction of new benefit packages such as Outpatient Therapeutic Care Services for Severe Acute Malnutrition (SAM) for Children Five (5) Years and Below, Optometric Services for Children 0 to 15 years old, Emergency Care Benefit, Preventive Oral Health Services in Primary Care, Physical Medicine and Rehabilitation Services including the coverage of Assistive Mobility Devices, and the Z Benefits for the coverage of Rare Diseases or the Z MIRACLES packages. Through these new packages, we reaffirm our promise to provide benefits that are timely, relevant, and more attuned than ever to the well-being of every Filipino.

PhilHealth continued to break barriers in 2024 by implementing reforms that make health care more accessible and equitable. Among these is the repeal of the Pay Whichever is Lower (PWIL) policy, ensuring members receive the full benefit they deserve without unnecessary deductions. The enhancement of the KonSulTa package further strengthened primary care, bringing preventive and promotive services closer to communities. Additionally, the lifting of the single period of confinement policy provided greater flexibility for patients requiring multiple treatments. To keep pace with rising health care costs, PhilHealth applied an additional 50% increase on top of the 30% adjustment to select high-impact packages, reinforcing our commitment to responsive and patient-centered care.

Fiscal Stewardship and Accountability

Sustaining Benefits Through Sound Budgeting.

In 2025, PhilHealth will operate under a P284-billion Corporate Operating Budget (COB) approved by its Board, marking about a 10% increase from 2024. This budget is designed to sustain expanded benefits despite the absence of government premium subsidies for indirect contributors.

Of the total amount, P271 billion is allocated for benefit payments, while administrative expenses remain at around P12.5 billion and capital outlay at roughly P259 million—a clear indication of the agency's strong focus on member benefits and service delivery.

The COB also incorporates the Board's decision to increase select case rates by an additional 50% on top of previous inflationary adjustments and to fund priority benefit packages. These include KonSulTa+Sustainable Development Goals (SDG) capitation at P1,700–P2,100 per person, 156 hemodialysis sessions at P6,350 per session, and Coverage for emergency care, outpatient mental health, severe acute malnutrition, and other outpatient services among others. These measures aim to reduce out-of-pocket expenses nationwide and improve access to essential health services.

Strengthening Provider Accountability

In line with its mandate to uphold the highest standards of integrity and accountability, the PhilHealth Board acted decisively on various administrative cases involving healthcare providers, both individual professionals and institutional partners, found to have violated established rules and regulations. Through the timely adjudication of these cases, the Board reinforced its commitment to ensuring ethical practices within the healthcare delivery system. These actions not only protect the interests of members but also preserve the credibility of PhilHealth's programs by maintaining a network of providers that adhere to quality and compliance standards.

Taken together, the approval of the 2025 Corporate Operating Budget and the strengthened administrative enforcement advance PhilHealth's dual mandate: delivering robust financial risk protection for members while ensuring firm and fair accountability among providers. These measures ensure that every peso collected is converted into accessible, high-quality healthcare and is protected through a swift, rules-based adjudication process whenever standards are compromised.

Strengthening Foundations, Shaping the Future

PhilHealth looks to the future with renewed strength and a clear sense of purpose, guided by its core values of integrity, innovation, stewardship, social solidarity, and excellence. The reforms and initiatives introduced this year, including the expansion of benefits, the reinforcement of governance, and the strengthening of accountability, have established a strong foundation for sustainability and universal health coverage. Moving forward, PhilHealth remains committed to protecting every member, fostering strategic partnerships, and leveraging technology to deliver accessible, high-quality care. With these guiding principles, the organization envisions a future where every Filipino enjoys the security of a resilient and inclusive health system.

PhilHealth's Corporate Social Responsibility Statement

Beyond our primary purpose of providing responsive social health insurance coverage to all Filipinos, PhilHealth recognizes its bigger responsibility to be sensitive and ever responsive to the needs of all National Health Insurance Program (NHIP) beneficiaries, its own workforce, and society in general. This commitment extends beyond healthcare financing, encompassing initiatives that promote overall wellbeing, inclusivity, and sustainable development.

For all NHIP beneficiaries, PhilHealth upholds its responsibility of ensuring adequate financial access to economical yet effective and high-quality health care services. It continuously enhances its benefit packages, strengthens partnerships with healthcare providers, and improves its systems to deliver seamless and accessible healthcare services to all, especially the most vulnerable sectors.

For its workforce, PhilHealth remains steadfast in fostering a conducive and engaging work environment that promotes professional growth, employee well-being, and organizational excellence. It values collaboration and partnerships that empower its people, ensuring a motivated and dedicated workforce that effectively serves the Filipino people.

For society as a whole, PhilHealth acknowledges its broader role in promoting social responsibility and environmental sustainability. It actively supports initiatives that advocate for a clean, green, and safe environment, contributing to a progressive, sustainable, and healthier Philippine society. Through these concerted efforts, PhilHealth reaffirms its dedication to not only providing quality healthcare coverage but also making a lasting positive impact on the nation.

2024 Corporate Social Responsibility Report

We at PhilHealth prioritize engagements and cultivate long-term relationships to fulfill social responsibility through the performance of daily tasks, collaborative relationships, and community and environmental efforts.

In 2024, other than the improvement of benefit packages such as the institutionalization of 156 Hemodialysis Sessions, Outpatient Therapeutic Care for Severe Acute Malnutrition, Adjustments of Case Rates, Lifting of the Single Period of Confinement, ischemic heart disease-acute myocardial infarction (IHD-AMI) Benefit Package, Oral Health Services, Outpatient Emergency Care and the Z Benefits for Breast Cancer, Peritoneal Dialysis and Kidney Transplantation, PhilHealth remains true to taking responsibility of contributing towards health, welfare and environment sustainability.

Work Life Balance: Good Health and Well-being of PhilHealth Employees

It has been our desire to provide employees with an environment that would help develop their talent and be equipped in the performance of their jobs, have meaningful work experience through a sense of belongingness, and offer different measures to ensure the physical, emotional and mental health of the employees for them to thrive and perform at their best. Allowing our employees to take a break during their busy working day to create mental energy and well-being through medical services and the promotion of sports and corporate wellness will encourage and ensure high quality of work.

PhilHealthy Movement



Participation in GOCC Athletic Association, MMDA Cup, and UNTV Sports Events



Contribution to the community

PhilHealth believes that its involvement with the communities in which it operates should go beyond the economic development and job creation aspects inherent to its activity. Our commitment goes beyond this and, therefore, we collaborate in initiatives that generate a positive impact on our environment through our social action, volunteering, donations, sponsorship and patronage activities.

Donation of Winnings to Philippine Alliance of Patient Organizations (PAPO) from PhilHealth KonSulTa's participation to UNTV Cup Season 11 (Charity Sports)



Blood Donation



Medical Missions



Tree Planting and Seedling Production



We take social responsibility whenever relevant and where we can contribute the most. Not only do we invest in the health of our members, but we are also concerned about the needs of our community.



“A Well-equipped Workforce is the Cornerstone of Exceptional Service”

PhilHealth calls for the public's cooperation to curtail fraud and unethical acts done towards the National Health Insurance Fund. We encourage whistleblowers with any information pertaining to the Corporation, its officials and personnel to contact us through:

whistleblower@philhealth.gov.ph; actioncenter@philhealth.gov.ph; opceo@philhealth.gov.ph

or by mail addressed to the **Office of the Corporate Secretary Room 1711, 17th Floor Citystate Centre, 709 Shaw Boulevard, Pasig City.**

Complete confidentiality of reports as well as protection provided by law will be given to all whistleblowers.



Republic of the Philippines

COMMISSION ON AUDIT

Commonwealth Avenue, Quezon City, Philippines

INDEPENDENT AUDITOR'S REPORT

THE BOARD OF DIRECTORS

Philippine Health Insurance Corporation
Citystate Centre Building
709 Shaw Boulevard
Pasig City

Report on the Audit of the Financial Statements

Qualified Opinion

We were mandated to audit the Financial Statements (FSs) of the **Philippine Health Insurance Corporation (PhilHealth)**, which comprise the Statements of Financial Position as at December 31, 2024 and 2023, and the Statements of Comprehensive Income, Statements of Changes in Equity and Statements of Cash Flows for the years then ended, and Notes to the FSs, including a summary of significant accounting policies.

In our opinion, except for the matters discussed in the *Bases for Qualified Opinion* section of our report, the accompanying FSs present fairly, in all material aspects, the financial position of **PhilHealth** as at December 31, 2024 and 2023, and its financial performance and cash flows for the years ended, in accordance with Philippine Financial Reporting Standards (PFRS).

Bases for Qualified Opinion

The faithful representation in the FSs of Provision for Insurance Contract Liabilities (ICL) amounting to P1.407 trillion and P1.150 trillion for the Calendar Years (CYs) 2024 and 2023, respectively, could not be ascertained due to the following factors: a) the completeness and accuracy of the claims and premium data could not be established, thereby casting doubt on the reliability of the reported figures; b) uncertainties regarding the propriety of the valuation methodology adopted, coupled with the delayed submission of relevant supporting documents, further undermined the reliability and verifiability of the calculated Provision for ICL; and c) use of unauthorized amounts diminished the reliability and comparability of information in the FSs hindering the users from making informed economic decisions. These deficiencies are not in accordance with Philippine Accounting Standard (PAS) 1 — Presentation of FSs and Revised Conceptual Framework for Financial Reporting (CFFR).

Likewise, the completeness and accuracy of the Contributions and Premium Receivable, Members' Contributions, and Fines & Penalties — Business Income accounts amounting to P53.238 billion, P239.573 billion, and P176.477 million, respectively, could not be reasonably ascertained due to the incomplete and non-recognition of unremitted income and corresponding interests and surcharges of undetermined amounts from non-remitting employers and employers with payment gaps. This deprived PhilHealth of the funds necessary to carry out its mandate specifically in the administration of the National Health Insurance Program, and affected the faithful representation of account balances in the FSs, which are not in accordance with Paragraph 15 of PAS 1 — Presentation of FSs and Paragraphs 2.12 and 2.13 of the Revised CFFR.

Similarly, the faithful representation in the FSs of the Premium Contributions - Direct Contributor and Receivables from Direct Contributor, net accounts, with year-end balances of P199.223 billion and P44.104 billion, respectively, could not be ascertained due to the following deficiencies, contrary to the requirements of fair presentation outlined in PAS 1 and CFFR: a) the Statement of Premium Accounts transacted online for CY 2024, amounting to P162.099 billion, cannot be validated due to the unavailability of the repository database and restrictions in access to the Electronic Premium Remittance System, PhilHealth Members Portal, and Electronic Collection Reporting System. These limitations hinder the verification of amounts paid by members, restrict audit capabilities, and prevent the determination of the accuracy and existence of remitted collection by Accredited Collecting Agents (ACAs); b) skipped Electronic PhilHealth Agents Receipt series numbers imposed limitations on audit process, hindering the validation of the completeness of online collections from ACAs; c) the correctness of the Allowance for Impairment or the Expected Credit Loss computed for Receivables from Direct Contributor amounting to P9.135 billion was doubtful, as the actual outstanding receivables per member/employer have not been adequately established. Consequently, the Corporation's collection efficiency ratio, an essential factor in the computation of Loss Rate, could not be accurately determined given that the denominator (actual outstanding receivables) was not properly accounted for; and d) control deficiencies in the collection process further contributed to the uncertainty surrounding the correctness of the Premium Contributions - Direct Contributor and Receivable from Direct Contributor accounts, which highlight weaknesses in the internal control systems and procedures that are designed to safeguard the integrity of the collection process.

Also, the fair presentation in the FSs of the Members' Benefits, Accrued Benefit Claims - In Course of Settlement (ICS), and Accrued Benefits Payable (ABP) - Processed accounts, with year-end balances of P185.452 billion, P26.569 billion, and P4.271 billion, respectively, could not be established due to the improper recognition of related transactions in claims processing. The following related deficiencies highlight policy and procedural gaps that resulted in year-end balances inconsistent with Paragraph 15 of PAS 1, PAS 37 and, Paragraph 1.17 of the CFFR: a) in PhilHealth Regional Office (PRO) NCR and Rizal, the recognition of benefit expenses and their corresponding liabilities upon the generation of checks is not in accordance with the accrual basis of accounting, which requires transactions to be recognized when incurred, not when paid. This practice may result in the understatement of the Members' Benefits and ABP — Processed accounts by an undetermined amount, potentially leading to misinformed decision-making by stakeholders; and b) inconsistencies between the recognized amounts for ABP - Processed and the reconciled payable amounts, as indicated in the respective claims reconciliation reports, reinforced the Audit Team's reservations in PROs NCR and Rizal, II and VI on the reliability of the recorded amounts for the Members' Benefits and ABP Processed accounts.

Further, the faithful representation in the FSs of Provision for Health Benefits – Incurred But Not Yet Received (IBNR) account amounting to P70.902 billion for the CY 2024 could not be established due to several issues: a) the risk of misstatements in the related accounts, including ABP - ICS, ABP – ICS - Return to Hospital (RTH), and Provision for Health Benefits - Denied Claims, which may affect the accuracy of the Provision for Health Benefits – IBNR by an undetermined amount, and b) the non-reconciliation of the Provision for Health Benefits – IBNR with the actuarial valuation and the inconsistencies noted in the assumptions utilized in valuation methodology diminished the comparability and the reliability of information in the FSs, thus, hindering the users from making informed economic decisions. These deficiencies are not in accordance with the relevant provisions of PAS 1 - Presentation of FSs and Revised CFFR on faithful presentation of information in the FSs.

Moreover, the Premium Receivable from Direct Contributors, with a year-end balance of P44.104 billion, net, was misstated due to the non-recognition of adjustments with an unidentified amount arising from the collection of mandated one percent premium increment for CY 2022 from self-earning individuals, self-employed professional practitioners, and other direct contributors as prescribed under the Universal Health Care (UHC) Act which is inconsistent with the requirements set forth in PAS 1 and the Conceptual Framework for General Purpose Financial Reporting, emphasizing the importance of faithfully representing transactions in the appropriate accounting period.

Meanwhile, the faithful representation in the FSs of Financial Liabilities – ABP - ICS account amounting to P26.569 billion for the CY 2024 could not be ascertained due to: a) the different approaches by the Office of the Actuary and the Comptrollership Department in the computation of ABP-ICS, with respect to claims and the periods covered, resulted in a difference of P354.300 million, b) the presence of exceptions in 110,603 ICS claims totaling P1.351 billion and c) the use of inconsistent measurement bases for unencoded claims, where case rate values per membership category were not uniformly applied. These discrepancies and exceptions diminish the reliability and comparability of information in the FSs, which are inconsistent with PAS 1 and CFFR, thus, hindering the users from making informed economic decisions.

Lastly, the faithful representation in the FSs of Financial Liabilities - ABP - ICS - RTH account amounting to P1.978 billion for CY 2024 could not be established due to: a) the recognized amount of ABP – ICS – RTH was not sustained with the actuarial valuation; and b) the presence of 10,527 claims amounting to P272.664 million with deficiencies and exceptions, which are inconsistent with the PAS 1 and CFFR.

PhilHealth Management **has fully acknowledged and accepted** the audit observations, recognizing the critical nature of the issues identified. While PhilHealth Management has demonstrated a commitment to addressing these concerns, the necessary actions to rectify the audit findings have not yet been fully implemented. Consequently, the corrective measures recommended have not been reflected in the FSs and accompanying notes for the year ended December 31, 2024. This ongoing effort by Management, though positive, underscores the importance of continued diligence to ensure the full and timely resolution of these matters in future reporting periods.

Responsibilities of Management and Those Charged with Governance for the Financial Statements

Management is responsible for the preparation and fair presentation of these FSs in accordance with PFRSs, and for such internal control as management determines is necessary to enable the preparation of FSs that are free from material misstatement, whether due to fraud or error. In preparing the FSs, management is responsible for assessing the PhilHealth's ability to continue as a going concern, disclosing, as

applicable, matters related to going concern and using the going concern basis of accounting unless management either intends to liquidate the PhilHealth or to cease operations, or has no realistic alternative but to do so.

Those charged with governance are responsible for overseeing the PhilHealth's financial reporting process.

Auditor's Responsibilities for the Audit of the Financial Statements

Our responsibility is to conduct an audit of the PhilHealth's FSs in accordance with International Standards of Supreme Audit Institutions and to issue an auditor's report. Because of the matters described in the *Bases for Qualified Opinion* section of our report, we were able to obtain sufficient appropriate audit evidence to provide a basis for an audit opinion on these FSs.

We are independent of the PhilHealth in accordance with the Revised Code of Conduct and Ethical Standards for the Commission on Audit Officials and Employees (Code of Ethics) together with the ethical requirements that are relevant to our audit of the FSs in the Philippines, and we have fulfilled our other ethical responsibilities in accordance with these requirements and the Code of Ethics.

Report on Other Legal and Regulatory Requirements

Our audits were conducted for the purpose of forming an opinion on the basic FSs taken as a whole. The supplementary information for the year ended December 31, 2024 required by the Bureau of Internal Revenue as disclosed in Note 34 to the FSs is presented for purposes of additional analysis and is not a required part of the basic FSs prepared in accordance with PFRSs. Such supplementary information is the responsibility of management.

COMMISSION ON AUDIT



NORMA BELLA F. PORTILES

Supervising Auditor

Audit Group F - Philippine Health Insurance Corporation

Cluster 6-Social, Cultural, Trading, Promotional and Other Services

Corporate Government Audit Sector

June 10, 2025



Statement of Management's Responsibility for Financial Statements

The Management of the Philippine Health Insurance Corporation is responsible for the preparation of the financial statements as at December 31, 2024 and 2023, including the additional components attached thereto in accordance with the prescribed financial reporting framework indicated therein. The responsibility includes designing and implementing internal controls relevant to the preparation and fair presentation of financial statements that are free from material misstatement whether due to fraud or error, selecting and applying appropriate accounting policies and making accounting estimates that are reasonable in the circumstances.

The Board of Directors reviews and approves the financial statements before such statements are issued to the regulators, creditors and other users.

The Commission on Audit has audited the financial statements of the Philippine Health Insurance Corporation in accordance with the Philippine Financial Reporting Standards (PFRS) where applicable, as well as government accounting standards and other pertinent rules and regulations, and has expressed its opinion on the fairness of presentation upon completion of such audit, in its report to the Board of Directors.



TEODORO J. HERBOSA, MD,
FPCS, FACS

DOH Secretary
Chairperson of the Board



EDWIN M. MERCADO,
MD, MHA, MMSe

Acting President and
Chief Executive Officer



RENATO L. LIMSIACO, JR.,
CPA, DM, CESE

Senior Vice President

PHILIPPINE HEALTH INSURANCE CORPORATION

STATEMENTS OF FINANCIAL POSITION

As at December 31, 2024 and 2023
(In Philippine Peso)

	Notes	2024	2023 (As Restated)
ASSETS			
Current Assets			
Cash and cash equivalents	5	54,272,107,334	30,779,119,082
Investment in time deposits	6	51,941,636,979	135,606,097,538
Receivables, net	7	110,350,577,121	80,602,658,375
Inventories	8	172,358,282	125,716,250
Other current assets	9	386,853,645	2,722,550,287
Total Current Assets		217,123,533,361	249,836,141,532
Non-Current Assets			
Investment securities at amortized cost	10	391,653,929,474	336,127,635,854
Investment property	11	855,663,291	855,663,291
Property and equipment, net	12	1,146,632,147	827,659,343
Right-of-use of assets	13	1,315,324,748	432,170,366
Intangible assets, net	14	305,500,380	259,353,119
Other non-current assets, net	15	239,392,594	208,180,216
Total Non-Current Assets		395,516,442,634	338,710,662,189
TOTAL ASSETS		612,639,975,995	588,546,803,721
LIABILITIES			
Current Liabilities			
Financial liabilities	16	36,846,157,845	38,728,256,966
Inter-agency payables	17	30,685,475,009	762,080,485
Trust liabilities	18	1,088,751,895	1,056,381,446
Provision for health benefits	19	74,467,831,648	57,507,818,227
Other payables	20	1,424,355,776	2,458,148,334
Total Current Liabilities		144,512,572,173	100,512,685,458
Non-Current Liabilities			
Deferred credits/Unearned income	21	769,089,893	818,228,649
Lease payable	22	1,347,913,581	451,812,395
Leave benefits payable	23	843,818,884	814,468,334
Provision for insurance contract liabilities	24	1,406,915,873,309	1,149,656,439,006
Total Non-Current Liabilities		1,409,876,695,667	1,151,740,948,384
TOTAL LIABILITIES		1,554,389,267,840	1,252,253,633,842
EQUITY			
Members' equity	25	(941,749,291,845)	(663,706,830,121)
Total Equity		(941,749,291,845)	(663,706,830,121)
TOTAL LIABILITIES AND EQUITY		612,639,975,995	588,546,803,721

PHILIPPINE HEALTH INSURANCE CORPORATION

STATEMENTS OF COMPREHENSIVE INCOME

For the Years Ended December 31, 2024 and 2023
(In Philippine Peso)

	Notes	2024	2023
Premium contributions	26	239,573,300,339	236,158,650,315
Less: Benefit claims expenses	28	185,451,901,932	74,207,188,265
GROSS MARGIN FROM OPERATIONS		54,121,398,407	161,951,462,050
Operating Expenses			
Personnel services	29	5,462,651,693	5,348,565,974
Other operating expenses	30	10,436,856,327	4,054,742,038
TOTAL OPERATING EXPENSES		15,899,508,020	9,403,308,012
NET OPERATING INCOME (LOSS)		38,221,890,387	152,548,154,038
Add: Interest and other income	27	26,081,978,932	21,516,226,869
NET INCOME BEFORE CHANGE IN ICL		64,303,869,319	174,064,380,907
OTHER LOSSES	30.44	(257,259,434,303)	(882,783,126,475)
NET INCOME (LOSS)		(192,955,564,984)	(708,718,745,568)

PHILIPPINE HEALTH INSURANCE CORPORATION

STATEMENTS OF CHANGES IN EQUITY

For the Years Ended December 31, 2024 and 2023
(In Philippine Peso)

	Notes	2024	2023 (As Restated)
RETAINED EARNINGS	25.3		
Retained Earnings at January 1		–	–
Net income before change in ICL		64,303,869,319	174,064,380,907
Interest income from reserve fund (Excess actual reserves)		(13,347,414,223)	–
Prior year adjustment		4,797,008,943	14,437,516,296
Total Retained Earnings		55,753,464,039	188,501,897,203
Surplus of the two year projected expenditures		183,712,078,544	–
Remittance to BTr in compliance to DOF Cir. No. 003-2024 and Section XLIII of GAA)		(89,883,905,683)	–
Retained Earnings transferred to reserves			(188,501,897,203)
Retained Earnings at December 31		149,581,636,900	–
RESERVE FUND	25.1		
Reserve at January 1		464,286,992,149	275,785,094,946
Net income		(183,712,078,544)	–
Surplus transferred to reserves		–	188,501,897,203
Reserve Fund at December 31		280,574,913,605	464,286,992,149
INTEREST INCOME FROM RESERVE FUND			
(Excess actual reserves)	25.2	13,347,414,223	–
Provision for insurance contract liability	24/25.4	(1,385,253,256,573)	(1,127,993,822,270)
TOTAL MEMBERS' EQUITY		(941,749,291,845)	(663,706,830,121)

PHILIPPINE HEALTH INSURANCE CORPORATION

STATEMENTS OF CASH FLOWS

For the Years Ended December 31, 2024 and 2023
(In Philippine Peso)

	Note	2024	2023
CASH FLOWS FROM OPERATING ACTIVITIES			
Cash Inflows			
Cash received from premium contributions		204,097,678,836	197,809,239,822
Collection of other income		591,495,240	1,237,371,504
Gain (Loss) on foreign exchange		50,854	40,267
Collection of rent		–	600,000
Trust Receipts		31,723,854	44,393,232
Total Cash Inflows		204,720,948,784	199,091,644,825
Cash Outflows			
Unutilized Funds Pursuant to the GAA FY 2024 (RA 11975) and DOF Circular No. 003-2024		(60,000,000,000)	–
Payment of benefit claims		(165,007,990,000)	(118,900,340,656)
Payment of operating expenses		(9,478,132,508)	(8,099,271,234)
Payment of financial charges		(21,037,245)	(29,555,380)
Loss on Foreign Exchange		(30,947)	(44,764)
Trust Disbursements		(13,735,426)	(26,500,639)
Total Cash Outflows		(234,520,926,126)	(127,055,712,673)
Net Cash Provided by/(Used in) Operating Activities		(29,799,977,342)	72,035,932,152
CASH FLOWS FROM INVESTING ACTIVITIES			
Cash Inflows			
Matured time deposits		192,987,540,432	157,103,588,752
Matured treasury bills		–	9,350,077,535
Matured treasury bonds		28,974,300,000	41,679,228,000
Interest received from investments		26,842,133,903	18,713,680,940
Proceeds from disposal of assets		676,290	380,100
Total Cash Inflows		248,804,650,625	226,846,955,327
Cash Outflows			
Placement on time deposits		(109,323,022,012)	(166,283,301,101)
Placement on treasury bills		(28,284,279,260)	–
Placement on treasury bonds		(56,897,519,411)	(106,631,289,408)
Interest paid on placement of bonds		(215,819,205)	(1,011,889,408)
Fixed assets purchase		(558,953,715)	(314,822,914)
Total Cash Outflows		(195,279,593,603)	(274,241,302,831)
Net Cash Provided by/(Used in) Investing Activities		53,525,057,022	(47,394,347,504)
CASH FLOWS FROM FINANCING ACTIVITIES			
Cash Inflows		–	–
Total Cash Inflows		–	–
Cash Outflows			
Finance lease payments		(232,091,428)	(286,168,567)
Total Cash Outflows		(232,091,428)	(286,168,567)
Net Cash Provided By/(Used In) Financing Activities		(232,091,428)	(286,168,567)
INCREASE/(DECREASE) IN CASH AND CASH EQUIVALENTS		23,492,988,252	24,355,416,081
CASH AND CASH EQUIVALENTS, JANUARY 1	5	30,779,119,082	6,423,703,001
CASH AND CASH EQUIVALENTS, DECEMBER 31	5	54,272,107,334	30,779,119,082

PHILIPPINE HEALTH INSURANCE CORPORATION
Notes to the Financial Statements
(All amounts in Philippine Peso unless otherwise stated)

1. GENERAL INFORMATION

The National Health Insurance Act of 1995 or Republic Act (RA) No. 7875, and its amendments, otherwise known as the "National Health Insurance Act of 2013, instituted a National Health Insurance Program (NHIP) that shall provide health insurance coverage and ensure affordable, acceptable, available and accessible health care services for all Filipinos. The Philippine Health Insurance Corporation (PhilHealth), a tax-exempt Government-Owned and/or Controlled Corporation (GOCC), was established to administer the Program at the central and local levels. The Head Office is located at 709 CityState Centre Building, Barangay Oranbo, Shaw Blvd., Pasig City.

On February 20, 2019, RA No. 11223 or the Universal Health Care (UHC) Act was enacted. This law automatically provides social health risk protection for all Filipino citizens in the NHIP.

The Corporation is an attached agency of the Department of Health (DOH) for policy coordination and guidance, governed by a Board of Directors (BODs) composed of 13 members and has the powers and functions provided for in Article IV, Section 16 of RA No. 7875 as amended. The Corporation has the power, among others, to formulate and promulgate policies, set standards, rules and regulations necessary to ensure quality of care and overall accomplishment of program objectives, formulate and implement guidelines contributions and benefits; establish branch offices, manage grants and donations, acquire property, collect, deposit, invest, administer and disburse the National Health Insurance Fund (NHIF), to enter into contract with health care institutions, to fix a reasonable compensation, allowances and benefits of all positions including the President and Chief Executive Officer (PCEO), based on a comprehensive job analysis and audit of actual duties and responsibilities subject to the approval of the President of the Philippines.

The NHIF as amended shall consist of premiums from direct or indirect contributors; other appropriations earmarked by the national and local governments purposely for the implementation of the program; subsequent appropriations provided for under Sections 46 and 47 of RA No. 7875, as amended; donations and grants-in-aid; and all accruals thereof. Under Section 26, Article VI of RA No. 7875, as amended, the use, disposition, investment, administration and management of the NHIF, including any subsidy, grant or donation received for the program operations shall be governed by applicable laws, and in the absence thereof, existing resolution of the BODs of the Corporation subject to limitations prescribed in the Act.

The financial statements of the Corporation for the year ended December 31, 2024 were approved and authorized for issue by the BODs on March 26, 2025.

2. SUMMARY OF SIGNIFICANT ACCOUNTING POLICIES

2.1. Basis of Preparation and Statement of Compliance

Presently, PhilHealth is now classified as a Commercial Public Sector Entities (CPSE) per COA Resolution No. 2020-013 dated January 31, 2020, re: Renaming Government Business Entities (GBEs) and Non-Government Business Entities (non-GBEs) into CPSEs and Non-CPSEs pursuant to the 2018 Edition of the Handbook of International Public Sector Accounting Pronouncements (HIPSAP) published by the International Federation of Accountants (IFAC) and the International Public Sector Accounting Standard Board (IPSASB) and its adoption of the Philippine Financial Reporting Standards (PFRSs) as its financial reporting framework.

2.2. Measurement Bases

The financial statements are measured in Philippine Peso (P), which is also the Corporation's functional and presentation currency. All values are rounded-off to the nearest peso values, unless otherwise stated.

The financial statements have been prepared on a historical cost basis. Historical cost is generally based on the fair value of the consideration given in exchange of an asset and fair value of the consideration received in exchange for incurring a liability.

Fair value is the price that would be received to sell an asset or paid to transfer a liability in an orderly transaction between market participants at the measurement date.

The Corporation uses market observable data to a possible extent when measuring the fair value of an asset or a liability. Fair values are categorized into different levels in a fair value hierarchy based on inputs used in the valuation techniques as follows:

- Level 1 - Quoted (unadjusted) market prices in active markets for identical assets or liabilities.
- Level 2 - Valuation techniques for which the lowest level input that is significant to the fair value measurement is directly or indirectly observable.
- Level 3 - Valuation techniques for which the lowest level input that is significant to the fair value measurement is unobservable.

2.3. Adoption of New and Amended PFRS

The accounting policies adopted are consistent with those of the previous financial year, except for the adoption of the following new and amended PFRS which the Corporation adopted effective for annual periods beginning January 1, 2024.

Unless otherwise indicated, the adoption of the new and amended PFRS did not have any material effect on the financial statements. Additional disclosures have been included in the notes to financial statements, as applicable.

Amendments to PFRS 16, Lease Liability in a Sale and Leaseback - The amendment specify requirements for seller-lessees to measure the lease liability in a sale and leaseback transaction. The amendment does not change the accounting leases unrelated to sale and leaseback transactions.

Amendments to PAS 1, Non-current Liabilities with Covenants - The amendments clarify that only covenants with which an entity must comply on or before the reporting date will affect a liability's classification as current or non-current. In addition, the amendments require additional disclosures for non-current liabilities arising from loan arrangements that are subject to covenants to be complied with within 12 months after the reporting period.

2.4. New and Amended PFRS Issued But Not Yet Effective

Relevant new and amended PFRS, which are not yet effective as at January 31, 2024 and have not been applied in preparing the financial statements, are summarized below.

Effective for annual periods beginning on or after January 1, 2025:

PFRS 17, Insurance Contracts — This standard will replace PFRS 4, Insurance Contracts. It requires insurance liabilities to be measured at current fulfilment value and provides a more uniform measurement and presentation approach to achieve consistent, principle-based accounting for all insurance contracts. It also requires similar principles to be applied to reinsurance contracts held and investment contracts with discretionary participation features issued.

In response to the challenges brought by the Coronavirus disease (COVID-19) pandemic, the Insurance Commission issued Circular Letter 2020-062, Amendment of Section 1 of Circular Letter No. 2018-69, Deferral of IFRS 17 Implementation, which provides a two-year deferral on the implementation of the standard from the 2023 effectivity date. Therefore, all life and nonlife insurance companies in the Philippines shall adopt PFRS 17 for annual periods beginning on or after January 1, 2025.

Under prevailing circumstances, the adoption of the foregoing new and amended PFRS is not expected to have any material effect on the financial statements of the Corporation. Additional disclosures were included in the financial statements, as applicable.

2.5. Financial Assets and Liabilities

Date of Recognition

The Corporation recognizes a financial asset or a financial liability in the statements of financial position when it becomes a party to the contractual provisions of a financial instrument. In the case of a regular way purchase or sale of financial assets, recognition and derecognition, as applicable is done using settlement date accounting.

Initial Recognition

Financial instruments are recognized initially at fair value, which is the fair value of the consideration given (in case of an asset) or received (in case of a liability). The initial measurement of financial instruments, except for those designated at FVPL, includes transaction cost.

"Day 1" Difference

Where the transaction in a non-active market is different from the fair value of other observable current market transactions in the same instrument or based on a valuation technique whose variables include only data from observable market, the Corporation recognizes the difference between the transaction price and fair value (a "Day 1" difference) in profit or loss.

In cases where there is no observable data on inception, the Corporation deems the transactions price as the best estimate of fair value and recognizes "Day 1" difference in profit or loss when the inputs become observable or when the instrument is derecognized. For each transaction, the Corporation determines the appropriate method of recognizing the "Day 1" difference.

Classification of Financial Instruments

The Corporation classifies its financial assets at initial recognition under the following categories: (a) financial assets at FVPL, (b) financial assets at amortized cost and, (c) financial assets at Fair Value through Other Comprehensive Income (FVOCI). Financial liabilities, on the other hand, are classified as either financial liabilities at FVPL or other financial liabilities at amortized cost. The classification of a financial instrument largely depends on the Corporation's business model.

As at December 31, 2024 and December 31, 2023, the Corporation does not have financial assets and liabilities at FVPL and debt instruments measured at FVOCI.

Financial Assets at Amortized Cost

A financial asset shall be measured at amortized cost if both of the following conditions are met:

- The financial asset is held within a business model whose objective is to hold financial assets in order to collect contractual cash flows; and
- The contractual terms of the financial asset give rise on specified dates to cash flows that are solely payments of principal and interest on the principal amount outstanding.

After initial recognition, financial assets at amortized cost are subsequently measured at amortized cost using the effective interest method, less allowance for impairment, if any. Amortized cost is calculated by taking into account any discount or premium on acquisition and fees that are an integral part of the effective interest rate. Gains and losses are recognized in profit or loss when the financial assets are derecognized and through amortization process. Financial assets at amortized cost are included under current assets if realizability or collectability is within 12 months after the reporting period. Otherwise, these are classified as noncurrent assets.

Included in this category are Cash and Cash Equivalents, Receivables and Other Receivables.

Financial Liabilities at Amortized Cost

Financial liabilities are categorized as financial liabilities at amortized cost when the substance of the contractual arrangement results in the Corporation having an obligation either to deliver cash or another financial asset to the holder, or to settle the obligation other than by the exchange of a fixed amount of cash or another financial asset for a fixed number of its own equity instruments.

These financial liabilities are initially recognized at fair value less any directly attributable transaction costs. After initial recognition, these financial liabilities are subsequently measured at amortized cost using the effective interest method. Amortized cost is calculated by taking into account any discount or premium on the issue and fees that are an integral part of the effective interest rate. Gains and losses are recognized in profit or loss when the liabilities are derecognized or impaired or through the amortization process.

This category includes financial liabilities, inter-agency payables, trust liabilities, leave benefits payable and other payables.

2.6. Impairment of Financial Assets at Amortized Cost

The Corporation records an allowance for Expected Credit Loss (ECL). The ECL is based on the difference between the contractual cash flows due in accordance with the contract and all the cash flows that the Corporation expects to receive. The difference is then discounted at an approximation to the asset's original effective interest rate.

2.7. Derecognition of Financial Assets and Liabilities

Financial Assets

A financial asset (or, where applicable a part of a financial asset or part of a group of similar financial assets) is derecognized when:

- The right to receive cash flows from the asset has expired;
- The Corporation retains the right to receive cash flows from the asset, but has assumed an obligation to pay them in full without material delay to a third party under a "pass-through" arrangement; or
- The Corporation has transferred its right to receive cash flows from the asset and either: (a) has transferred substantially all the risks and rewards of the asset, or (b) has neither transferred nor retained substantially all the risks and rewards of the asset, but has transferred control of the asset.

When the Corporation has transferred its right to receive cash flows from an asset and has neither transferred nor retained substantially all the risks and rewards of the asset nor transferred control of the asset, the asset is recognized to the extent of the Corporation's continuing involvement in the asset. Continuing involvement that takes the form of a guarantee over the transferred asset is measured at the lower of the original carrying amount of the asset and the maximum amount of consideration that the Corporation could be required to repay.

Financial Liabilities

A financial liability is derecognized when the obligation under the liability is discharged, cancelled or expired. When an existing financial liability is replaced by another from the same lender on substantially different terms, or the terms of an existing liability are substantially modified, such an exchange or modification is treated as a derecognition of the original liability and the recognition of a new liability. The difference in the respective carrying amounts is recognized in profit or loss.

2.8. Offsetting of Financial Assets and Liabilities

Financial assets and liabilities are offset and the net amount is reported in the statements of financial position if, and only if, there is a currently enforceable legal right to offset the recognized amounts and there is an intention to settle on a net basis, or to realize the assets and settle the liabilities simultaneously. This is not generally the case with master netting agreements, and the related assets and liabilities are presented gross in the statements of financial position.

2.9. Classification of Financial Instrument between Liability and Equity

A financial instrument is classified as liability if it provides for a contractual obligation to:

- Deliver cash or another financial asset to another entity;
- Exchange financial assets or financial liabilities with another entity under conditions that are potentially unfavorable to the Corporation; or
- Satisfy the obligation other than by the exchange of a fixed amount of cash or another financial asset for a fixed number of own equity shares.

If the Corporation does not have an unconditional right to avoid delivering cash or another financial asset to settle its contractual obligation, the obligation meets the definition of a financial liability.

2.10. Inventories

Inventories include office supplies and materials inventory, semi-expendable machinery and equipment, and semi-expendable furniture, fixtures and books. These are initially measured at cost.

At each reporting date, inventories are assessed for impairment. If an item of inventory is impaired, its carrying value is reduced to net realizable value, and an impairment loss is recognized immediately in profit and loss. Any reversal of impairment is recognized also in profit and loss.

2.11. Other Current Assets

Other current assets represent assets of the Corporation which are expected to be realized within one year or within the Corporation's normal operating cycle whichever is longer. Other current assets are presented in the Statement of Financial Position at net realizable value.

2.12. Property and Equipment

Property and equipment are tangible assets that are held for use in the production or supply of goods or services, for rental to others, or for administrative purposes, and are expected to be used during more than one period.

Items of property and equipment are initially measured at cost. Such cost includes purchase price and all incidental costs necessary to bring the asset to its location and condition. Subsequent to initial recognition, items of property and equipment are measured in the statement of financial position at cost less any accumulated depreciation and any accumulated impairment losses. Depreciation, which is computed on a straight-line basis, is recognized so as to allocate the cost of assets less their residual values over their estimated useful lives.

The depreciation periods for property and equipment, based on above policies, are as follows:

Particulars	Estimated Useful Life (in years)
Land improvements	10
Building and building improvements	30
Leasehold improvements	3 to 5 or the remaining of the contract duration whichever is shorter
IT equipment	5
Furniture and fixtures	10
Office equipment	5
Communication equipment	10
Library books	5
Medical equipment	10
Transportation equipment	7

The estimated useful lives and depreciation method are reviewed periodically to ensure that the periods and method of depreciation is consistent with the expected pattern of economic benefits from items of property and equipment.

Land is not depreciated. If there is an indication that there has been a significant change in useful life or residual value of an asset, the depreciation of that asset is revised prospectively to reflect the new expectations.

When assets are sold, retired or otherwise disposed of, their costs and related accumulated depreciation and impairment losses, if any, are removed from the accounts and any resulting gain or loss is reflected in profit or loss for the period.

2.13. Intangible Assets

Intangible asset represents computer software and licenses. This is initially measured at cost and is presented in the Statement of Financial Position at cost less accumulated amortization and any accumulated impairment losses.

Computer software with finite life is amortized over its estimated useful life (2 to 10) years using the straight-line method while the licenses are amortized over the life of the license. The amortization period and method are reviewed periodically to ensure that these are consistent with expected pattern of economic benefits from the intangible assets. If there is an indication that there has been a significant change in the useful life or residual value of an intangible asset, the amortization is revised prospectively to reflect the new expectations.

Intangibles that are classified with indefinite life are not amortized but are subject to impairment test annually. Part of the intangibles internally developed software either with definite or indefinite life that are being used in the operations.

When assets are sold, retired or otherwise disposed of, their cost and related accumulated amortization and impairment losses, if any, are removed from the accounts and any resulting gain or loss is reflected in profit or loss for the period.

2.14. Impairment of Non-Financial Assets

At each reporting date, property and equipment and intangible asset accounts are reviewed to determine whether there is any indication that those assets have suffered an impairment loss. If there is an indication of possible impairment, the recoverable amount of any affected asset (or group of related assets) is estimated and compared with the carrying amount. If the estimated recoverable amount is lower, the carrying amount is reduced to its estimated recoverable amount, and an impairment loss is recognized immediately in profit and loss.

If an impairment loss subsequently reverses, the carrying amount of the asset is increased to the revised estimate of its recoverable amount, but not in excess of the amount that would have been determined had no impairment loss been recognized for the asset in prior years. A reversal of an impairment loss is recognized immediately in profit or loss.

2.15. Reserve Fund

Reserve Fund represents portion of the accumulated revenues not needed to meet the cost of the current year's expenditures, subject to the reserve ceiling.

It is recorded in compliance with Office Order No. 0145, s. of 2012 and based on the provisions of Section 11 of RA No. 11223 or the Universal Health Care Act, which states that PhilHealth shall set aside a portion of its accumulated revenues not needed to meet the cost of the current year's expenditures as reserve funds: *Provided*, that the total amount of reserves shall not exceed a ceiling equivalent to the amount actuarially estimated for two (2) years' projected Program expenditures. Provided further; that whenever actual reserves exceed the required ceiling at the end of the Corporation's Fiscal Year, the excess of the Corporation's Reserve Fund shall be used to increase the program's benefits and to decrease the amount of member's contributions.

Any unused portion of the reserve fund that is not needed to meet the current expenditure obligations or support the abovementioned programs shall be placed in investments to earn an average annual income at prevailing rates of interest and shall be referred to as the Investment Reserve Fund.

The computation of the Reserve is not equal to the Insurance Contract Liabilities (ICL). The reserve is a two year projected expenditures while the ICL is the estimate of the expected future claims payments to be incurred less expected premium contributions in relation to the liabilities that the Corporation is contractually obligated to pay to our members. It is computed as the expected net of present value of expenses (claims + expenses) and present value of collection plus margin for adverse deviation.

2.16. Retained Earnings

This account represents the cumulative results of normal and continuous operations of a Commercial Public Sector Entity including prior period, effects of changes in accounting policy and error other capital adjustments. This may include funds set aside for various purposes in accordance with existing laws, rules and regulations.

2.17. Revenue Recognition

Revenue is recognized to the extent that is probable that the economic benefits will flow to the Corporation and the revenue can be reliably measured. Revenue is measured at the fair value of the consideration received, excluding discounts and rebates. Revenue is recognized either at a point in time or over a period of time.

Premium contributions

Revenue is recognized as the members' contributions become due. Interest, fines and penalties are recognized when the event that triggers the interest, fine or penalty occurs. If the collectability is not reasonably assured, fines and penalties are recognized only when cash is received.

Interest Income

Interest income is recognized as the interest accrues taking into account the effective interest.

Rent Income

Income from rental of property is recognized on a straight-line basis over the lease term.

2.18. Expense Recognition

Expenses are decreases in economic benefits during the accounting period in the form of outflows or depletions of assets or incurrence of liabilities that result in decrease in equity, other than those relating to distributions of equity to participants.

Benefit Claims Expense

This represents expenses incurred by the Corporation for health care services, in-patient, out-patient, PCB and Z benefit packages availed of by the members and their dependents. Benefit Claims Expenses are generally recognized at the date of admission (per Corporate Order 2021-0051).

Operating Expenses

These include personnel services and maintenance and other operating expenses which are recognized as expense in the period they are incurred.

2.19. Leases

A lease is a contract that conveys the right to use an identified asset for a period of time in exchange for a consideration.

Determination as to whether a contract is, or contains, a lease is made at the inception of the lease. Accordingly, the Corporation assesses whether the contract meets three key evaluations which are:

- The contract contains an identified asset, which is either explicitly identified in the contract or implicitly specified by being identified at the time the asset is made available to the lessee;
- The lessee has the right to obtain substantially all of the economic benefits from use of the identified asset throughout the period of use, considering its rights within the defined scope of the contract; and,
- The lessee has the right to direct the use of the identified asset throughout the period of use. The lessee assesses whether it has the right to direct 'how and for what purpose' the asset is used throughout the period of use.

Corporation as a Lessee

Leases are recognized as a Right-of-Use (ROU) asset and a corresponding liability at the date at which the leased asset is available for use by the Corporation.

The Corporation's leasing activity is mainly for office spaces and warehouses. The non- cancellable contracts period for leases is 3 to 5

years and has no extension clause. Upon expiration of the lease contract, it is extended on a month-to-month basis only until a new contract is executed or a new office space will be leased.

ROU Assets

At commencement date, the Corporation measures ROU assets at cost. The cost comprises:

- i. The amount of the initial measurement of lease liabilities;
- ii. Any lease payments made at or before the commencement date less any lease incentives received;
- iii. Any initial direct costs; and
- iv. An estimation of costs to be incurred by the Corporation in dismantling and removing the underlying asset, when applicable.

ROU assets are recognized at the present value of the liability at the commencement date of the lease, adding any directly attributable costs. After the commencement date, ROU assets are carried at cost less any accumulated amortization and accumulated impairment losses, and adjusted for any re-measurement of the related lease liabilities. ROU assets are amortized over the shorter of the lease terms including renewals or the useful lives of the underlying assets. In addition, ROU asset is periodically reduced by impairment losses, if any and adjusted for certain measurement of the lease liability.

ROU assets are office spaces and warehouses being leased by PhilHealth with a multi- year contract.

Below is the summary of information regarding ROU asset of the Corporation:

	2024	2023
Carrying amount of Right-of-Use asset	1,315,324,748	432,170,366
Depreciation expense	307,915,015	272,458,495
Interest expense	36,839,046	19,037,558
Short-term leases	407,261,860	325,222,188
Total	2,067,340,669	1,048,888,607

Lease Liabilities

At commencement date, the Corporation measures a lease liability at the present value of future lease payments using the interest rate implicit in the lease, if that rate can be readily determined. Otherwise, the Corporation uses its incremental borrowing rate.

Lease payments included in the measurement of a lease liability comprise the following:

- i. Fixed payments, including in-substance fixed payments;
- ii. Variable lease payments that depend on an index or a rate, initially measured using the index or rate as at the commencement date;
- iii. Amounts expected to be payable by the lessee under residual value guarantees; and
- iv. The exercise price under a purchase option that the Company is reasonably certain to exercise; lease payments in an optional renewal period if the Company is reasonably certain to exercise an extension option; and penalties for early termination of a lease unless the Company is reasonably certain not to terminate early.

A lease liability is subsequently measured at amortized cost. Interest on the lease liability and any variable lease payments not included in the measurement of lease liability are recognized in profit or loss unless these are capitalized as costs of another asset. Variable lease payments not included in the measurement of the lease liability are recognized in profit or loss when the event or condition that triggers those payments occurs.

If there is a change in the lease term or if there is a change in the assessment of an option

to purchase the underlying asset, the lease liability is re-measured using a revised discount rate considering the revised lease payments on the basis of the revised lease term or reflecting the change in amounts payable under the purchase option.

Short-Term Leases and Leases of Low-Value Assets

Short-term leases and lease of low value assets are exempted from the recognition of ROU asset and lease liability as allowed under PFRS 16. These are recognized in the profit or loss when incurred.

The Corporation does not have leases of low-value assets. Classified under short-term leases are office spaces that are on a month to month basis renewal and have no multi- year contract.

Corporation as a Lessor

Leases where the Corporation does not transfer substantially all the risks and benefits of ownership of the assets are classified as operating leases. Initial direct costs incurred in negotiating operating leases are added to the carrying amount of the leased asset and amortized over the lease term on the same basis as the rental income. Contingent rents are recognized as revenue in the period in which these are earned. Operating leases are recognized as income in the statements of comprehensive income on a straight-line basis over the lease term.

The Corporation determines whether an arrangement is, or contains a lease based on the substance of the arrangement. It makes an assessment of whether the fulfillment of the arrangement is dependent on the use of a specific asset or assets and the arrangement conveys a right to use the asset.

2.20. Employee Benefits

Short-term Benefits

Short-term benefits given by the Corporation to its employees include salaries and wages, compensated absences, 13th month pay, Mid-year bonus, employer share contributions and other de minimis benefits, among others. These are granted pursuant to E.O. 150, or the Compensation and Position Classification System for Government Corporations.

These are recognized as expense in the period the employees render services to the Corporation.

2.21. Related Parties

Related party relationship exists when one party has the ability to control, directly, or indirectly through one or more intermediaries, the other party or exercises significant influence over the other party in making financial and operating decisions. Such relationships also exist between and/or among entities which are under common control with the reporting enterprise, or between, and/or among the reporting enterprise and its key management personnel, directors, or its major shareholders. In considering each possible related party relationship, attention is directed to the substance of the relationship, and not merely the legal form.

2.22. Provisions

Provisions are recognized when the Corporation has a present obligation (legal or constructive) as a result of a past event, it is probable that an outflow of resources embodying economic benefits will be required to settle the obligation, and a reliable estimate can be made of the amount of the obligation.

Provisions are made using the best estimates of the amount required to settle the obligation and discounted to present values using a pre-tax rate that reflects current market assessments of the time value of money and the risks specific to the obligation. Changes in estimates are reflected in profit or loss in the period they arise.

2.23. Provisions for Insurance Contract Liabilities (ICL)

Provision for ICL is the estimate of the expected future claims payments to be incurred less expected premium contributions in relation to the liabilities that the Corporation is contractually obligated to pay to our members. It is computed as the expected net of present value of expenses (claims + expenses) and present value of collection plus

margin for adverse deviation. Since 2005, PFRS 4 have mandated that these liabilities must be properly recognized and reported in the financial statements.

Adequacy test is also performed to assess at the end of each reporting period whether the recognized insurance contract liabilities are adequate, using current estimates of future cash flows under its insurance contract. If the test shows that the liability is inadequate, the entire deficiency is recognized in profit or loss.

The ICL is a different computation and not equal to the reserve fund which is a two years' projected program expenditure to be set aside in accordance with R.A. 7875 as amended by RA Nos. 9241, 10606 and 11223.

2.24. Contingencies

Contingent liabilities are not recognized in the financial statements. These are disclosed in the notes to financial statements unless the possibility of an outflow of resources embodying economic benefits is remote. Contingent assets are not recognized in the financial statements but are disclosed when an inflow of economic benefits is probable.

2.25. Events after the Reporting Period

Post year-end events that provide additional information about the Corporation's financial position at reporting date (adjusting events) are reflected in the financial statements. Post year-end events that are not adjusting events are disclosed in the notes to financial statements when material.

3. SIGNIFICANT JUDGMENTS, ACCOUNTING ESTIMATES AND ASSUMPTIONS

The preparation of the financial statements in accordance with PFRS requires Corporation to exercise judgment, make accounting estimates and assumptions that affect the reported amounts in the financial statements and related notes. The judgment and estimates used in the financial statements are based upon Corporation's evaluation of relevant facts and circumstances as of the date of the financial statements. Actual results could differ from such estimates.

Judgment and estimates are continually evaluated and are based on historical experiences and other factors, including expectations of future events that are believed to be reasonable under the circumstances.

3.1. Judgments

In the process of applying the Company's accounting policies, management has made certain judgment, apart from those involving estimations, which have the most significant effect on the amounts recognized in the financial statements.

3.2. Estimates and Assumptions

The key assumptions concerning the future and other key source of estimation uncertainty at the financial reporting date, that have significant risk of causing a material adjustment to the carrying amounts of assets and liabilities within the next financial year are discussed below.

Estimating Premium Income from the Direct Contributors - Employed Sector

The estimate is based on SPA generated by employers. The amount estimated shall be based on the applicable period as at the reporting period. The generated SPA shall come from the extraction of databases and shall cover all employers registered all over the country.

Estimating Allowance for Impairment of Receivables

The Corporation maintains allowance for impairment losses at a level considered adequate to provide for potential uncollectible receivables. This amount is evaluated based on such factors that affect the collectability of the accounts. The amount of the allowance is computed by multiplying the exposure at default by Loss rate (100% less collection efficiency rate). An increase in allowance for impairment losses would increase the recorded operating expenses and decrease current assets.

Estimating Useful Lives of Property and Equipment

The Corporation estimates the useful lives of the property and equipment based on the period over which these assets are expected to

be available for use. The estimated useful lives are reviewed periodically and are updated if expectations differ from previous estimates due to physical wear and tear, technical or commercial obsolescence and legal or other limits on the use of these assets. In addition, estimation of the useful lives is based on collective assessment of industry practice, internal technical evaluation and experience with similar assets. It is possible, however, that future results of operations could be materially affected by changes in estimates brought about by changes in factors mentioned above. The amounts and timing of recorded expenses for any period would be affected by changes in these factors and circumstances. There is no change in the estimated useful lives of depreciable property and equipment in 2024 and 2023,

Estimating Benefit Claims Payables

One of the accounting estimates being made is the setting-up of accrued benefit expense for Incurred But Not Yet Paid (IBNP) claims which consist of the following:

- a. In-Course of Settlement (ICS) - these are claims received and still in process at the end of the reporting period. It includes claims already approved for payment awaiting ADA and RTH of the current year. The estimate is based on the case rate amount extracted from the N-Claims.
- b. Provision for Denied Claims - Denied claims are claims determined to be invalid and unworthy of payment due to an absolute deficiency that cannot be remedied through return to Health Facility or due to a finding of an unmet requirement. However, a provision is necessary for those claims with pending motion for reconsiderations which are highly probable that payment shall be made after evaluation.
- c. Incurred But Not Yet Reported (IBNR) - These are claims which are estimated to be in the possession of the Health Facilities (HFs) as of the end of the year and have yet to be submitted to the Corporation. The IBNR is the balance after deducting the ICS and provision for denied claims from Incurred But Not yet Paid (IBNP) when IBNP are those claims yet to be paid by PhilHealth. The amount to be recorded is actuarially estimated. It is computed as follows:

$$\text{IBNR} = \text{IBNP} - \text{ICS} - \text{Provision for Denied}$$

Methodology for Estimating the IBNP

The method applied in estimating IBNP claims is called claims development (or lag) method. It is an estimation technique under which historical claim data, such as the number and amount of claims are grouped into the lag time periods in which claims were incurred and the time periods in which they were paid. The development method uses these groupings to create a claims processing or lag development pattern, which is used to determine completion factors to help estimate the unpaid portion of incurred claims and come up with the ultimate value of claims. The paid claims are then subtracted from the ultimate value of claims by admission month to estimate IBNP reserves.

The IBNP was estimated by admission month, and separately by COVID and non-COVID claims. The ultimate value of claims was estimated by admission months and subtracted the cumulative paid claim to arrive at the estimated IBNP claims.

Calculation of IBNP Claims – Non- COVID Claims

Ultimate claims for Non-COVID claims were estimated using the Paid Chain Ladder/Development Method (PDA), wherein the reported paid claims by admission month are divided by the appropriate completion factors. We based the completion factors on historical experience except for the immediate few months for which another method needs to be employed as completion factors may not be credible during these months. The paid claims are then subtracted from the ultimate value of claims by admission months to estimate IBNP reserves.

Calculation of IBNP Claims – COVID Claims

IBNP claims for COVID claims were calculated separately for “testing” and “other than testing,” utilizing distinct illness codes. However, the

ultimate value of COVID claims was estimated by multiplying average paid per case (AVPC) to the ultimate claim count taking into account the frequency and severity of COVID claims instead of historical claims data due to lack of a well-established payment pattern and insufficient experience to use a credible lag method. The AVPC is obtained by dividing the cumulative paid claims amount by the paid claims count. The ultimate claim count was based on the number of reported claims to PhilHealth, and the official DOH reported number of COVID cases and number of administered tests. For 2023, since more stable paid claims information for the pandemic years had become available, lesser uncertainty margins had been considered.

Further, the following are included in the computation of the IBNP:

- **Claims Handling Expense**
It refers to the cost incurred by PhilHealth in managing and processing claims. Five percent claims handling expense loading was applied to 100% of the IBNR and 50% of the ICS. This assumes that 50% of expenses are associated with opening the claim, and the remaining 50% is for closing the claim.
- **Provision for Denied Claims**
Denied claims are claims determined to be invalid and unworthy of payment due to an absolute deficiency that cannot be remedied through return to Health Facility or due to a finding of an unmet requirement.
Based on the history of payments realized for claims initially denied, there are claims that may be approved even ten years after admission date.
In projecting the provision for denied claims, the historical ratio of total paid claims at various development periods for previously denied claims to the total denied claims as of December 31, 2024 was first determined. After which, the selected ratio for future developments was multiplied by the total denied claims to come up with the provision.
- **Margin for Adverse Deviation**
It reflects the degree of uncertainty of the best estimate assumption. Because of the high level of uncertainty inherent in estimating IBNP reserves, and to reflect a higher level of comfort in the recorded number than that in the best estimate of the IBNP, a 10% margin for adverse deviation was added in the computation.

4. FINANCIAL RISK MANAGEMENT

Risk Management is not the sole responsibility of the Management and the BOD. It is the responsibility of every employee in all levels in the Corporation. To ensure the systematic management of risks it is facing while implementing the NHIP, PhilHealth established the Enterprise-wide Risk Management and developed the Risk Management Manual and Policy. These Manual and Policy will serve as a guide in the implementation of the risk management framework and processes in the Corporation.

Alongside, it adopted the Risk Governance Framework and defined the roles and responsibilities in the management of risks in the Corporation. It talks about the three lines of defenses in managing risks in the Corporation.

- a. The first line of defense has the ownership, responsibility and accountability for directly managing the risks. These are the Business Process Owners (BPOs) who have the responsibility of managing the day-to-day risks that they will encounter as they do their respective works. And they have the responsibility to report the risks so that immediate action can be done to address those risks that affect their work in particular and the Corporation in general;
- b. The second line of defense oversees, monitors and facilitates the implementation of an effective Risk Management practices by the first line of defense. These are the Risk Management Committee of the Board (RiskCom), the Management and Project

Management Team-Risk Management (PMT-RM) as the technical arm of RiskCom. They are mainly responsible for developing policies for Management's or BOD's approval and monitor the risk management policies, define work practices and advise when needed by the BPOs with regard to risk management and compliance; and

- c. The third line of defense is the audit functions for both internal and external auditors which will provide assurance to the Board and Management on the effectiveness of the first and second line of defense in managing risks.

4.1. Financial Risk Factors

This is the risk that results from unexpected changes in external markets, prices, rates and liquidity supply and demand.

a. Credit risk

This refers to the risk of loss arising from the Corporation's counterparty to perform contractual obligations in a timely manner. This includes risk due to (a) failure of a counterparty to make required payments on their obligations when due (Default Risk) and (b) default of a counterparty before any transfer of securities or funds or once final transfer of securities or funds has begun but not been completed (Settlement Risk).

The Corporation's placements in fixed term deposits with Authorized Government Depository Banks (AGDBs) are in accordance with Sec. 5.2 of Department of Finance (DOF) Circular No. 002-2022, on the Revised Guidelines on Authorized Government Depository Banks, which mandates all Government Owned and Controlled Corporations (GOCCs), National Government Agencies (NGAs) and Local Government Units (LGUs) to maintain and deposit government funds only with AGDBs.

The Corporation implements a structured and standardized evaluation guideline, credit ratings and approval processes. Investments undergo technical evaluation to determine their viability/acceptability. Due Diligence process (i.e., yield versus comparable financial instruments, term of the indebtedness including redemption feature, liquidity of the issuance, monitoring of issuer and counterparty risks including risks arising from implementation, and other portfolio and strategic considerations) and information from third party (e.g., Philippine Ratings Services Corporation and Credit Ratings and Investors Services Philippines, Inc.) are used to determine if counterparties are credit-worthy. With respect to bond underwriters and selling agents, PhilHealth as a qualified institutional buyer of debt securities under Section 10.1 (L) of the Securities Regulation Code, deals only with domestic universal bank or domestic investment house duly assigned by the issuer as underwriter or selling agent for the distribution of debt issuances.

Purchase of peso denominated government securities under the Non-Restricted Trading Environment (NRTE) of the Philippine Dealing Exchange is based on Treasury Circular No. 04-2014, Implementing DOF Department Order No. 068- 2014 entitled, "Revised Rules and Regulations for the Issuance, Placement, Sale, Service and Redemption of Treasury Bills and Bonds under R.A. No. 245, As Amended". Evaluation of counterparty trading participant in the NRTE platform, at the minimum, is measured on the Government Securities Eligible Dealer's (i) good standing in the Philippine Dealing Exchange, (ii) Risk-Based Capital Adequacy Ratio (CAR), (iii) Financial Standing on Earnings and Profitability, (iv) compliance to Securities Regulation Code and Code of Ethics Governing Financial Market Activities in the Philippines, and (iv) positive track record of service with other government agencies.

To avoid significant concentrations of exposures to specific industries or group of issuers and borrowers, PhilHealth investments are regularly monitored to ensure that these are always within the prescribed cumulative ceilings specified in Sec. V.D.2 of CO 2021-0057 PhilHealth Omnibus Guidelines on Fund Investments (Corporate Order No. 2021-0057). The said Corporate Order outlines the policies and guidelines in determining and managing exposure limits to investment instruments

including the negative list of investments that are considered not ethically and socially responsible by the Corporation such as companies with exposures in sin products like tobacco, liquor, alcohol, including gaming, and mining and quarrying.

Investments in government securities are not impaired. Only investments in corporate bonds are subject to impairment. Allowance

EAD		*PD		*LGD		ECL
Exposure at Default	X	Probability of Default	X	Loss Given Default	X	Expected Credit Loss

for impairment is computed as follows:

**Probability of Default and Loss Given Default is based on Bloomberg*

As regards criteria for accreditation of applicant collecting agents for premium collections, PhilHealth requires submission of three-year Audited Financial Statements, as one of the documentary requirements for accreditation. Financial evaluation of the applicants comprised the analysis on liquidity ratio, solvency ratio, capital adequacy ratio and profitability ratio vis-a-vis the current Bangko Sentral ng Pilipinas (BSP) Performance Indicators for banks, including non-banks with quasi-banking function. Once accredited, on an annual basis, submission of their yearly financial statements is required to monitor financial standing of the Accredited Collecting Agents (ACAs) to mitigate any losses that may be incurred by the concerned ACAs. Other documentary requirements such as Articles of Incorporation, Certificate of Good Standing with Existing Industry Association, Electronic Banking Authority, among others are likewise submitted by PhilHealth ACAs when renewing their accreditation to assess their capabilities as collecting agents.

On Cash Management: The Corporation maintains its accounts in any of the Authorized Government Depository Banks (AGDBs) in compliance with DOF Circular No. 01-2017 namely: Land Bank of the Philippines and Development Bank of the Philippines. These are the depository and servicing banks for the Corporation's over-the-counter collections, disbursements, funding and settlement accounts which are covered by Payroll Servicing Agreement and Memorandum of Agreement on One Fund Disbursement Account, respectively. Hence, lesser vulnerability of default given the Corporation's compliance to regulatory agencies and the service agreements with the AGDBs.

b. Liquidity Risk

This refers to the risk of loss, though solvent, due to insufficient financial resources to cover for liabilities as they fall due. It also involves the risk of excessive costs in securing such resources. This risk also refers to (a) unanticipated changes in liquidity supply and demand that may affect the organization through inability to meet contractual obligations or default (Funding Liquidity Risk) and (b) asset illiquidity or the risk of loss arising from inability to realize the value of assets, without significant reduction in price, due to bad market conditions (Market Liquidity Risk).

PhilHealth manages this risk through daily monitoring of cash flows in consideration of future payment due dates and regular premium collection receipts. PhilHealth also maintains sufficient portfolio of highly marketable assets that can easily be liquidated as protection against unforeseen interruption to cash flow such as Special Savings Deposits with AGDBs, Treasury Bills and Government Securities with remaining life of less than one (1) year.

As to management of ACAs, they are required to remit their collections to PhilHealth on a daily basis in accordance with the DOF Circular Nos. 01-2015 dated June 1, 2015 and 01-2017 dated May 11, 2017. Compliance to existing covenants in the Collection and Remittance Agreements with ACAs is strictly monitored by the Corporation and close monitoring through daily reconciliation of collections and remittances by ACAs are being done and implemented including the imposition of penalties and interests for late remittances, under remittances and late submission of required reports and documents.

On Cash Management, the Corporation also maintains its collections

account with the AGDBs as prescribed in DOF Circular No. 01-2017. The collection comprises remittances from its collecting agents and other receipts which include fund support coming from the National Government via funds releases through the Bureau of Treasury (BTr) and the Department of Budget and Management (DBM). These accounts are covered by service agreements: Memorandum of Agreement on Opening of One-Way Deposit Collection Account and Sweeping of Available Balance to Mother Accounts, Collection and Deposit Pick up Agreement, hence ensuring the steady inflow of funds to meet the daily fund requirements of the Corporation.

c. Interest Rate Risk

Interest rate risk is the risk that the fair value or future cash flows of a financial instrument will fluctuate because of changes in market interest rates.

5. CASH AND CASH EQUIVALENTS

This account is composed of the following:

	2024	2023
Collecting officers	192,723,515	92,901,541
Petty cash fund	4,816,928	4,587,625
Cash in bank	5,331,000,848	4,140,608,917
Special savings deposit (SSD)	48,743,566,043	26,541,020,999
Total	54,272,107,334	30,779,119,082

- 5.1. Collecting officers represent collections at the end of the month made by collecting officers which are to be deposited on the following working day.
- 5.2. The Corporation uses the imprest fund system in handling its petty cash fund. This fund is for the use of miscellaneous expenditures which cannot be conveniently paid by check wherein an officer holding the fund is also properly bonded in accordance with law.
- 5.3. Cash in banks represent various bank deposits that are unrestricted and available for current operations. Included in this account are cash denominated in foreign currencies that are translated to peso using the closing rate as of reporting dates.
Cash in bank earns interest based on the prevailing interest rates. Interest earned on bank deposits amounted to P1,883,362 and P1,623,148 in December 31, 2024 and December 31, 2023 respectively.
- 5.4. Special Savings Deposits (SSDs) are term deposits of one (1) day up to ninety (90) days with interest rates higher than the regular savings deposit rates and evidenced by either passbook or schedule of deposits. The interest rates ranging from 5.520 to 5.950 percent and 5.151 to 5.300 percent as of December 31, 2024 and December 31, 2023, respectively.
Interest Earned on SSDs amounted to P1,875,585,179 and P1,029,695,126 in December 31, 2024 and December 31, 2023, respectively.

6. INVESTMENTS IN TIME DEPOSITS

	2024	2023
Investment in time deposits - local	51,940,265,675	135,604,790,555
Investment in time deposits - foreign	1,371,304	1,306,983
Total	51,941,636,979	135,606,097,538

Investments in Time Deposits are term deposits of ninety-one (91) to three hundred sixty- four (364) days with interest rate ranging from 5.850 to 6.215 percent and 5.800 to 6.350 percent per as of December 31, 2024 and December 31, 2023, respectively.

7. RECEIVABLES

This account is composed of the following:

	2024	2023
Receivable from direct contributors	53,238,261,323	48,538,095,465
Due from NGAs	80,505,393,377	49,237,566,382
Due from LGUs	210,493,756	221,579,356
Due from LGUs - PCPN	46,757,413	168,215,436
Accrued interest receivable from investment	6,297,076,325	6,450,894,713
Other receivables	59,037,372	48,617,228
Receivables	140,357,019,566	104,664,968,580
Allowance for impairment	(30,006,442,445)	(24,062,310,205)
Receivables, net	110,350,577,121	80,602,658,375

- 7.1. Receivable from direct contributors represent accruals of premium contributions from direct contributor members. For direct contributors from the employed sector (government and private) it is based on the generated spa (note 3.2) while the other membership categories were based on the extracted data from the database. No accrual of interest and penalties for missed periods. See note 2.17

	2024	2023
Receivable from direct contributors	53,238,261,323	48,538,095,465
Allowance for impairment	(9,134,620,878)	(3,173,108,592)
Receivables, net	44,103,640,445	45,364,986,873

Expected Credit Loss (ECL) for CY 2024 and CY 2023 is as follows:

	2024	2023
Premium Receivable - Government	12,883,527,043	11,346,899,124
Premium Receivable - Private	36,573,170,286	34,338,455,309
Premium Receivable ~Self Earning	3,665,399,336	2,569,034,708
Premium Receivable - Land Based	116,164,658	116,164,658
Premium Receivable ~ Organized Group		4,000
Total	53,238,261,323	48,370,557,799
Loss Rate	17.158%	6.560%
Expected Credit Loss	9,134,620,878	3,173,108,592

The ECL is computed by multiplying the exposure at default by the Loss Rate (1-Collection Efficiency Rate). The significant increase in the allowance for impairment is due to the higher actuarially computed potential collection over actual collection in CY 2024 vis-a vis CY 2023.

- 7.2. Due from NGAs account represents premium contributions for the following:

	2024	2023
NHTS		
No. of enrollees	15,912,756	12,618,921
Amount	37,957,697,000	28,076,492,000
Senior Citizen		
No. of enrollees	12,703,773	8,557,139
Amount	41,270,304,800	20,537,134,800

Point-of-Service (POS)		
No. of enrollees	317,271	187,623
Amount	839,239,200	450,295,200
Fortuitous event		
No. of enrollees	56,135	56,135
Amount	134,724,000	134,724,000
Department of Public Works and Highways (DPWH)		
Amount	269,975,573	–
DBM-Procurement	33,281,923	38,749,501
Due from NGAs – BIR	170,881	170,881
Total	80,505,393,377	49,237,566,382
Allowance for impairment	(20,671,858,800)	(20,671,858,800)
Net amount	59,833,534,577	28,565,707,582

Details of Due from NGA's:

Enrollment of 2,384,258 indigent members under the Listahanan of the DSWD identified through NHTS-PR for CY 2024 based on billing to the DBM dated July 22, 2024,	7,152,774,000
Enrollment of 909,477 indigent members under the Listahanan of the DSWD identified through NHTS-PR for CY 2024 based on billing to the DBM dated October 18, 2024.	2,728,431,000
Enrollment of 4,146,634 Senior Citizens profiled as members in the database for CY 2022 based on billing to the DBM dated Sept. 12, 2024	20,733,170,000
Enrollment of 129,648 financially-incapable Point-Of-Service (POS-FI) patients as determined by DOH for CY 2024 based on billing to the DBM dated October 10, 18, and 30, 2024.	388,944,000
DBM-Procurement	33,281,923
DPWH - fund transfer	269,975,573
Due from NGAs – BIR	170,881
Less: Allowance for impairment	–
2024 Total	31,306,747,377

Enrollment of 12,618,921 indigent members under the Listahan 2 of the DSWD categorized through NHTS-PR program for CY 2023 based on billing to the DBM dated March 22, 2023 and May 26, 2023.	28,076,492,000
2023 Total	28,076,492,000

Enrollment of 187,623 qualified members under the 2019 POS program for the period January to December 2018 per billing to DBM	450,295,200
2018 Total	450,295,200

Enrollment of 2,258,757 Senior Citizens which were automatically renewed for the period January to December 2016 per billing to DBM through DOH [P12,998,827,200 - P7,577,809,200 (Collection)]	5,421,018,000
Less: Allowance for impairment	(5,421,018,000)
Total 2016	–

Enrollment of 5,416,178 Senior Citizens for CY 2015 per billing to DBM	12,998,827,200
Less: Allowance for Impairment	(12,998,827,200)
Total 2015	–
Enrollment of 56,135 families of which 55,474 families came from Region VIII who were casualties of Typhoon Yolanda and for the extended insurance coverage under the Fortuitous Event Program implemented as a component of the NHIP for the billing period January to December 2014.	134,724,000
Enrollment of 882,204 Senior Citizens for the period of October to December 2014 per billing to DBM	2,117,289,600
Less: Allowance for Impairment	(2,252,013,600)
Total 2014 (Net)	–
Grand Total	59,833,534,577

- 7.2.1 Due from NGA's - DBM represents accruals of premium contribution for Indirect Contributor for NHTS, Senior Citizen, POS and members affected by fortuitous event.
- 7.2.2 Due from NGA's - DBM Procurement represents prepayment for purchases of common supplies.
- 7.2.3 Due from NGA's- DPWH represents transfer of fund for the construction of PhilHealth Regional Office II building.
- 7.2.4 Allowance for Impairment was provided for Due from the National Government for the years 2016 and below.
- 7.3. Due from LGUs is the account representing the outstanding accounts receivable of premium contributions from various LGUs. The details of the account are as follows:

Region	2024	2023
National Capital Region (NCR)	6,040	6,040
Cordillera Administrative Region (CAR)	105,365	105,365
II	–	–
III	60,751,528	69,349,028
IVA	59,500	127,600
V	795,000	795,000
VI	145,230,923	147,530,923
VII	3,545,400	3,665,400
Total	210,493,756	221,579,356
Less: Allowance for Impairment	(199,962,767)	(215,725,656)
Due from LGUs, net	10,530,989	5,853,700

The ECL is 94.9970068% of the exposure default amounting to P210,493,756 as at December 31, 2024 and 97.3581295% of the exposure default amounting to P221,579,356 as at December 31, 2023.

- 7.4. Due from LGU- Primary Care Provider Network (PCPN) are fund transferred through frontloaded capitation to contracted Public Network for primary care services to our members. As of reporting period, only four PhilHealth Regional Offices have engaged Public Network. The balance of the frontloaded capitation is at P46,757,413 and P168,215,436 as at December 31, 2024 and December 31, 2023, respectively.

PRO	2024	2023
CAR	6,310,878	15,976,299
III	–	25,293,361
IV-A	40,446,535	72,983,582
XII	–	53,962,194
Total	46,757,413	168,215,436

- 7.5. Accrued Interest Receivable from investment amounting to P6,297,076,325 and P6,450,894,713 as at December 31, 2024 and December 31, 2023, respectively, represent interest earned as at the reporting date from special savings deposit, time deposit, investment securities at amortized cost which are due for collection on the following year.
- 7.6. Other Receivables amounting to P59,037,372 and P48,617,228 as at December 31, 2024 and December 31, 2023, respectively, represent disallowances and due from officers and employees. The allowance in CY 2023 was reverse since due from employees and Disallowances are not subject to impairment.

8. INVENTORIES

Inventories as of December 31, 2024 and December 31, 2023 were stated at cost less any allowance for impairment.

This account includes the following:

	2024	2023
Office supplies and materials inventory	109,697,004	89,600,570
Semi-expendable machinery and equipment	44,912,260	19,294,077
Semi-expendable furniture, fixture and books	19,201,329	18,273,914
Inventories	173,810,593	127,168,561
Less: Allowance for Impairment	(1,452,311)	(1,452,311)
Inventories, net	172,358,282	125,716,250

- 8.1. Supplies and Materials Inventory account amounting to P109,697,004 and P89,600,570 as at December 31, 2024 and December 31, 2023, respectively, represents small tangible items that are expected to be used within one (1) year from the reporting date.
- 8.2. Semi-Expendable Machinery and Equipment account amounting to P44,912,260 and P19,294,077 as at December 31, 2024 and December 31, 2023, respectively, represents office equipment, information and communication technology costing less than P50,000.
- 8.3. Semi-Expendable Furniture, Fixtures, and Books account amounting to P19,201,329 and P18,273,914 as at December 31, 2024 and December 31, 2023, respectively, represents furniture, fixtures and books costing less than P50,000 inventories, hence, no additional impairment was provided for the current year.

9. OTHER CURRENT ASSETS

This account is composed of the following:

	2024	2023
Other Receivables		
Other Current Assets	416,175,740	2,776,735,267
Other Current Assets	416,175,740	2,776,735,267
Less: Allowance for Impairment	(29,322,095)	(54,184,980)
Other Current Assets, net	386,853,645	2,722,550,287

- 9.1. Other Current Assets amounting to P416,175,740 and P2,776,735,267 as at December 31, 2024 and December 31, 2023, respectively, represent Fines and Penalties from ACAs, payment recovery, DCPM amounting to P372,203,771, and compromise penalties of hospitals amounting to P24,264,032 for the taxable year 2003-2004 per BIR decision with reference No. PO6-15 dated April 14, 2015 is included in the balance as well. The Debit-Credit Payment Method (DCPM) was introduced in 2021 during the pandemic to comply with Section 47(l) of the

Revised Implementing Rules and Regulations of the National Health Insurance Act of 2013 of Republic Act No. 7875, as amended by R.A. Nos. 9241 and 10606, prescribes that "all completed claims, except those under investigation, shall be paid within the sixty (60) calendar days from the receipt of the Corporation. As such, for in process claims ageing 60 days and beyond were paid and tagged as DCPM population subject to processing and reconciliation.

The DCPM recorded as Other Current Assets amounting to P372,203,771 are now subject to reconciliation and Payment Recovery Policy of the Corporation.

- 9.2. Allowance for Impairment amounting to P29,322,095 and P54,184,980 were provided due to ECL as at December 31, 2024 and December 31, 2023, respectively. Details as follows:

2024	Total Amount	Allowance	Net Amount
Other Receivables - Accrued Fines & Penalties from ACAs	3,194,216	(2,185)	3,192,031
Other Receivables - Personal Calls	4,304	-	4,304
Other Receivables - Refund from Benefit Payment - Employer	9,329,679	(2,811,416)	6,518,263
Other Receivables - Filing Fees	73,566	-	73,566
Other Receivables - GPAI	67,473	(67,473)	-
Other Receivables - Payment Recovery	4,560,397	-	4,560,397
Other Receivables - DCPM	372,203,771	-	372,203,771
Other Receivables - Others	26,742,334	(26,441,021)	301,313
Total	416,175,740	(29,322,095)	386,853,645

2023	Total Amount	Allowance	Net Amount
Other Receivables - Accrued Fines & Penalties from ACAs	3,015,562	(2,987,172)	28,390
Other Receivables - Personal Calls	69,237	(69,043)	194
Other Receivables - Refund from Benefit Payment - Employer	30,960,520	(29,574,738)	1,385,782
Other Receivables - Filing Fees	210,500	-	210,500
Other Receivables - GPAI	67,473	(67,473)	-
Other Receivables - Payment Recovery	13,907,670	-	13,907,670
Other Receivables - DCPM	2,701,167,046	-	2,701,167,046
Other Receivables - Others	27,337,259	(21,486,554)	5,850,705
Total	2,776,735,267	(54,184,980)	2,722,550,287

Allowance are provided for long outstanding other receivables while other receivables such as personal calls, filing fees, receivables under payment recovery from the HCI and the DCPM are not subject to impairment since these are all deemed collectibles.

The significant decrease of the allowance is due to collection of the other receivable - refund from benefit payment and the reversal of the allowance for other receivable - accrued fines and penalties from ACAs since these are all deemed collectibles since they are accredited collecting agents as of reporting period. Further, any balance of the accrued fines and penalties can be deducted from the service fee payable to them.

10. INVESTMENT SECURITIES AT AMORTIZED COST

This account is composed of the following:

	2024	2023
Investment in Treasury Bills	28,284,279,260	-
Investment in Government Bonds	347,228,619,666	331,680,257,444
Investment in Corporate Bonds	16,147,910,000	4,447,910,000
Total	391,660,808,926	336,128,167,444
Less: Allowance for Impairment	(6,879,452)	(531,590)
Investment, net	391,653,929,474	336,127,635,854

10.1. Investment in Treasury Bills with a maturity period of 91 to 364 days and with an average interest rate of 5.9993 percent as at December 31, 2024. There are no placement as at December 31, 2023.

	Interest Rate	2024	2023
Treasury Bills	6.0330	9,425,067,727	-
Treasury Bills	5.9820	9,429,650,719	-
Treasury Bills	5.9830	9,429,560,814	-
Total		28,284,279,260	-

10.2. Investment in Government Bonds with a maturity period of more than one (1) year and with an average interest rate of 5.2587 percent and 3.9794 percent as at December 31, 2024 and December 31, 2023, respectively.

	Interest Rate	2024	2023
Retail T-bonds	5.067%	75,500,000,000	71,000,000,000
RTB non-restricted environment	5.450%	271,728,619,666	258,180,257,444
Tokenized Treasure Bonds	-	-	2,500,000,000
Sub-Total		347,228,619,666	331,680,257,444

10.3. Investment in Corporate Bonds with a maturity period of more than one (1) year and with an average interest rate of 6.3335 and 5.9024 percent as at December 31, 2024 and December 31, 2023, respectively.

	Interest Rate	2023	2022
2.5 yrs SM Prime Holdings Fixed Rate Bonds	6.2069	1,000,000,000	1,000,000,000
4 yrs SM Prime Holdings Fixed Rate Bonds	6.2151	1,000,000,000	1,000,000,000
3 yrs SM Prime Holdings Fixed Rate Bonds	6.5754	200,000,000	-
6 yrs SM Prime Holdings Fixed Rate Bonds	6.3275	500,000,000	500,000,000
5 yrs SM Prime Holdings Fixed Rate Bonds	6.7537	200,000,000	-
7 yrs SM Prime Holdings Fixed Rate Bonds	6.9650	300,000,000	-
3 yrs Robinson Land Corp. Fixed Rate Bonds	6.0972	270,000,000	270,000,000
5 yrs Robinson Land Corp. Fixed Rate Bonds	6.1663	856,910,000	856,910,000
1.5 yr DBP Fixed Rate Bonds_Series 4	6.4126	821,000,000	821,000,000
1.5 yr DBP Fixed Rate Bonds_Series 5	6.1020	5,000,000,000	-

1.5 yr DBP Fixed Rate Bonds_Series 6A	6.0503	3,000,000,000	-
3.0 yr DBP Fixed Rate Bonds_Series 6B	6.1294	3,000,000,000	-

Sub-Total	16,147,910,000	4,447,910,000
Less: Allowance for Impairment	(6,879,452)	(531,590)
Total	16,141,030,548	4,447,378,410

Interest earned on Investment Securities at Amortized Cost amounted to P18,551,293,516 and P12,163,006,707 as at December 31, 2024 and December 31, 2023, respectively.

11. INVESTMENT PROPERTY

This account is composed of the following:

- 11.1. A parcel of land situated in Quezon City with a total area of 17,230.50 square meters (sq. m.). This property has a carrying amount of P439,377,751. The Head Office of PhilHealth shall be constructed in this lot. Relative to the construction of the proposed PhilHealth Corporate Center, the Corporation acquired an additional 145 sq. m. lot from the National Housing Authority (NHA) located in Barangay Pinyahan, East Avenue, Quezon City at a cost of P2,439,735. It has an average fair market value of P3,122,167 as of March 11, 2024
- 11.2. A property with a total area of 4,355 sq. m. acquired from Fort Bonifacio Development Corporation (FBDC), in exchange for the Corporation's investments in FBDC located in Fort Bonifacio, Taguig City. This asset with a carrying amount of P413,845,805 was appraised with an average fair market value of P4,569,048,500 as of March 11, 2024

12. PROPERTY AND EQUIPMENT

This account is composed of the following: *(refer to the table on next page)*

Included under Land and Buildings accounts are the following:

- 12.1. A parcel of lot with a total area of 1,544 sq. m. and a building with a total floor area of 3,287 sq. m. purchased by PRO III for its permanent Regional Office in San Fernando, Pampanga at a cost of P25,520,363. The land and building were appraised with an average fair market value of P101,333,000 as of March 12, 2024. PRO III also purchased a warehouse in San Fernando City, Pampanga with a total area of 1,831 sq. m. and was appraised with an average fair market value of P28,449,500 as of March 12, 2024.
- 12.2. A parcel of lot with a total area of 2,897 sq. m. located in Tuguegarao City which will be used as the PRO II's Regional Office. This property was received by way of donation, through a Memorandum of Agreement (MOA) executed between the Department of Public Works and Highways (DPWH) Region II and the Corporation. This property which is carried in the books at P4,056,000 and was appraised with an average fair market value of P52,580,500 as of March 12, 2024.
- 12.3. A parcel of lot with a total area of 600 sq. m. located in Gov. Democrito O. Plaza Government Center, Patin-ay, Prosperidad, Agusan del Sur which will be used as the PRO CARAGA's Local Health Insurance Office (LHIO) San Francisco Corporate Office. This property was received by way of donation, through Deed of Donation executed by the Provincial Government of Agusan del Sur in favor of PhilHealth office. This property is recorded at P1,086,000.
- 12.4. A parcel of lot with a total area of 3,000 sq.m. amounting to P16,500,000 (market value) located in the Regional Center Site in Rawis, Legaspi City as allocated by the Regional Government Center of Region 5 through Certificate of Allocation RGCS-CA-2024-07 dated June 26, 2024. The property has an assessed value of P1,650,000.

2024

	Land and land improvements	Building and structure / leasehold improvements	Infra. Assets	Construction in progress	Furniture & fixtures equipment & books	Motor vehicles	Total
Cost							
Balance at beginning of year	20,514,062	271,442,444	161,613	30,641,378	2,252,567,053	441,809,051	3,017,135,601
Additions	138,045,682	44,510,122		3,452,923	355,312,459	3,590,000	544,911,186
Adjustments	(2,874,998)	(30,129,333)	-	(572,107)	(89,396,361)	(11,390,166)	(134,362,965)
Balance at end of year	155,684,746	285,823,233	161,613	33,522,194	2,518,483,151	434,008,885	3,427,683,822
Accumulated Depreciation							
Balance at beginning of year	1,498,376	193,093,991	-	-	1,787,556,344	207,327,547	2,189,476,258
Depreciation	68,653	18,233,559	29,090	-	134,026,675	36,518,421	188,876,398
Adjustments	-	(30,319,467)	-	-	(56,002,458)	(10,979,056)	(97,300,981)
Balance at end of year	1,567,029	181,008,083	29,090	-	1,865,580,561	232,866,912	2,281,051,675
Net Book Value, Dec. 31, 2024	154,117,717	104,815,150	132,523	33,522,194	P652,902,590	201,141,973	1,146,632,147

2023

	Land and land improvements	Building and structure / leasehold improvements	Infra. Assets	Construction in progress	Furniture & fixtures equipment & books	Motor vehicles	Total
Cost							
Balance at beginning of year	20,514,062	264,490,613	-	29,017,394	2,248,131,498	333,671,182	2,895,824,749
Additions	-	12,175,674	161,613	1,691,463	101,233,775	259,730,500	374,993,025
Adjustments	-	(5,223,843)	-	(67,479)	(96,798,220)	(151,592,631)	(253,682,173)
Balance at end of year	20,514,062	271,442,444	161,613	30,641,378	2,252,567,053	441,809,051	3,017,135,601
Accumulated Depreciation							
Balance at beginning of year	1,498,376	175,073,281	-	-	1,699,918,231	194,947,882	2,071,437,770
Depreciation	-	23,620,225	-	-	163,212,719	36,501,497	223,334,441
Adjustments	-	(5,599,515)	-	-	(75,574,606)	(24,121,832)	(105,295,953)
Balance at end of year	1,498,376	193,093,991	-	-	1,787,556,344	207,327,547	2,189,476,258
Net Book Value, Dec. 31, 2023	19,015,686	78,348,453	161,613	30,641,378	465,010,709	234,481,504	827,659,343

12.5. A property with a total area of 2,713.sq.m located in Fermin Drive, Brgy. Carmino Nuevo, Zamboanga City. It has a carrying value of P81,390,000 and an appraised value of P107,163,500 as of December 31, 2024.

12.6. Three (3) parcel of lot located in Barangay Tolali, Marawi City, Lanao del Sur with a total area of 1,843 sq. m. amounting to P36,136,470 with an appraised value of P6,000 per sq.m.

12.7. The significant decrease of accumulated depreciation for Building and Other Structure/Leasehold Improvements is due to the dropping/ derecognition of various leasehold improvements dated CY 2003-2020 wherein lease contracts attributed for these improvements have expired.

PRO IV-B	118,020,169	(73,273,319)	44,746,850
PRO V	78,146,518	(29,655,681)	48,490,837
PRO VI	95,066,141	(25,455,140)	69,611,001
PRO VII	2,801,182	(2,147,634)	653,548
PRO VIII	20,994,128	(5,400,107)	15,594,021
PRO X	88,921,902	(18,922,653)	69,999,249
PRO XI	49,585,375	(12,575,562)	37,009,813
PRO CARAGA	69,775,206	(59,732,115)	10,043,091
Total	2,007,381,737	(692,056,989)	1,315,324,748

2023

	Total Amount	Accumulated Depreciation	Net Amount
Head Office	441,721,703	(395,366,236)	46,355,467
PRO NCR	149,228,616	(74,035,177)	75,193,439
PRO CAR	19,802,526	(4,950,887)	14,851,639
PRO I	5,433,703	(2,074,372)	3,359,331
PRO III	141,479,810	(49,160,411)	92,319,399
PRO IV-A	82,520,259	(55,394,278)	27,125,981
PRO IV-B	107,158,529	(62,564,750)	44,593,779
PRO V	78,146,519	(3,606,841)	74,539,678
PRO VI	22,285,297	(4,433,338)	17,851,959
PRO VII	87,505,524	(83,885,597)	3,619,927
PRO VIII	12,941,045	(10,127,694)	2,813,351

13. RIGHT-OF-USE ASSETS

Right-of-Use (ROU) assets are office spaces and warehouses being leased by PhilHealth with a multi-year contract.

Summary of ROU Assets follows:

2024			
	Total Amount	Accumulated Depreciation	Net Amount
Head Office	362,632,041	(70,511,786)	292,120,255
PRO NCR	868,088,664	(292,924,898)	575,163,766
PRO CAR	62,236,190	(18,621,874)	43,614,316
PRO I	3,052,975	(2,298,454)	754,521
PRO III	140,671,013	(65,057,450)	75,613,563
PRO IV-A	47,390,233	(15,480,316)	31,909,917

PROXI	50,112,775	(43,754,111)	6,358,664
PRO CARAGA	70,722,319	(47,534,567)	23,187,752
PRO BARMM	16,704,773	(16,704,773)	-
Total	1,285,763,398	(853,593,032)	432,170,366

13.1. The significant decrease of the accumulated depreciation is due to the derecognition of Right-of-Use Assets after the expiration of the lease contracts.

Details of ROU Assets as at December 31, 2024 are as follows:

Lessor	Use	Lease Period	Amount
Head Office			
Zapanta Realty & Dev't. Corp.	Office Space	06/01/2024 to 05/31/2027	151,956,774
Columbia Estate Properties, Inc.	Office Space	06/01/2024 to 05/31/2027	141,497,460
Columbia Estate Properties, Inc.	Warehouse	06/01/2024 to 05/31/2027	19,285,580
Fortune General Ins. Corp.	Office Space	06/01/2024 to 05/31/2027	15,935,787
Fortune Life Ins. Co., Inc.	Office Space	06/01/2024 to 05/31/2027	33,956,440
Sub-Total Head Office			362,632,041
PRO NCR			
BNC Properties Inc.	LHIO Parañaque	08/31/2022 to 07/31/2027	35,632,781
Columbia Estate Properties, Inc.	NCR Central Branch and LHIO Q.C	01/01/2023 to 12/31/2027	187,329,463
Fibertex Corporation	LHIO Rizal	03/01/2022 to 02/28/2027	29,805,510
Intraland Resources and Devt. Co.	NCR North Branch	01/01/2023 to 12/31/2026	78,405,600
Iriz One Properties Inc	LHIO Pasig	01/01/2023 to 12/31/2027	38,699,044
Lica Management	LHIO Mandaluyong	05/01/2023 to 04/30/2028	50,607,009
Lica Management	LHIO Makati	08/01/2023 to 08/01/2025	19,426,212
Manong R Realty & Dev't. Corporation	LHIO Fairview	01/01/2023 to 12/31/2027	36,480,000
Mega Accents Home Furnishing Inc.	LHIO Las Piñas	09/01/2022 to 08/31/2027	57,600,000
ORAS Realty Corporation	LHIO Manila	01/01/2023 to 12/31/2026	21,100,800
Panorama Technocenter	Office of the Vice President	12/15/2023 to 12/15/2026	81,662,573
2616 Really, Inc	South Branch	08/01/2024 to 08/31/2027	199,692,000
Kalayaan Devt. and Industrial Corp.	LHIO Caloocan	10/01/2024 to 04/31/2027	26,308,800
Department of Migrant Workers	POEA	01/01/2024 to 12/31/2026	2,177,280
Manong R Realty & Dev't. Corporation	Warehouse	09/01/2024 to 09/01/2025	3,161,592
Sub-Total PRO NCR			868,088,664
PRO CAR			
Lester Astudillo	Abra Office Space	04/01/2022 to 03/03/2027	5,276,892
Montanosa Pastoral Resources Corp	Baguio Office Space	04/01/2023 to 03/31/2025	12,172,602

Precious Sarah Residence-Apayao	Apayao Office Space	07/16/2023 to 07/15/2025	1,039,177
KEDAWEN RENTALS	Mt. Province-Office Space	01/01/2024 to 12/31/2026	2,493,247
SNOBT Inc.	PRO NCR Office Space	05/01/2024 to 04/30/2027	31,882,858
A and B Realty/Alejandro A. Berad	Benguet Office Space	04/16/2024 to 04/15/2027	8,174,970
JDT COMMERCIAL CENTER - B	Ifugao Office Space	04/01/2024 to 03/31/2027	1,196,444
Sub-Total PRO CAR			62,236,190

PRO I			
Municipality of Laoa, Pangasinan	LHIO Eastern Pangasinan Warehouse	03/01/2022 to 02/28/2025	339,610
B & D Bldg, Allied Properties	Satellite Office, La Union	05/16/2022 to 05/15/2025	810,249
Marmor Realty	LHIO Western Pangasinan	01/01/2023 to 12/31/2025	1,903,116
Sub-Total PRO I			3,052,975

PRO III			
3Aces Commercial Building	LHIO Sta. Maria	06/01/2023 to 07/31/2028	14,769,206
Tarlac Metrotown Corporation	LHIO Tarlac	12/15/2023 to 12/14/2028	8,575,670
Dona Rita Realty Corp.	Branch Office	07/06/2022 to 07/05/2027	53,503,199
Analita T. Mangahas	LHIO Gapan	08/01/2022 to 07/31/2027	10,418,399
AAZ Building	Warehouse	06/01/2024 to 05/31/2029	12,754,053
Oscar Santos	LHIO Olongapo	08/01/2022 to 07/31/2027	18,232,199
JCB Building	LHIO Gapan satellite office	11/09/2020 to 11/28/2025	1,426,970
Juan D. Nepomuceno Sons, Inc.	LHIO Angeles	04/30/2021 to 04/29/2026	14,730,365
Alice B. Gonzales	LHIO Iba	04/30/2021 to 04/29/2026	6,260,952
Sub-Total PRO III			140,671,013

PRO IV-A			
Lucena Grand Central Terminal, Inc.	PRO IVA Office Space	10/01/2024 to 09/30/2027	5,126,731
Calamba Medical Center	LHIO Calamba Storage Space	07/01/2020 to 06/30/2025	2,491,822
Manuelito Lorica	LHIO Gumaxa Office Space	02/01/2021 to 01/31/2026	6,676,068
James Uy	LHIO SPC Office Space	08/01/2023 to 07/31/2028	16,745,257
Lucena Grand Central Terminal, Inc.	LHIO Lucena Office Space	06/01/2024 to 06/30/2027	7,305,021
Calamba Medical Center	LHIO Calamba Space	11/01/2024 to 10/31/2027	9,045,334
Sub-Total PRO IV-A			47,390,233

PRO IV-B			
Willy D. Tan	LHIO Lipa Warehouse	09/05/2023 to 09/04/2028	1,644,537
Willy D. Tan	LHIO-Lipa	11/25/2024 to 11/25/2027	13,130,863

Edgardo P. Perez	LHIO Tanauan	06/02/2022 to 06/02/2027	17,562,077
Sunview Proper	Regional Office Warehouse	12/07/2023 to 12/06/2028	10,522,783
XRC Mall Developer	Regional Office	07/01/2020 to 06/30/2025	59,818,372
Karangyan Builders & Traders	LHIO Oriental Mindoro	06/01/2021 to 05/31/2026	6,425,659
Bernardo B. Go, Jr.	LHIO-Palawan	06/29/2023 to 06/28/2028	8,915,878
Sub-Total PRO IV-B			118,020,169
PRO V			
ANST Corp.	Office Space	12/31/2023 to 11/30/2026	45,757,069
SORCOM Leasing	Office Space	12/31/2023 to 11/30/2026	6,539,562
Westhub	Office Space	10/01/2023 to 09/30/2026	10,845,164
Delos Santos	Office Space	10/01/2023 to 09/30/2026	15,004,723
Sub-Total PRO V			78,146,518
PRO VI			
HEVA Management & Devt. Corp.	LHIO Iloilo	01/01/2023 to 12/31/2027	17,394,808
Barangay Caticlan	Philhealth Express	06/16/2023 to 06/15/2028	258,777
Northern Negros Planters Asso	LHIO Sagay	01/01/2023 to 12/31/2027	4,631,712
Joseph Royce Agapilo Tamayo Ojacastro	Office Space-LHIO Akklan	01/13/2024 to 01/12/2029	18,428,385
Donna Marie Encamacion	Office Space-LHIO Antique	11/09/2023 to 11/28/2028	11,347,800
Pedro P. Zayco	Office Space-LHIO Kabankalan	10/19/2023 to 10/18/2028	11,234,045
Heva Management and Development Corporation	Warehouse (PRO VI)	01/01/2023 to 12/31/2027	5,877,809
Pueblo de Panay, Inc.	Office Space (LHIO Capiz)	01/16/2024 to 01/15/2029	11,402,465
426 Holdings, Inc.	Office Space (LHIO Bacolod)	10/18/2023 to 10/18/2028	10,364,951
Danilo L. Yap	Office Space (Boracay Satellite)	01/16/2024 to 01/15/2029	4,125,389
Sub-Total PRO VI			95,066,141
PRO VII			
Province of Siquijor	Office Space	01/01/2017 to 12/31/2026	2,585,426
Talibon DC General Merchandise	Warehouse	03/16/2020 to 03/15/2025	215,756
Sub-Total PRO VII			2,801,182
PRO VIII			
Hugh Trevor Realty Warehouse	Office Space	12/15/2022 to 12/15/2025	1,979,835
OPBL REALTY	Office Space	07/01/2024 to 06/30/2027	8,343,606

MAV Properties	Office Space	05/16/2024 to 04/17/2027	6,840,014
Nestor Alvarez	Office Space	11/01/2024 to 10/31/2027	2,484,122
CINCO Real Property Estate Leasing	Office Space	12/09/2022 to 11/18/2025	1,346,551
Sub-Total PRO VIII			20,994,128

PRO X

Limketkai and Sons, Inc.	Office Space	06/01/2024 to 05/31/2027	72,238,297
N&G Realty and Devt. Corp.	Warehouse	06/01/2024 to 05/31/2027	12,684,435
Gonzales-Gimeno Realty, Inc.	Office Space	01/01/2024 to 12/31/2025	2,613,013
Cherob Real Estate	Office Space	01/01/2024 to 12/31/2027	1,386,157

Sub-Total PRO X 88,921,902

PRO XI

DDIS Inc.	Warehouse	7/1/2022 to 06/30/2027	5,767,069
828 Commercial Building	Office Space	07/01/2022 to 06/30/2027	1,228,796
Roche Building	Office Space	05/01/2023 to 05/31/2028	8,118,078
Valgosons Realty Incorporated	Office Space	04/16/2024 to 04/15/2027	34,471,432

Sub-Total PRO XI 49,585,375

PRO CARAGA

Abcalo Commercial Building	Office Space	07/01/2020 to 06/30/2025	43,598,301
Mn Bayalas Commercial	LHIO Bislig	12/19/2023 to 12/19/2026	3,930,713
CARAGA Ramlizdy Corp.	LHIO Butuan City	12/01/2020 to 11/30/2025	12,129,493
Saint Francis Realty	Office Space	06/01/2022 to 05/31/2025	7,060,208
JTP Realty	LHIO Tandag	03/01/2022 to 02/28/2025	3,056,491

Sub-Total PRO CARAGA 69,775,206

GRAND TOTAL 2,007,381,737

14. INTANGIBLE ASSETS - Net

This account is composed of the following:

2024				
	Computer Software	Website	Devt. In Progress - Computer Software	Total
Cost				
Balance at beginning of year	533,377,359	31,940	47,651,497	581,060,796
Additions	206,149,743	-	-	206,149,743
Adjustments	(389,083)	-	-	(389,083)
Balance at end of year	739,138,019	31,940	47,651,497	786,821,456
Accumulated Amortization				
Balance at beginning of year	266,695,532	-	-	266,695,532

Amortization	52,560,345	–	–	52,560,345
Balance at end of year	319,255,877	–	–	319,255,877
Accumulated Impairment Loss	162,065,199	–	–	162,065,199
Carrying Amount	257,816,943	31,940	47,651,497	305,500,380

2023				
	Computer Software	Website	Dev't. In Progress – Computer Software	Total
Cost				
Balance at beginning of year	478,285,622	31,940	47,651,497	525,969,059
Additions	–	–	–	–
Adjustments	55,091,737	–	–	55,091,737
Balance at end of year	533,377,359	31,940	47,651,497	581,060,796
Accumulated Amortization				
Balance at beginning of year	261,227,306	–	–	261,227,306
Amortization	5,468,226	–	–	5,468,226
Adjustments	–	–	–	–
Balance at end of year	266,695,532	–	–	266,695,532
Accumulated Impairment Loss	55,012,145	–	–	55,012,145
Carrying Amount	211,669,682	31,940	47,651,497	259,353,119

This account mainly pertains to various software applications/programs, the majority of which are licenses used, to fully utilize the capability of a software system to operate in a virtual environment. It includes computer development in progress – computer software which up to now is for customization.

From the total cost amount, intangibles with finite life is valued at P440,020,504 while intangibles with indefinite life is at P299,149,454 of which are internally developed and are being used in the operation amounting to P11,046,970. The balance is for development in progress – computer software.

15. OTHER NON-CURRENT ASSETS

This account is composed of the following:

	2024	2023
Advances to Special Disbursing Officers	1,615,160	72,967
Advances to Officers & Employees	335,952	1,536,198
Prepayments	37,733,181	22,596,881
Guaranty Deposits	150,994,993	123,759,591
Other Deposits	4,211,498	3,007,765
Sub-Total	194,890,784	150,973,402
DBM (transfer of NHIP Program from GSIS to PHIC)	155,235,240	155,235,240
PDIC (per MB Reso. 459 dated 04/07/2005)	327,103	327,103
PROs (from Various Health Providers-DCS)	700,555	700,555
Unserviceable Equipment	40,631,849	47,818,721
Serviceable equipment	1,048,334	1,540,186
Receivable from NGAs	394,741,430	394,749,368
Receivable from LGUs	446,137,783	448,977,370
Gross Long-term receivable	1,038,822,294	1,049,348,543

Less: Allowance for Impairment	(994,320,484)	(992,141,729)
Sub-Total	44,501,810	57,206,814
Grand Total	239,392,594	208,180,216

- 15.1. The Special Disbursing Officer (SDO) is an Accountable Officer duly designated, responsible and accountable for the proper management of funds for a specific legal purpose or activity and is properly bonded in accordance with law. The amount is P1,615,160 and P72,967 as at December 31, 2024 and December 31, 2023, respectively. The amount represents the funds on hand from the said officers as of reporting date.
- 15.2. Advances to Officers and Employees amounting to P335,952 and P1,536,198 as at December 31, 2024 and December 31, 2023, respectively, represent cash advances granted to authorized officers and employees for legal authorized purpose such as local and foreign travels.
- 15.3. Prepayments amounting to P37,733,181 and P22,596,881 as at December 31, 2024 and December 31, 2023, respectively, represent authorized payments made for insurance of motor vehicles of the Corporation from the Government Service Insurance System (GSIS), prepaid rent, prepaid registrations and other prepayments.
- 15.4. Guaranty Deposits amounting to P150,994,993 and P123,759,591 as at December 31, 2024 and December 31, 2023, respectively, represent transactions made by the Head Office and PROs in compliance with the requirements provided in the contracts for office rentals. These deposits are made for the faithful performance of the provisions of the lease agreements and shall cover possible damages to the leased premises. These are refundable at the end of the service agreement.
- 15.5. Other Deposits amounting to P4,211,498 and P3,007,765 as at December 31, 2024 and December 31, 2023 pertains to advance deposit for office rental.
- 15.6. Long Term Receivable from the DBM amounting to P155,235,240 as at December 31, 2024 and December 31, 2023 represents surcharges for late remittance of the employer counterpart for premium contribution. However, allowance for impairment account of the same amount was provided for after evaluation of such factor as aging of accounts, collection experience in relation to particular receivable and identified doubtful accounts.
- 15.7. Long Term Receivable from Philippine Deposit Insurance Corporation (PDIC) amounting to P327,103 as at December 31, 2024 and December 31, 2023 was the result of the Monetary Board Resolution No. 459 dated April 7, 2005 placing Hermosa Savings and Loan Bank, Inc. under liquidation. As of last communication last March 10, 2023 as per directive of Audit Committee, no collection has been received. Nevertheless, PDIC has mentioned that we are on the priority list for payment upon liquidation of assets.
- 15.8. Debit/Credit Scheme (DCS) amounting to P700,555 as at December 31, 2024 and December 31, 2023 refer to the balance of advance payment to HCPs for the year 1999. Allowance for impairment of P700,555 was provided due to closure of the hospital facilities.
- 15.9. Unserviceable Equipment account amounting to P40,631,849 and P47,818,721 as at December 31, 2024 and December 31, 2023, respectively, represent equipment that are already for disposal. This account shall be further reclassified as Non-Current Asset-Held for Sale, once the requirements set upon by the Standard are met.
- 15.10. Serviceable Equipment account amounting to P1,048,334 and P1,540,186 as at December 31, 2024 and December 31, 2023,

respectively, represent pieces of equipment which are still functional but already obsolete and fully depreciated and ready for disposal. This account shall be further reclassified as Non-Current Asset- Held for Sale, once the requirements set upon by the Standard are met.

- 15.11. Receivable from NGAs amounting to P394,741,430 and P394,749,368 as at December 31, 2024 and December 31, 2023 represents deficiency in employer share of the Health Insurance Premium Contributions by different government agencies nationwide from CYs 2001 to 2008.
- 15.12. Receivable from LGUs represents long overdue premium contributions from various cities and municipalities all over the Philippines for the enrollment of their respective constituents under the Sponsored Program of the Corporation. It was reclassified from Current Receivable to Other Assets, details as shown below:

PRO	2024	2023
NCR	1,307,600	1,307,600
CAR	20,893,161	20,893,161
I	35,962,267	35,962,267
II	14,914,961	14,914,961
IV-A	14,124,485	14,124,485
IV-B	938,740	938,740
V	277,168,300	278,980,773
VIII	6,079,686	6,079,686
X	315,431	1,342,545
XI	13,036,341	13,036,341
CARAGA	59,480,259	59,480,259
BARMM	1,916,552	1,916,552
Total	446,137,783	448,977,370

- 15.13. Allowance for impairment was provided for the following uncollectible receivable accounts on the Corporation's policy on the setting up of allowance for doubtful or uncollectible/ impairment accounts receivable per PhilHealth Corporate Order No. 2017-0010 dated January 20, 2017.
- 15.14. Per PhilHealth Corporate Order No. 2017-0010, the Corporation shall establish and maintain an Allowance for Doubtful/Impairment Accounts representing an estimate of the amount of accounts receivable that will not be collected. The goal in recording this allowance is to show, as accurately as possible, the net realizable value of accounts receivable on the financial reports. The allowance for impairment shall be updated at the end of the fiscal year.

	2024	2023
DBM (transfer of NHIP Program from GSIS to PhilHealth)	155,235,240	155,235,240
PDIC (per MB Resolution 459 dated April 7, 2005)	327,103	327,103
PROs (from various Health Providers- DCS)	700,555	700,555
Receivable from NGAs	394,741,430	394,741,430
Receivable from LGUs	443,316,156	441,137,401
Total Allowance for Impairment	994,320,484	992,141,729

16. FINANCIAL LIABILITIES

This account is composed of the following:

	2024	2023
Accounts payable	1,304,127,477	1,008,249,743
Accrued benefits payable	35,026,580,520	37,238,607,745
Due to officers and employees	511,025,501	476,810,765
Operating lease payable	4,409,347	4,573,713
Other financial liabilities	15,000	15,000
Total	36,846,157,845	38,728,256,966

Below is the breakdown of the Accounts Payable:

	2024	2023
Personnel Services	28,162,954	28,162,954
Miscellaneous and Other Operating Expenses (MOOE)	1,092,893,137	919,290,262
Capital Expenditures (CAPEX)	183,071,386	60,796,527
Total	1,304,127,477	1,008,249,743

- 16.1. Accounts Payable – Personnel Services amounting to P28,162,954 as at December 31, 2024 and December 31, 2023, represent GSIS and PhilHealth premium which was part of the salary adjustments given to PhilHealth employees but subsequently disallowed by COA. The said amount is still outstanding and not yet remitted to GSIS in compliance with Opinion No. 56, s. of 2018 from the Office of the Government Corporate Counsel dated March 26, 2018 stating that PhilHealth should keep the amount originally intended to be remitted to GSIS as premiums corresponding to the adjusted salaries of its employees without prejudice to its remittance in the event the Notice of Disallowance is lifted.
- 16.2. Accounts Payable ~ Miscellaneous and Other Operating Expenditures(MOOE) amounting to P1,092,893,137 and P919,290,262 as at December 31, 2024 and December 31, 2023, respectively, represent procurement of goods or services which are due for payment.
- 16.3. Accounts Payable - CAPEX amounting to P183,071,386 and P60,796,527 as at December 31, 2024 and December 31, 2023, respectively, represent procurement of property and equipment which are due for payment.
- 16.4. Accrued Benefits Payable are accrued expenses from claims filed by members or HCLs and received by the corporation but not yet processed and unpaid as at reporting period
- Below is the breakdown of the Accrued Benefits Payable:

	2024	2023
Benefit Claims Processed	4,270,910,897	4,750,316,499
Benefit Claims Processed-Primary Care Benefit (PCB)	1,741,915	2,670,515
Benefit Claims Processed- KonSulTa	986,178,590	40,087,748
In-Course of Settlement (ICS)	28,546,696,519	32,037,165,698
ICS – in process	26,568,685,332	29,817,405,775
ICS – RTH	1,978,011,187	2,219,759,923
PCB	119,741,079	132,940,453
KonSulTa	1,101,311,520	275,426,832
Total	35,026,580,520	37,238,607,745

- 16.5. Accrued Benefit Payable - Processed amounting to P4,270,910,897 and P4,750,316,499 as at December 31, 2024 and December 31, 2023, respectively, represent benefit claims awaiting payment for the next crediting date. It also includes pending claims payment due to non-submission of official receipt from the HCI as well as non-negotiated checks by the HCI as of reporting date.
- 16.6. Accrued Benefit Payable - Primary Care Benefit (PCB) processed amounting to P1,741,915 and P2,670,515 as at December 31, 2024 and December 31, 2023, respectively, represent PCB awaiting payment.
- 16.7. Accrued Benefit Payable — KonSulTa processed amounting to P986,178,590 and P40,087,748 represents benefit claims for KonSulTa package awaiting payment as at December 31, 2024 and December 31, 2023, respectively.

The PhilHealth KonSulTa Program is a flagship initiative that aims to provide comprehensive primary care services to our members, ensuring accessible and affordable healthcare for all. It emphasizes preventive care, early diagnosis, and timely treatment to enhance the over-all well-being of members. It is governed by PhilHealth Circular No. 2024-0013.

The KonSulTa Benefit Package shall be paid as an annual capitation computed and released as performance-based payment. The maximum per capita rate shall be One Thousand Seven Hundred pesos (P1,700) for both public and private to be paid in tranches. The first tranche shall be Forty percent (40%) of P1,700 per registered member with First Patient Encounter (FPE). The second tranche shall be 60% of 1,700 or P1,020 based on the number of FPE done and achieved performance targets such as but not limited to Primary Care Consultation, Utilization of Laboratory services done and medicines dispensed.

- 16.8. Accrued Benefits Payable - ICS amounting to P28,546,696,519 and P32,037,165,698 as at December 31, 2024 and December 31, 2023, respectively, are estimated benefit claims still in process as of the reporting period. It includes ICS-RTH which are tagged within the allowable 60-day period within which to refile the RTH claims. The remaining prior year RTH claims were re-classified to Provision for Health Benefit - Incurred But Not yet Reported since these are all in the possession of the HCI and to conform with the total provision of Incurred But Not yet Paid (IBNP) per valuation report of the Actuary.
- 16.9. Accrued Benefits Payable — PCB amounting to P119,741,079 and P132,940,453 as at December 31, 2024 and December 31, 2023, respectively, are claims which are estimated to be outstanding as of report date.
- 16.10. Due to Officers and Employees amounting to P511,025,501 and P476,810,765 as at December 31, 2024 and December 31, 2023, respectively, represents liability to officers and employees for salaries, benefits and other emoluments including authorized expenses paid in advance by the officers and employees. Included in the accounts payable is the accrued personnel services (PS) for Welfare Support Assistance (WESA) from CY 2012 to CY 2020, based on the following grounds, to wit:

The PhilHealth Board of Directors issued PHIC Board Resolution No. 385, s. 2001, granting the payment of the Welfare Support Assistance (WESA) of P4,000.00 each to all qualified employees effective January 01, 2001. This is in lieu of the subsistence and laundry allowances paid to public health workers under Republic Act (R.A.) No. 7305, otherwise known as the Magna Carta of Public Health Workers. However, On February 07, 2008, the COA issued Notice of Disallowance (ND) PHIC 2008-003 (2004) disallowing the payment of WESA contending that the payment was made without legal basis in the absence of approval from the Office of the President. On Appeal, the COA en banc sustained the disallowance.

PhilHealth then filed a Petition before the Supreme Court (SC) seeking to reverse and set aside the Decision of the COA. The SC in its Decision dated November 29, 2016, held that the Health Secretary approved the grant of the WESA together with ten (10) other members of the Board does not make the act any short of the approval required under the law. As far as the Magna Carta and its Revised Implementing Rules and Regulations are concerned, the then Health Secretary Dr. Alberto G. Romualdez, Jr. voted in favor of the WESA's issuance, and for as long as there exists no deception or coercion that may vitiate his consent, the concurring votes of his fellow Board members does not change the fact of his approval. PhilHealth's grant of the WESA was aptly sanctioned not only by Section 12 of the SSL which explicitly identifies laundry and subsistence allowance as excluded from the integrated salary, but also by statutory authority, particularly, Section 22 and 24 of the Magna Carta. In view of such fact, the PhilHealth officers cannot be found to have approved the issuance of the same in bad faith or in gross negligence amounting to bad faith for it was well within the parameters set by law.

As a result of the decision of the SC (GR No. 213453) reversing the ND and COA decision on WESA on November 29, 2016, PhilHealth Management, with the concurrence of the Board of Directors, decided that such WESA should be given retroactive effect so that the rate of WESA, in lieu of Subsistence and Laundry Allowance, at P4,000 shall be deemed continuously granted starting February 2012 after its restoration by virtue of the PhilHealth Board of Directors Resolution dated December 2019.

Such an interpretation is consistent with the constitutionally-protected right of labor and that any doubt in the interpretation of laws, including compensation laws, should be resolved in favor of labor. Section 3 of Article XIII of the 1987 Constitution on Social Justice and Human Rights reads: "The State shall afford full protection to labor, local and overseas, organized and unorganized, and promote full employment and equality of employment opportunities for all." Section 18, Article II of the 1987 Constitution states that the State affirms labor as a primary social economic force. It shall protect the rights of workers and promote their welfare" and Section 10 declares that the State shall promote social justice in all phases of national development."

As at December 31, 2024, the remaining balance for WESA amounting to P95,756,730 are for retired and resigned employees.

- 16.11. Operating Lease Payable amounting to P4,409,347 and P4,573,713 as at December 31, 2024 and December 31, 2023, respectively, are for Rental expense during the month-to-month extension of the lease contract and lease contracts with 1 year or less term.
- 16.12. Other Financial Liabilities — this account is used to recognize the amount of other financial liabilities incurred which cannot be appropriately classified under any of the specific financial liability accounts.

17. INTER-AGENCY PAYABLE

This account consists the following:

	2024	2023
Due to BIR	243,710,396	203,657,423
Due to GSIS	79,052,536	98,020,839
Due to Pag-IBIG	6,736,785	4,560,725
Due to PhilHealth	14,354,965	14,884,305
Due to NGAs	29,883,914,750	10,339
Due to Government Corporations	427,443,703	427,443,702
Due to LGUs	727,585	829,628
Due to Treasurer of the Phil	29,534,289	12,673,524
Total	30,685,475,009	762,080,485

- 17.1. Due to BIR consists of liability for taxes withheld from employees' compensation and income payment to suppliers of goods and services which will be remitted in the subsequent period.
- 17.2. Due to GSIS, Due to Pag-IBIG and Due to PhilHealth comprise of amortization of loans availments, life and retirement insurance premiums payable to GSIS, contributions and amortization of loan availments payable to Pag-IBIG fund, and medical insurance premiums payable to PhilHealth deducted from the salaries of the Corporation's officials and employees and the employer's counterpart which will be remitted to the appropriate agencies in the subsequent periods.
- 17.3. Due to NGAs refers to the interest income of the Philippine Health Information Exchange projects of the Department of Health for remittance amounting to P9,067 and the unutilized government subsidy amounting to P29,883,905,683 to be returned to the Bureau of Treasury. However, on October 29, 2024, the Supreme Court in Pimentel vs. House of Representative et.al petition issued a Temporary Restraining Order (TRO) enjoining the respondent including PhilHealth from transferring the remaining 29.9 billion of PhilHealth Fund Balance scheduled transfer to the Unprogrammed Appropriation effective immediately.
- 17.4. Due to Government Corporations refer to accrual of UMID billing from GSIS and SSS as well as loan payment to National Home Mortgage Finance Corporation.
- 17.5. Due to LGUs refers to overpayment of premium contributions under sponsored programs which shall be applied to their additional enrollment during the year, if any. It also includes local tax withheld located outside the region for remittance to LGU.
- 17.6. Due to Treasurer of the Philippines are payables for auditing services.

18. TRUST LIABILITIES

Trust Liabilities refer to funds from other sources which are held in trust for a specific purpose. A separate bank account is opened if specified or stated in the Agreement.

This account consists the following:

	2024	2023
UNFPA Project	38,453	38,057
Unclaimed Refund from HCPs	312,862,693	312,345,456
AHP - Protest Bond	82,125,320	76,125,320
Donations	7,862,292	8,623,879
Bail bond payable/guaranty/ security deposit pay	83,279,232	62,978,128
Retention fee	-	5,295,341
Global Development Project	1,986,538	1,958,886
Philippine Training Institute	6,793,310	6,540,391
Philippine Training Institute-NSSF	544,643	622,675
PhilHealth Run 2013	900	900
PhilHealth Run 2015	87,219	87,219
Calamity fund	80,768	80,752
PhilHealth Provident Fund	33,994,925	25,271,964
COVID-19 National Vaccine Indemnity Fund	522,114,112	521,995,906
LBP Electronic Salary Loan	103,223	667,464
Disallowance/Charges	389,766	389,767
Others	36,488,501	33,359,341
Total	1,088,751,895	1,056,381,446

- 18.1. Unclaimed Refund from Health Care Providers — refers to the benefit payment that must be reimbursed by the accredited

hospital to the members for a specified illness. The refund results from either an under-deduction or non-availment of benefits at point-of-service due to various circumstances at the time of hospitalization, and these have remained unclaimed by the members for some time now. The unclaimed refunds have been retrieved by PhilHealth from the accredited hospitals for immediate return to the concerned members. The list may be accessed through the PhilHealth website under Unclaimed Refund where basic information on the refunds was posted.

- 18.2. Accredited Health Provider (AHP) Protest Bond — these are cash protest bond paid by the HF's for cases appealed with the Corporation. The amount is equivalent to the fine imposed in

	2024	2023
Westmont Investment Corporation	2,945,656	2,945,656
Strategies and Alliance Corporation	3,043,853	3,476,458
Land Bank of the Philippines	110,000	110,000
Donation received by PROs	1,762,783	2,091,765
Total	7,862,292	8,623,879

the decision appealed from.

- 18.3. Donations include funds received from the following entities, including earned interest thereon:
- 18.4. Guaranty/Security Deposits Payable — this account is credited to recognize the incurrance of liability arising from the receipt/ withholding of cash or cash equivalents to guaranty (a) that the winning bidder shall enter into contract with the procuring entity; (b) performance by the contractor of the terms of the contract; and (c) that the contractor shall correct all discovered defects and clear/settle all third party liabilities. This account is debited for refund after the fulfillment of the purpose of the bond or forfeiture upon failure to comply with the purpose of the bond/retention, and/or adjustments.
- 18.5. Retention Fee — this account was reclassified to Guaranty/ Security Deposits Payable account.
- 18.6. Philippine Training Institute and NSFF - this account is credited to recognize collections from the conduct of the Certificate Course of ICD-10 Coding and interest income earned. This account is debited for the reimbursement of expenses incurred during the conduct of the Certificate Course of ICD-10 Coding.
- 18.7. Calamity Fund- refers to the donations collected from the employees to be distributed to employees affected by calamities.
- 18.8. PhilHealth Provident Fund (PPF) - refers to the employees' member contributions and loan payments deducted from the salaries to be remitted on the PPF office on the following month.
- 18.9. COVID-19 National Vaccine Indemnity Fund - a trust fund created to compensate any serious adverse effects (SAEs) such as death, permanent disability, or hospital confinement arising from the use of COVID-19 vaccine, experienced by people inoculated through the COVID-19 Vaccination program. It includes interest earned from the cash account of the trust fund.
- 18.10. Land Bank of the Philippines (LBP) Electronic Salary Loan - refers to loan payments of employees' to be remitted to LBP on the following month.

19. PROVISION FOR HEALTH BENEFITS

Below is the composition of the Provision for Health Benefits.

	2024	2023
Provision for IBNR	70,901,587,244	54,994,635,343
Provision for Denied	3,566,244,404	2,513,182,884
Total	74,467,831,648	57,507,818,227

19.1. Provision for Health Benefits — IBNR amounted to P70,901,587,244 and P54,994,635,343 as at December 31, 2024 and December 31, 2023, respectively. They are claims which are actuarially estimated to be in the possession of the Health Care Institutions (HCIs) as of the end of the month and have yet to be submitted to the Corporation within the allowable 60-day period after the date of discharge per Corporate Order No. 2015-0017 dated December 8, 2015. It includes Returned to Hospitals (RTH) which are still in the possession of the HCI and are yet to be refiled to PhilHealth.

It is computed as the balance after deducting the ICS and provision for denied claims from Incurred But Not yet Paid (IBNP) when IBNP are those claims yet to be paid by PhilHealth. The amount recorded includes, computation for claims handling and margin for adverse deviation.

Below is the composition of Provision for Health Benefit — IBNR

	2024	2023
Incurred But Not Yet Reported	38,279,660,971	19,925,757,103
Returned To Hospitals (RTH)	26,198,270,591	24,395,657,575
Claims Handling	2,595,689,585	2,532,767,581
Margin for Adverse Deviation	3,827,966,097	8,140,453,084
Total	70,901,587,244	54,994,635,343

The balance of the Provision for CY 2024 and 2023 is shown below.

Provision for IBNR, December 31, 2023	54,994,635,343
Less: Claims paid applied to balance	(16,588,865,500)
IBNR, balance for 2023	38,405,769,843
Less: Change as at December 31, 2024	32,495,817,401
Provision for IBNR, December 31, 2024	70,901,587,244

Provision for IBNR, December 31, 2022	95,098,573,409
Add: Adjustment due to understatement	808,499,918
IBNR, adjusted balance	95,907,073,327
Less: Claims paid applied to balance	(17,290,843,633)
IBNR, balance for 2023	78,616,229,694
Less: Change as at December 31, 2023	(23,621,594,351)
Provision for IBNR, December 31, 2023	54,994,635,343

The method applied in estimating IBNP claims is called claims development (or lag) method. It is an estimation technique under which historical claim data, such as the number and amount of claims are grouped into the lag time periods in which claims were incurred and the time periods in which they were paid. The development method uses these groupings to create a claims processing or lag development pattern, which is used to determine completion factors to help estimate the unpaid portion of incurred claims and come up with the ultimate value of claims. The paid claims are then subtracted from the ultimate value of claims by admission month to estimate the IBNP reserves.

The IBNP was estimated by admission month, and separately by COVID and non-COVID claims. The ultimate value of claims was estimated by admission months and subtracted the cumulative paid claim to arrive at the estimated IBNP claims.

19.2. Provision for Health Benefits - Denied Claims

The Provision for Health Benefits — Denied Claims amounted to P3,566,244,404 and P2,513,182,884 as at December 31, 2024 and December 31, 2023, respectively. They are claims determined to be invalid and unworthy of payment due to an absolute deficiency that cannot be remedied through return

to Health Facility or due to a finding of an unmet requirement. However, Health Facilities have filed a Motion for Reconsideration hence, it is probable that payment shall be made. The amount of the provision is actuarially estimated or computed.

20. OTHER PAYABLES

This account consists the following:

	2024	2023
Undistributed collections	1,274,859,440	2,424,370,183
Due to Non-government organization/civil society	433,329	433,329
Payable to Primary Care Provider Networks	86,568,943	–
Other payables – others	62,494,064	33,344,822
Total	1,424,355,776	2,458,148,334

20.1. Undistributed Collections refer to the unidentified premium collections, remittances of ACAs and accreditation fees directly credited to PhilHealth bank accounts as of reporting period.

20.2. Due to Non-Government Organization represents various reimbursement of PHIC Board of directors for meals and accommodation.

20.3. Payable to Primary Care Provider Networks represents additional payment after liquidation of the front loaded capitation.

20.4. Other Payables — Others includes employees' association dues to PhilHealth White, amortization of loan availed from PhilHealth Employees Association (PhiCEA), and PROs Cooperative deduction from the salary of PhilHealth employees.

21. DEFERRED CREDITS/UNEARNED INCOME

This account consists the following:

	2024	2023
Advance premium from direct contribution members	234,991,373	279,714,321
Premium contribution for NHTS enrollment (CYs 2013 and 2014)	516,844,200	516,844,200
Accreditation fees – ACAs	115,834	420,801
Accreditation fees – HCPs	3,799,759	2,793,000
Subsidy from LGUs	13,032,427	18,150,027
Others	306,300	306,300
Total	769,089,893	818,228,649

Other deferred credits/unearned income account refers to the payment of premium contribution, accreditation fees received in advance by the Corporation or income received not pertaining to the current year.

22. LEASE PAYABLE

Lease Payable amounting to P1,347,913,581 and P451,812,395 as at December 31, 2024 and December 31, 2023, respectively, are lease payable of the Corporation mostly office and warehouse rental ranging from 1 to 5 years.

23. LEAVE BENEFITS PAYABLE

This provision is measured at the best estimate of the amount needed to settle them at the end of the reporting period. The obligation is measured at its "actual expected value". Charges to this account are disbursements for terminal leave pay and monetization.

Balance, 12/31/2023	814,468,334
Less: Monetization 2023	(604,091)
Accrued Balance 2023	813,864,243
Less: Payments	(104,162,696)
Add : Increase in Provision for CY 2024	133,593,386
PRO Balance	523,951
Leave Benefits Payable, 12/31/2024	843,818,884

2024			
Region	Regular	Casual	Total
Head Office	183,559,977	16,337,562	199,897,539
NCR	53,757,821	28,418,008	82,175,829
CAR	35,389,479	6,081,711	41,471,190
I	25,103,365	12,253,079	37,356,444
II	31,228,670	5,944,000	37,172,670
III	36,626,574	18,638,200	55,264,774
IV-A	50,837,067	22,812,827	73,649,894
IV-B	19,408,408	12,070,399	31,478,807
V	23,981,068	11,237,952	35,219,020
VI	23,835,135	14,583,368	38,418,503
VII	25,473,093	8,802,110	34,275,203
VIII	24,008,667	5,463,228	29,471,895
IX	20,435,451	5,689,166	26,124,617
X	15,008,542	5,605,391	20,613,933
XI	16,505,187	8,175,752	24,680,939
XII	16,228,297	7,885,414	24,113,711
BARMM	26,687,305	7,399,460	34,086,765
CARAGA	11,531,313	6,815,838	18,347,151
Total	639,605,419	204,213,465	843,818,884

2023			
Region	Regular	Casual	Total
Head Office	176,982,487	15,577,760	192,560,247
NCR	52,448,664	26,988,901	79,437,565
CAR	32,658,554	5,929,276	38,587,830
I	29,988,559	11,153,305	41,141,864
II	28,966,812	6,126,667	35,093,479
III	35,136,109	20,134,585	55,270,694
IV-A	48,468,068	22,512,960	70,981,028
IV-B	17,595,448	11,814,399	29,409,847
V	23,073,433	10,945,966	34,019,399
VI	19,917,016	12,426,651	32,343,667
VII	24,394,134	9,306,641	33,700,775
VIII	23,847,723	6,320,960	30,168,683
IX	22,106,855	5,888,962	27,995,817
X	14,287,022	6,413,386	20,700,408
XI	14,447,154	6,841,220	21,288,374
XII	14,297,456	8,050,748	22,348,204
BARMM	10,969,078	7,013,715	17,982,793
CARAGA	23,748,674	7,084,895	30,833,569
Total	613,333,246	200,530,997	813,864,243

The provision of P843,818,884 and P813,864,243 represents money value of the earned leave credits as of December 31, 2024 and December 31, 2023 of officers and employees of PhilHealth.

24. PROVISION FOR INSURANCE CONTRACT LIABILITIES

Provision for ICL is the estimate of the expected future claims payments to be incurred less expected premium contributions in relation to

the liabilities that the Corporation is contractually obligated to pay to our members. It is computed as the expected net of present value of expenses (claims + expenses) and present value of collection plus margin for adverse deviation.

The actuarial valuation is based on PFRS 4 and projected / estimated by Actuary and was subjected to deliberation and approval of the PhilHealth Board.

Further, ICL is an estimate of future liabilities and does not include incurred/ filed or actual claims payable. Incurred/ filed or actual claims payables are recorded and classified as Accrued Benefits Payable in Note 16.4 and Provisions for Health Benefit in Note 19 for claims already incurred but not yet reported to PhilHealth.

Requisite Information	Assumptions Used			
	CY 2024		CY 2023	
Valuation	Closed group valuation		Closed group valuation	
Exposure	Registered Member Count as of Nov. 2024; with gross-up factors to the projected total population in 2024 (allocated to Sex, Sector, Age)		Projected Count as of December 2023 based on Registered Members Count as f Aug. 2023 (allocated to Sex, Sector, Age)	
Mortality rates	Graduated rates & age-last-birthday per 2017 PICM		Graduated rates & age-last-birthday per 2017 PICM	
Morbidity rates	On-Level (based On 2020-2024)		On-Level (based 2019-2023)	
Discount rates	IC 2024 Spot Rates		IC 2023 Spot Rates	
Inflation rate/Increase in Benefit Payout (Medical Trend)	Medical trends from 2025-2030 are base n the increases in benefit enhancements: 2025=92.73%, 2026=45.63%, 2027=21.83%, 2028=21.24%, 2029=16.20%, 2030=15.41% Then, 6.5% per year in anicipated starting 2031		3.2% - Historical Trend (Medical Trend & PCB Trend)	
Salary increase rate	3.7%		4.4%	
On premium increase	Premium as stated in RA 11223		Premium as stated in RA 11223	
National Government Subsidy	P195B-CY2026 and P100B-CY 2027 and onwards		P61.5B annually as appropriated in GAA or FY 2024	
On benefits	Year	Adj Rate	Year	Adj Rate
	2025	-21.0%	2030	-39.1%
	2026	-26.4%	2031	-40.1%
	2027	-31.1%	2032	-41.2%
	2028	-34.9%	2033	-42.3%
	2029	38.8%	2034-2044	43.4%
Average PCB Cost	PCB Package = P 1,700 Initial FPE (40%) - 100% utilization Follow up (60%) - 50% utilization Ave. PCB=70% of P1,700=P1,190		Php 587 per member	
Operating expense	Loss-Adjustment Ratio (LAE) 3.75% unpadded 3.45% MfAD 0.30% LAE = 20% PS + 80% MOOE + 100% Non-Cash; and further based on 3-year average.		5.0% (Expense Loading)	
Age at Retirement	63 for Government & Private; 60 for IPP and OFP		60	
Collection Efficiency	Increasing collection in: Private Sector, IPP and OFP with 10% inc. in year 1, 20% inc. in year 2, and 30% inc. in collection in year 3 onwards		Gradual increase to 100% by 2026 for private sector	
Frequency	Historical Indication		Historical Indication	
Severity	Historical Indication		Historical Indication	

Assumption	Particulars	
	CY 2024	CY 2023
Include GAA	No subsidy in 2025; P20B from PAGCOR and P80B for IC in the succeeding years	Includes the government subsidy of ₱1.5B in the inflows
Lifespan	2017 Phil. Intercompany Mortality Study by Life Insurance Committee of the Actuarial Society of the Philippines	2017 Phil. Intercompany Mortality Study by Life Insurance Committee of the Actuarial Society of the Philippines.
Margin for Adverse Deviation (MfAD)	0.30% to reflect the degree of uncertainty of the best estimate assumption of Net ICL. It is computed as the difference between Loss-Adjustment Ratio of 3.75% and unpadding ratio of 3.45%	10% to reflect the degree of uncertainty of the best estimate assumption of Net ICL.

Following the assumptions aforementioned, the ICL as approved by the Board of Directors to be recorded for CY2024 and 2023 are as follows:

Actuarial Estimates	2024	2023
Discounted Outflows	6,590,029,780,213	5,564,470,507,002
Discounted Inflows	5,417,015,507,496	4,519,328,789,724
Net ICL	1,173,014,272,717	1,045,142,717,278
MfAD	233,901,600,592	104,514,221,728
Net ICL including MfAD	1,406,915,873,309	1,149,656,439,006

Actuarial Estimates	2024	2023
Discounted Outflows	6,590,029,780,213	5,564,470,507,002
Discounted Inflows	5,417,015,507,496	4,519,328,789,724
Net ICL	1,173,014,272,717	1,045,142,217,278
MfAD	233,901,600,592	104,514,221,728
ICL, end of the year	1,406,915,873,309	1,149,656,439,006
ICL, beginning of the year	1,149,656,439,006	266,873,312,531
Change in ICL	257,259,434,303	882,783,126,475

The Actuarial Services and Risk Management Sector (ASRMS) performed Liability Adequacy Test as required by PFRS 4, using the same assumptions as stated above and in Note 2.23 as required by IC, the result of which is P1,406,915,873,309. The increase by P257,259,434,303 is recognized as Other Losses to be consistent on the recommendation by COA-GAS to take up the Increase as Other Losses in compliance to PAS 8, change in accounting estimates.

We note of the following limitation of the estimates:

The determination of the actual numbers of the Indirect Contributors that the National Government needs to support through premium subsidies. To address this concern, PhilHealth is coordinating with the Department of Social Welfare and Development (DSWD) and the Philippine Statistics Office (PSO);

The supply-side constraints of the health sector in the Philippines, such as the availability of health care facilities, medical equipment and health care professionals, will have a significant impact on the ability of members to avail of the existing and future benefit packages of the Corporation. These constraints might lead to lower benefit expenses in the future than what is currently estimated;

On the level of Government support to the Corporation in the future, Section 58 of RA 7875 explicitly states that the National Government guarantees the financial viability of the NHIP. Moreover, Section 37 of RA No. 11223 allows for DOH, in coordination with PhilHealth to request for supplemental funding from Congress in order to achieve the objectives of universal health care. These legal provisions might lead to higher government support, subsidies or otherwise, than what is currently estimated;

The Corporation is also in discussions with the Philippine Interpretations Committee (PIC) to craft a more appropriate ICL measure considering the special character of PhilHealth and the provisions of the UHC Act, such as the two-year expenditure ceiling on the reserve levels.

25. MEMBERS' EQUITY

Members' Equity consists the following:

	2024	2023
RETAINED EARNINGS		
Retained Earnings/(deficit) at Jan 1	-	-
Net Income excluding before change in ICL	64,303,869,319	174,064,380,907
Interest Income from reserve fund		
(Excess actual reserve)	(13,347,414,223)	
Prior year adjustments	4,797,008,943	14,437,516,296
Total Retained Earnings/(Deficit)	55,753,464,039	188,501,897,203
Surplus of the two year projected expenditures	183,712,078,544	
Remittance to BTR in compliance to DOF Cir. No. 003-2024 and Section XLIII of GAA	(89,883,905,683)	
Retained Earnings transferred to reserves		(188,501,897,203)
Retained Earnings/(Deficit) at December 31	149,581,636,900	-
RESERVE FUND		
Accumulated Revenue at January 1	464,286,992,149	275,785,094,946
Surplus of the two year projected expenditures	(183,712,078,544)	
Retained Earnings transferred to reserves		188,501,897,203
Reserve Fund at December 31	280,574,913,605	464,286,992,149
INTEREST INCOME FROM RESERVE FUND		
(Excess Actual Reserve)	13,347,414,223	
PROVISION FOR INSURANCE CONTRACT LIABILITIES (ICL)		
	(1,385,253,256,573)	(1,127,993,822,270)
Total Members' Equity	(941,749,291,845)	(663,706,830,121)

25.1. Reserve fund is recorded in compliance with Office Order No. 0145, s. of 2012 and based on the provisions of Section 11 of RA No. 11223 or the UHC Act, which states that PhilHealth shall set aside a portion of its accumulated revenues not needed to meet the cost of the current year's expenditures as reserve funds: Provided, That the total amount of reserves shall not exceed a ceiling equivalent to the amount actuarially estimated for two (2) years' projected Program expenditures which for this purpose is at P560.55 billion for CY 2023:

For CY 2024, the amount of the two years' projected Program expenditure is at P 280.6 (rounded-off) billion as computed by the Department of Finance and in compliance to the Special Provision 1(d), XLIII of the RA No. 11975 or General Appropriations Act (GAA) for FY 2024:

Provided further; that whenever actual reserves exceed the required ceiling at the end of the Corporation's Fiscal Year, the excess of the Corporation's Reserve Fund shall be used to increase the program's benefits and to decrease the amount of member's contributions.

Any unused portion of the reserve fund that is not needed to meet the current expenditure obligations or support the abovementioned programs shall be placed in investments to earn an average annual income at prevailing rates of interest and shall be referred to as the Investment Reserve Fund.

The excess of the two-year projected expenditures of 183.712 billion from the beginning balance of the reserve of P464.287 billion was transferred to the Retained Earnings account to conform with the computation of the reserve fund of 280.575 billion for CY 2024,

Below is the computation of the two years' projected program expenditure.

Year	Average of Two (2) Years Expenditure (Amounts in Billion)
2018-2019	269.715
2019-2020	277.957
2020-2021	276.055
2021-2022	300.770
2022-2023	278.378
Total of the Average of Two Years Expenditure	1,402.875
Two Years' projected program expenditure (P1,402.875 divided by five (5) years)	
	280.575

- 25.2. Interest income from reserve fund (excess actual reserves) amounting to P13,347,414,223 represent income earned from investing the reserve fund to comply with Section 11 of RA No. 11223 which state that "No portion of the reserve fund or income thereof shall accrue to the general fund of the National Government or to any of its agencies or instrumentalities, including government-owned or controlled corporations".

Below is the computation of Interest Income attributed to Reserve Fund.

Type	Investment Portfolio	Investment Income Attributed Investment Portfolio
Treasury Bonds	232.128	10.955
Retail Treasury Bonds	48.447	2.392
Total	280.575	13.347

- 25.3. Retained Earnings represent the cumulative results of normal and continuous operations of a Commercial Public Sector Entity including prior period, effects of changes in accounting policy and error other capital adjustments. This may also include funds set aside for various purposes in accordance with existing laws, rules and regulations. The Retained Earnings account amounted to P149,581,636,900 for CY 2024 and nil in CY 2023.

Excluded in the Retained Earnings is the investment income for the reserve fund amounting to 13.347 billion.

- 25.4. The difference from the ICL amount in the liability account amounting to P21,662,616,736 pertains to Non-Paying Members reserves previously set up before providing reserves for all members.

- 25.5. The resultant negative figure for the Members' Equity amounting to P941,749,291,845 is the net effect of change in accounting estimate in performing the adequacy test as required by PFRS 4 for 2024.

The Management believes that resulting significant increases in liabilities and net deficit for Members' Equity in CY 2024 and 2023 will not affect PhilHealth's ability to continue as a going

concern. Further, the Management is constantly monitoring the liquidity by assessing/projecting cash flows to ensure that liabilities are paid when they fall due. In addition, Section 59, of the Revised Implementing Rules and Regulations of the National Insurance Act of 2013 provides for Government Guarantee: By virtue of the sovereign guarantee provision, the Government of the Philippines guarantees the financial viability of the Program.

26. PREMIUM CONTRIBUTIONS

Details of Premium Contributions are as follows:

	2024	2023
Direct Contributors	199,222,845,019	157,353,034,317
Indirect Contributors	40,350,455,320	78,805,615,998
Total	239,573,300,339	236,158,650,315

26.1. Below is the breakdown of the Direct Contributors:

	2024	2023
Government	52,347,059,557	42,302,905,260
Private	138,092,036,082	105,907,437,477
Kasambahay	108,036,611	82,417,469
Family Driver	45,560	47,517
Migrant Worker - Land Based	189,572,789	246,390,099
Migrant Worker - Sea Based	173,518	346,071
Filipino with Dual Citizenship / Living Abroad	641,352	714,961
Women about to give birth	39,225	47,550
Professional Practitioner	86,159,109	52,268,514
Self-Earning Individual	6,551,509,026	7,498,713,197
Self-Earning Individual - Sole Proprietor	27,840,776	5,270,827
Self-Earning Individual - Group Enrollment Scheme	1,788,563,647	1,232,547,552
Foreign National	30,250,508	22,816,098
Others	917,259	1,111,725
Direct Contributors	199,222,845,019	157,353,034,317

Below is the breakdown of the Indirect Contributors:

	2024	2023
Indigents - NHTS	18,193,464,000	35,256,167,000
Senior Citizens	20,733,170,000	42,931,355,000
Special Government Programs - PAMANA	31,680,000	-
Sponsored	1,392,141,320	618,093,998
Indirect Contributors	40,350,455,320	78,805,615,998

The amounts collected come from the following members in accordance with Title III Section 5 of the Implementing Rules and Regulations of RA No. 7875 as amended otherwise known as the National Health Insurance Act of 2013 to wit:

- Income from the Direct Contributors come from the premium contributions of the following:
 - Government employees;
 - Private employees
 - All other workers rendering services, whether in government or private offices, such as job order contractors, project-based contractors and the likes

4. Owners of micro-enterprises
 5. Owners of small, medium and large enterprises
 6. Household Help – as defined in RA 10361 on "Kasambahay Law"
 7. Family Drivers
 8. Migrant Workers – as defined in RA 10022 (Migrant Workers Act) and RA 10801 (OWWA Act)
 9. Informal Sector
 10. Self-earning individuals
 11. Professional practitioners
 12. Filipinos with Dual Citizenship
 13. Naturalized Filipino Citizens
 14. Citizens of other countries working and/or residing in the Philippines
 15. Women about to give birth
 16. Foreign Retirees (Registered with Philippine Retirement Authority)
 17. All Filipinos aged 21 years and above who have the capacity to pay premiums.
- b. Income from the Indirect Contributors come from the premium contributions of the following:
1. Indigent-National Household Targeting System (NHTS), Senior Citizen, Persons with Disability (PWD) and Point of Service (POS) as appropriated in the General Appropriations Act (GAA);
 2. Sponsored members paid by another individual, government agency, or private entity according to the rules prescribed by the Corporation; and
 3. Special government programs as appropriated in the GAA.

27. INTEREST AND OTHER INCOME

Interest and other income account is as follows:

	2024	2023
Interest Income	25,799,518,987	20,700,881,752
Other Income	282,459,945	815,345,117
Total	26,081,978,932	21,516,226,869

27.1. Below is the breakdown of the Interest Income:

	2024	2023
Investment securities at amortized cost	18,551,293,516	12,163,006,708
SSDs and Investment in Time Deposit	1,875,585,179	1,029,695,126
Investment in Time Deposit	5,370,756,930	7,506,556,770
Savings and current deposits	1,883,362	1,623,148
Interest Income	25,799,518,987	20,700,881,752

27.2. Below is the breakdown of the Other Income:

	2024	2023
Accreditation fees – HCPs	31,373,459	32,124,718
Fines and penalties	176,477,296	700,636,775
Rent income	–	714,698
Donations in Kind	19,442,960	–
Gain on foreign exchange	217,125	158,488
Gain on sale of property, plant and equipment	4,800	14,100

Gain on sale of unserviceable property	281,578	322,548
Other gain	31,685	6,500
Reversal of Impairment loss	47,304,627	48,676,694
Miscellaneous income	7,326,415	32,690,596
Other Income	282,459,945	815,345,117

Reversal of Impairment Loss is credited when the allowance is being adjusted due to excessive provision or reversal of the allowance when collection was made.

Miscellaneous Income consists of income ranging from P100,000 and above. This includes income from proceeds from disposal of unserviceable properties, sale of valueless records, forfeited performance security and refund of benefit payment collected from delinquent employers.

Rent Income is an income earned from the portion of PRO III office building being leased out to a private entity.

28. BENEFIT CLAIMS EXPENSES

Benefit Claims Expenses for 2024 and 2023 are recognized at the time of admission.

This account is composed of the following:

	2024	2023
Members' Benefits for Direct Contributors	92,413,466,236	34,473,633,624
Members' Benefits for Indirect Contributors	87,087,211,879	39,303,979,133
Interim financing mechanism	98,478,800	–
PhilHealth Konsultasyon sulit at tama (KONSULTA) package	5,852,745,017	429,575,508
Total	185,451,901,932	74,207,188,265

Further details of the expense are the following:

	2024	2023
Paid	128,890,795,944	76,404,658,238
In-Course of Settlement (ICS)	23,012,227,066	18,910,941,493
Incurred But Not Yet Reported (IBNR)	33,548,878,922	(21,108,411,466)
Total	185,451,901,932	74,207,188,265

Paid claims refers to benefit claims that have been received and paid within the applicable year, either via check or bank crediting; ICS, in contrast, represents claims that have been received within the applicable year but still in process at the end of the reporting period; whereas IBNR refers to claims which are estimated to be in the possession of Healthcare Facilities (HFs) as of the end of the year and have yet to be submitted to the Corporation within the allowable 60-day period after the date of discharge.

The distribution of the Benefit Claims Expenses in 2022 and 2021 are as follows:

	2022	2021
Paid	128,890,795,944	76,404,658,238
In-Course of Settlement (ICS)	23,012,227,066	18,910,941,493
Incurred But Not Yet Reported (IBNR)	33,548,878,922	(21,108,411,466)
Total	185,451,901,932	74,207,188,265

The 2023 figure on the IBNR was negative due to the adjustments/change in the IBNR made in 2023 in the amount of P40.912 billion as computed by Actuary based on the recorded IBNR especially the years 2022 and 2021 as presented above. The adjustment is a change in accounting estimate and per PAS 8, change in accounting estimate shall be taken up on the period of change. Thus, benefit expense was recorded at P74.207 billion after effecting the adjustments.

28.1. Below is the breakdown of the Members' Benefits for Direct Contributors:

	2024	2023
Government	11,788,809,681	3,804,023,192
Private	31,575,330,536	8,520,948,761
Kasambahay	24,339,368	96,929,967
Family Driver	(34,812,534)	47,965,885
Migrant Worker - Land Based	864,024,347	493,975,614
Migrant Worker - Sea Based	(702,988,794)	2,003,835,137
Filipino with Dual Citizenship / Living Abroad	(2,259,554)	9,182,592
Professional Practitioner	(177,251,268)	429,309,094
Self-Earning Individual - New	31,402,367,726	7,843,820,839
Self-Earning Individual - Sole Proprietor	28,205,476	33,707,908
Self-Earning Individual - Group Enrollment	1,029,153,245	1,880,720,143
Foreign National	(110,537,290)	248,912,462
Lifetime Members	17,245,634,291	7,500,485,892
Others	(516,548,994)	1,559,816,138
Direct Contributors	92,413,466,236	34,473,633,624

The negative resultant of per membership category of benefit expense is mainly due to change in accounting estimate of the IBNP as computed by the Actuary. The decrease in the estimate of a particular membership category for CY 2024 is more than the previously estimated in CY 2023, hence, the negative resultant.

28.2. Below is the breakdown of the Members' Benefits for indirect Contributors:

	2024	2023
Senior citizens	31,206,592,843	4,580,027,113
Indigent - NHTS	34,021,042,361	5,443,312,872
Sponsored	21,859,576,675	29,280,639,148
Indirect Contributors	87,087,211,879	39,303,979,133

29. PERSONNEL SERVICES

Personnel services account includes:

	2024	2023
Salaries and wages	2,618,224,034	2,670,502,725
Other compensation	2,106,482,267	2,055,352,059
Statutory contributions	598,738,674	576,032,091
Other personnel services	139,206,718	46,679,099
Total	5,462,651,693	5,348,565,974

30. OTHER OPERATING EXPENSES

This account is composed of the following:

	2024	2023
MOOE	3,732,620,675	2,642,826,368
Financial Expenses	66,670,269	48,668,167
Non-cash expenses	6,637,565,383	1,363,247,503
Total	10,436,856,327	4,054,742,038

This account represents the administrative costs which must be within the limits prescribed in Section 72, Financial Management of the Revised Implementing Rules and Regulations of RA No. 7875, as amended by RA No. 9241 and RA No. 10606, known as the "National Health Insurance Act of 2013."

30.1. Below is the breakdown of the MOOE:

	2024	2023
Traveling Expenses	267,164,456	162,175,554
Training and Scholarship Expenses	111,353,009	39,601,308
Supplies and Materials Expenses	235,601,549	181,243,116
Semi-Expendable Expenses	132,462,399	73,074,412
Water Expenses	14,135,931	10,242,182
Electricity Expenses	201,386,719	180,138,901
Communication Expenses	124,272,990	202,510,701
Awards / Rewards, Prizes and Indemnities	7,274,012	7,392,355
Research, Exploration and Development Expenses	56,672,231	34,082,608
Confidential, Intelligence and Extraordinary Expenses	7,279,161	5,769,098
Auditing Services	85,512,142	82,765,175
Consultancy Services	11,888,888	2,616,840
Other Professional Services	783,616,079	580,012,226
Janitorial Services	105,193,788	91,744,080
Security Services	223,529,211	189,313,995
Repairs & Maintenance	35,698,703	30,942,286
Taxes, Duties and Licenses	930,733	5,894,034
Fidelity Bond and Insurance Expenses	34,525,182	25,092,528
Advertising Expenses	71,677,950	33,695,710
Marketing and Promotional Expenses	327,245,753	90,081,687
Printing and Publication Expenses	9,817,387	9,728,110
Representation Expenses	75,878,781	61,224,248
Transportation and Delivery Expenses	3,766,575	2,768,029
Rent / Lease Expenses	407,261,860	325,222,188
Membership Dues and Contri. to Organizations	11,861,208	9,846,471
Subscription Expenses	43,341,731	11,269,684
Donations	1,760,145	400,163
Major Events Expenses	46,802,851	29,749,394
Other MOOE - Others	294,709,251	164,229,285
Total	3,732,620,675	2,642,826,368

30.2. Below is the breakdown of the Financial Expenses:

	2024	2023
Interest Expenses	36,839,046	19,037,558
Bank Charges	52,876	86,851
Other financial charges	29,778,347	29,543,758
Total	66,670,269	48,668,167

Interest expense are recorded when payment is made for leases classified as finance lease in compliance with Philippines Financial Reporting Standard on Leases (PFRS 16).

Other financial charges pertain to transaction fees/service fees of ACAs and Accredited Collecting Banks, rental fees of National Registry of Scripless Securities facility and other financial charges.

30.3. Below is the breakdown of the Non-Cash Expenses:

	2024	2023
Depreciation	496,791,413	495,792,936
Depreciation - buildings & other structures	1,640,095	1,417,764
Depreciation - machinery and equipment	130,347,603	158,862,502
Depreciation - furniture and fixtures and books	3,679,072	4,350,217
Depreciation - transportation equipment	36,518,421	36,501,497
Depreciation - leased assets improvements	16,593,464	22,202,461
Depreciation - Right-of-Use-Assets	307,915,015	272,458,495
Depreciation - land improvements	68,653	-
Depreciation - infrastructure asset	29,090	-
Amortization - Intangible assets	52,560,345	5,468,226
Impairment Loss	6,084,165,641	861,069,539
Impairment loss - loans and receivables	5,961,512,286	833,389,410
Impairment loss - other receivables	4,954,468	19,965,748
Impairment loss - financial assets held to maturity	6,380,188	980,984
Impairment loss - Inventories	-	274,575
Impairment loss - property, plant & equipment	169,302	171,654
Impairment loss - intangible assets	107,441,846	6,287,168
Impairment loss - other assets	3,707,551	-
Losses	4,047,984	916,802
Loss on sale of property, plant, & equipment	2,162,540	-
Loss on FOREX	139,358	175,952
Loss on assets	62,456	608,932
Loss on sale assets	258,883	131,918
Other losses	1,424,747	-
Total	6,637,565,383	1,363,247,503

30.4. Other Losses is used to recognize the amount of losses not falling under any of the specific loss account. The account was debited for the increase in the estimated provision ICL for CY 2023 while other gains account will be credited for any decrease in the ICL.

	2024	2023
ICL, end of the year	1,406,915,873,309	1,149,656,439,006
ICL, beginning of the year	1,149,656,439,006	266,873,312,531
Increase/Change in ICL	257,259,434,303	882,783,126,475

Under PAS 8 on Accounting Policies, Changes in Accounting Estimates and Errors, a change in accounting estimate is an adjustment of the carrying amount of an asset or a liability, or the amount of the periodic consumption of an asset, that results from the assessment of the present status of, and expected future benefits and obligations associated with, assets and liabilities. Changes in accounting estimates result from new information or new developments and, accordingly, are not correction of errors. Paragraph 36 states that:

The effect of a change in an accounting estimate, other than a change to which paragraph 37 applies, shall be recognized prospectively by including it in profit or loss in:

- (a) *the period of the change, if the change affects that period only; or*
- (b) *the period of the change and future periods, if the change affects both.*

Further, Paragraph 37 states that:

To the extent that a change in an accounting estimate gives rise to changes in assets and liabilities, or relates to an item of equity, it shall be recognized by adjusting the carrying amount of the related asset, liability or equity item in the period of the change.

31. RELATED PARTY DISCLOSURES

Compensation of key management personnel.

The key management personnel refer to the executive team, with the rank of Senior Vice President up to PCEO. These individuals have the authority and responsibility for planning, directing, and controlling the activities of the Corporation.

The aggregate compensation of the executive officers for CYs 2024 and 2023 is as follows:

	2024	2023
Salaries and wages	64,896,538	64,900,031
Terminal benefits	-	-
Total	64,896,538	64,900,031

The BODs for CYs 2024 and 2023 received the following:

	2024	2023
Honorarium/per diem	8,001,600	7,184,000
Total	8,001,600	7,184,000

32. CHANGES IN PRESENTATION FORMAT OF CERTAIN ACCOUNTS IN THE FINANCIAL STATEMENT

This is to reclassify the presentation of Trust Liabilities to Operating Activities in order to conform with the correct presentation.

	Presentation in 2023	Change in Presentation Format	Presentation in 2023
CASH FLOWS FROM OPERATING ACTIVITIES			
Cash Inflows			
Cash received from premium Contribution	197,809,239,822		197,809,239,822
Collection of other income	1,237,371,504		1,237,371,504
Gain on Foreign Exchange	40,267		40,267
Collection of rent income	600,000		600,000
Trust Receipts	–	44,393,232	44,393,232
Total Cash Inflows	199,047,251,593	44,393,232	199,091,644,825
Cash Outflows			
Payment of benefit claims	(118,900,340,656)		(118,900,340,656)
Payment of operating expenses	(8,106,244,684)	6,973,450	(8,099,271,234)
Payment of financial charges	(29,555,380)		(29,555,380)
Loss on Foreign Exchange	(44,764)		(44,764)
Trust Disbursements		(26,500,639)	(26,500,639)
Total Cash Outflows	(127,036,185,484)	(19,527,189)	(127,055,712,673)
Net Cash Provided by Operating Activities	72,011,066,109	24,866,043	72,035,932,152
CASH FLOWS FROM INVESTING ACTIVITIES			
Cash Inflows			
Matured time deposits	157,103,588,752		157,103,588,752
Matured treasury bills	9,350,077,535		9,350,077,535
Matured treasury bonds	41,679,228,000		41,679,228,000
Interest received from investments	18,713,680,940		18,713,680,940
Proceeds from disposal of assets	380,100		380,100
Total Cash Inflows	226,846,955,327		226,846,955,327
Cash Outflows			
Placement on time deposits	(166,283,301,101)		(166,283,301,101)
Placement on treasury bonds	(106,631,289,408)		(106,631,289,408)
Accrued interest paid on placement of bonds	(1,011,889,408)		(1,011,889,408)
Purchase of Fixed assets	(314,822,914)		(314,822,914)
Total Cash Outflows	(274,241,302,831)		(274,241,302,831)
Net Cash Provided by/(Used in) Investing Activities	(47,394,347,504)		(47,394,347,504)
CASH FLOWS FROM FINANCING ACTIVITIES			
Cash Inflows			
Trust receipts	473,565,282	(473,565,282)	–
Total Cash Inflow	473,565,282	(473,565,282)	–
Cash Outflows			
Trust disbursements	(448,699,239)	448,699,239	–
Finance lease payments	(286,168,567)		(286,168,567)

Principal	(267,829,227)		(267,829,227)
Interest	(18,339,340)		(18,339,340)
Total Cash Outflows	(734,867,806)	448,699,239	(286,168,567)
Net Cash Provided by/(Used in) Financing Activities	(261,302,524)	(24,866,043)	(286,168,567)
INCREASE/(DECREASE) IN CASH AND CASH EQUIVALENTS	24,355,416,081	-	24,355,416,081
CASH AND CASH EQUIVALENTS, JANUARY 1	6,423,703,001	-	6,423,703,001
CASH AND CASH EQUIVALENTS, December 31	30,779,119,082	-	30,779,119,082

33. OTHER SIGNIFICANT AND RELEVANT INFORMATION

33.1. Benefit Enhancements

Since September 2024, PhilHealth has added 13 new and improved benefits to help Filipinos with medical cost. Hemodialysis coverage increased, allowing up to 156 sessions a year. The KONSULTA Package now includes free treatment for tuberculosis, malaria, Human Immunodeficiency Virus (HIV) or Acquired Immunodeficiency Syndrome (AIDS), and animal bites. Support for severe dengue and COVID-19 hospitalization also increased.

For major illnesses, kidney transplant coverage is now up to P2,000,000, heart attack treatment up to P523,853, and cataract surgery up to P187,100. Peritoneal dialysis (Z benefit package) is now promoted as a better option for kidney patients.

In December 2024, PhilHealth also increased its financial support by 50 percent for common treatments, such as pneumonia coverage from P15,000 to P29,250, cesarean delivery from P19,000 to P37,050, and cholecystectomy or gallbladder surgery from P 31,000 to P60,450.

Starting in CY 2025, PhilHealth will introduce new packages, including new packages, including Diagnosed Related Groups and a Comprehensive Outpatient Benefit Package. More serious conditions will also be covered, such as heart valve surgery, prostate and cervical cancer and open-heart surgeries. Chemotherapy coverage for lung, liver, ovarian and prostate cancer will also be adjusted. Additionally, children with disabilities will receive better healthcare support.

As of March 31, 2025, the effects of the increases or enhancement of benefit packages is now translated into higher benefit expense amounting to P73.465 billion compared to last year of the same period of P36.727 billion.

33.2. By virtue of Special Provision 1(d), XLIII of the RA No. 11975 or GAA for FY 2024 which directs the DOF to issue guidelines to implement the collection of Unprogrammed Appropriation sourced from the Fund balance of the GOCC from any remainder resulting from the review and reduction of their reserve funds to reasonable levels taking into account the disbursement from prior years, the DOF issued DC No. 003-2024 dated Feb. 27, 2024 mandating all GOCCs (including PhilHealth) to return its excess reserves.

As such, the following events and actions of the Management were made in order to comply with the aforesaid directive:

- PhilHealth sought for legal opinion from the Office of the Government Corporate Counsel (OGCC) and the Governance Commission for GOCCs (GCG) on the applicability of the Special Provision 1(d), XLIII of the GAA to PhilHealth in the light of the provisions on Reserve Fund under RA No. 7875 (NHIP) and the RA No. 11223 or the UHC Act. In addition, PhilHealth also sought guidance from the Commission of Audit (COA) on the accounting treatment of the supposed remittance.
- On April 16, 2024, PhilHealth received OGCC opinion No. 055, s. 2024 dated April 11, 2024 expressing that the remittance to

the BTr of the Fund Balance would not violate the UHC Act as the amount remitted is understood not to constitute a portion of the Reserve Fund. They concluded that it would be proper and prudent for PhilHealth to abide by the DOF Circular which implements Section XLIII (1)(d) of the GAA.

- On April 25, 2024, PhilHealth received DOF letter dated April 24, 2024 directing PhilHealth to remit the P89.9 billion pesos the Bureau of Treasury (BTR) within 15 calendar days from receipt of their notice. The collection of fund was approved during the Cabinet meeting on April 3, 2024. The table below shows how the P89.9 billion was computed by the DOF, to wit:

Government Subsidy vs Utilization Amounts in Billion (rounded-off)						
	2023	2022	2021	2020	2019	2018
Premium for Indirect Contribution	78.8	80.1	80.2	63.4	69.4	59.7
Member Benefit for Indirect Contributors	4.0	56.1	53.1	64.1	70.5	58
Net Flow	38.8	24.0	27.1	-0.7	-1.1	1.7

89.9 billion

- On May 2, 2024 the Governance Commission for Government Owned or Controlled Corporation (GCG) issued its opinion, stating that they defer to the expertise of the DOF, being the agency mandated by law to implement Special Provision 1(d), XLIII of the 2024 GAA.
 - Based on the DOF computation, it was determined that the 2-year requirement constituting the Reserve Fund is equivalent to P280.575 billion. After considering the new computation, a reduction of 183.712 billion of the reserve fund (464.287-280.575) and shall be transferred to the Surplus Account.
 - Hence, on May 8, 2024, after ensuring that there is fund balance to comply with the DOF directive, the Management sought for the Board approval on the transfer of government subsidy amounting to 89.9 billion for premium contribution (CY 2021-2023) that are more than the benefit expenses of indirect contributors. This was then approved by the Board upon confirmation from Management that the said unutilized subsidies does not include PhilHealth premium contribution from direct contributors.
 - The releases shall be synchronized with investment maturities to avoid pretermination fees. Hence, on May 10, 2024, the Execom approved the schedule of remittance of the 89.9 Billion to the Bureau of Treasury as follows:
 - 1st tranche: May 10, 2024 – 20 billion
 - 2nd tranche: August 21, 2024 – 10 billion
 - 3rd tranche: October 16, 2024 – 30 billion
 - 4th tranche: March 31, 2025 – 29.884 billion
- On October 29, 2024, the Supreme Court in Pimentel vs. House of Representative et. al petition issued a Temporary Restraining Order (TRO) enjoining the respondent including PhilHealth from transferring the remaining P29.884 billion of PhilHealth Fund

Balance scheduled transfer to the Unprogrammed Appropriation effective immediately.

On February 4, 2025, and March 4, 2025, Oral Argument was conducted on the petition filed on the transfer of P89.9 billion to the Bureau of Treasury. The next schedule will be on April 2, 3, and 4, 2025 if necessary. Decision is yet to be issued on the petition.

33.3. The GAA FY 2025 was signed into law on December 30, 2024 and no appropriation was made for the subsidy for PhilHealth Indirect Contributors. Despite of the absence of appropriation, the management believes that we have sufficient reserve and surplus to pay for the benefit claims as they fall due. Further, as provided for in the RA No. 11223, Section 37, the DOH, in coordination with PhilHealth may request congress to appropriate supplemental funding to ensure the success of the program.

33.4. As provided for in chapter IX Section 37 of the RA No.11223, fifty percent (50%) of the National Government share from the income of the Philippine Amusement Gaming Corporation (PAGCOR) as provided for in Presidential Decree No. 1869, as amended: Provided, That the funds raised for this purpose shall be transferred to PhilHealth at the end of each quarter subject to the usual budgeting, accounting and auditing rules and regulations: Provided, further, That the funds shall be used by PhilHealth to improve its benefit packages and forty percent (40%) of the Charity Fund, net of Documentary Stamp Tax Payments, and mandatory contributions of the Philippine Charity Sweepstakes Office (PCSO) as provided for in RA No. 1169, as amended: Provided, That the funds raised for this purpose shall be transferred to PhilHealth at the end of each quarter subject to the usual budgeting, accounting, and auditing rules and regulations: Provided, further, That the funds shall be used by PhilHealth to improve its benefit packages.

As of the reporting date, SARO-BMB-C-24-0019112 dated December 17, 2024 amounting P21.170 billion and SARO-BMB-C-23-0026482 dated December 5, 2023 amounting to P21.170 billion for the improvement of benefit packages were released/issued. However, Notice of Cash Allocation (NCA) has yet to be issued.

33.5. Identified Strategies and Cybersecurity Action Plans to mitigate risk arising from the “Medusa” ransomware attack.

As the Corporation recognizes the critical importance of robust security measures to enhance its resilience against evolving threats (e.g. Medusa ransomware), proactive measures shall be undertaken to enhance our security posture. As such, we need to manage and reduce the risk to our cyber and physical infrastructure through the implementation of the following identified strategies and cybersecurity action plans:

1. Cyber Security Projects

In exigency of service after the discovery of the incident, a Corporate Personnel Order was issued, creating a Project Management Team for Cyber Security (PMT - CS), headed by and composed of existing personnel from InfoSec.

To bolster the Corporation's cyber security posture, the InfoSec prepared and processed the engagement and/or procurement of the following security solutions and projects:

1. Database Security Suite. This solution ensures the security of the production database through transparent data encryption. As backgrounder, the said project "safeguard[s] the Corporation's data from unauthorized access, use, disclosure, disruption, modification, perusal, inspection, recording or destruction, even from privileged users, that is 100% compatible with existing systems and database. This means that the Corporation will be assured that problems due to error/s in the

existing database encryption software will be corrected due to updated software including patches, upgrades and remote services." The said project was initially processed last 2023, and finally implemented this April 2024.

2. Vulnerability Assessment and Penetration Testing (VAPT). The VAPT is one of the most reliable methods for performing Risk Assessment" in accordance with globally accepted Security Standards and best practices, which requires the expertise of a third party to do an independent assessment of PhilHealth's security posture by undertaking technical reviews such as Penetration Testing (PT) and Vulnerability Assessment (VA). In a more plain language, VAPT is best described as "a proactive "hacking" activity in which [the organization] hack[s] [their] [own] infrastructure before hackers come looking for loopholes." Hence, this project has the end goal of gaining better understanding of potential threats and vulnerabilities, identify existing inherent and potential risks and vulnerabilities, evaluate the existing security measures and validate the effectiveness and efficiency of existing defenses. The documents pertaining to this project were already endorsed to the Secretariat for the Bids and Awards Committee (SBAC) for procurement

3. Managed Security Operations Center (MSOC). A managed service, which aims to have a better visibility on PhilHealth's cyber security posture by providing real-time monitoring. The same was already presented before the IT Governance Committee. Further, in a memorandum dated March 15, 2024 to the ICT Planning, Policy and Standards Division—Information Technology Management Division (IPPSD-ITMD), this Office already requested the inclusion of the said project to the Information System Strategic Plan (ISSP) for CY 2024-2026 of the Corporation. In their response, the IPPSD-ITMD required the submission of a copy of the Secretary Certificate, the same was already coordinated with the Corporate Secretary.

These projects will redound to the cyber security posture of the Corporation.

The Cybersecurity Action Plan will be further developed and calibrated to the status and needs of PhilHealth in view of the engagement of the Cyber Security consultant.

2. Compliance to the Statutory Requirement i.e. Data Privacy Act (DPA) and related issuance

For being the designated organic office to provide support to the Data Protection Officer, InfoSec immediately complied with the statutory requirement prescribed by the DPA and its related issuance, along with the directives of the National Privacy Commission. This includes submission of reports, notification to the Commission, and other similar activities.

3. Capacitating PhilHealth Personnel and Officials through the Security Education Training Awareness (SETA) Program

It is indisputable that the human component is the weakest link in the information security and cybersecurity chain. To remediate the same, it is incumbent for the Corporation to bolster its human firewall by capacitating its employees. In view thereof, InfoSec initiated a set of activities covered by the Corporate Personnel Order (CPO), authorizing its personnel to conduct activities that promote information security and data privacy and protection awareness in several regional offices and LHIOs.

4. PhilHealth cyber and physical infrastructure is proactively protected and secured through the following:
 - Maintaining an updated Inventory of Enterprise and Software Assets
 - Strengthen Data Protection through developing processes and technical controls to identify, classify, securely handle, retain, and dispose data. Encrypt data on end-user devices containing sensitive data, on removable media, in transit, and at rest.
 - Secure configuration of assets and software
 - Improve Account Management by conducting periodic audits
 - Improve Access Control Management
 - Strengthen Audit Log Management through log collection and analysis to detect malicious activity quickly.
 - Strengthen Malware Defenses
 - Improve Network and Infrastructure Management
 - Improve Network Monitoring and Defense
 - Improve Service Provider Management
 - Improve Application Software Security
 - Adoption of National Security-and-Privacy-by-Design Framework
 - Improve Physical and Environmental Security
 - Risk Management
 - Adoption and implementation of the Six-stage Cybersecurity Incident Response Model and
 - Business Continuity and Disaster Recovery program -this will be further developed and calibrated to the status and needs of PhilHealth in view of the engagement of the Cybersecurity consultant to be implemented immediately.

33.6. PhilHealth as Commercial Public Sector Entity (CPSEs)

PhilHealth is now classified as a CPSE per COA Resolution No. 2020-013 dated January 31, 2020, re: Renaming Government Business Entities (GBEs) and Non-Government Business Entities (Non-GBEs) into Commercial Public Sector Entities (CPSEs) and Non- Commercial Public Sector Entities (Non-CPSEs) pursuant to the 2018 Edition of the Handbook of International Public Sector Accounting Pronouncements (HIPSAP) Published by the International Federation of Accountants (IFAC) and the International Public Sector Accounting Standard Board (IPSASB) and its adoption of the PFRSs as its financial reporting framework.

33.7. POS Program

The DBM has issued the following Special Allotment Release Order (SAROs) for the payment of cost of availment for CY2017 benefit claims of financially incapable families under UHC through POS Program, chargeable against PhilHealth's authorized appropriation under RA No. 10924, FY 2017. The release of the corresponding cash allocation is subject to PhilHealth's submission of the names of CY2017 POS patients together with the actual amount of claims with the required premium contributions to DBM.

SARO No.	Amount of SARO	DBM Releases from CYs 2017 to 2024
SARO-BMB-C-17-0023166	91,333,530	91,333,530
SARO-BMB-C-17-0025119	103,350,096	103,350,096
SARO-BMB-C-17-0025794	2,805,316,374	1,196,668,091
Total	3,000,000,000	1,391,351,717

33.8. PhilHealth Supplemental Benefits

On December 28, 2018, the DBM issued SARO No. SARO-BMB-C-18-0035076 amounting to eight billion five hundred ninety-seven million four hundred seventy-five thousand seven

hundred seventy-six pesos (P8,597,475,776) which include the locally funded project "PhilHealth Supplemental Benefits" for all government employees of the Executive Branch amounting to P3,500,000,000. No NCA received yet for the P3,500,000 as of December 31, 2024.

33.9. Arrears of the National Government as an Employer

The information below, though not recorded in PhilHealth's books of accounts, are deemed necessary to be disclosed.

PhilHealth had adjusted premium contribution of the Employed Sector in CY 2013 through PhilHealth Circular No. 057, s. of 2012 which prescribes P875.00 per month as the maximum contribution shared equally by the Employer and the Employee at P437.50 each. However, DBM has only allocated P312.50 or a 40 per cent discrepancy. The Corporation had formally billed the DBM of the estimated National Government Employer Premium Differential and requested allocation of the unappropriated balances for the following periods:

Arrears	Amount
CYs 2001-2012	9,664,042,012
CY 2013	330,691,801
CY 2014	330,691,801
CY 2015	330,691,801
CY 2016	330,691,801
Total	10,986,809,216

33.10. Assigned lot to PRO II

A parcel of lot with a total area of 1,831 sq. m. located within the Government Center, Barangay Maimpis, City of San Fernando, Pampanga was assigned to PRO III per Deed of Assignment from Regional Government Center of San Fernando, Pampanga through a MOA executed by and between the RDC III and PhilHealth. A warehouse was constructed in this lot with a carrying amount of P2,520,000 in the financial statements.

33.11. Non-remittance of GSIS Premium for Disallowed Salary Adjustments

The GSIS premium amounting to P20,604,585.00 was part and parcel of the salary adjustments given to the Corporation's employees, but subsequently disallowed by COA. The said amount is still outstanding and not yet remitted to the GSIS per Opinion No. 56, Series of 2018 from the OGCC dated March 26, 2018 stating that PhilHealth should keep the amount originally intended to be remitted to the GSIS as premiums corresponding to the adjusted salaries of its employees without prejudice to its remittance in the event the ND is lifted.

33.12. Contingent Liability

Under contingent liability is the employer's share for PhilHealth Provident Fund (PPF) amounting to 1,208,222,508.94 for the years 2019, 2020 and 2021. The establishment of the PPF needs the approval of the President of the Philippines as observed through COA AOM hence, the timing and control of the approval is now beyond the control of the corporation. As of even date, the appeal is now with the Office of the President.

With other Government-Owned and Controlled Corporations and Government Financial Institutions that were able to secure approval for their Provident Fund, we are optimistic that the approval will be granted in due time.

33.13. Contingent Asset

Contingent assets are possible assets whose existence will be confirmed by the occurrence or non-occurrence of uncertain future events that are not wholly within the control of the entity.

Contingent assets are not recognized, but they are disclosed when it is more likely than not that an inflow of benefits will occur.

Under contingent assets are legal cases filed by PhilHealth against erring HCPs and employers which are more likely than not that an inflow of benefit will occur amounting to P127,969,453.30. It also includes notice of disallowances issued by the Commission on Audit which are on appeal with the Supreme Court amounting to P3,074,741,785.92.

33.14. Number and amount of Outstanding Legal Cases Filed by PhilHealth

PHILHEALTH OFFICE	Complaint Against Employers	HCPs	Total	Estimated Outcome
HEAD OFFICE		53,052,377	53,052,377	Probable
NCR	354,140		354,140	Probable
CAR			-	Probable
PRO I			-	Probable
PRO II	10,248,428		10,248,428	Probable
PRO III	35,702,803		35,702,803	Probable
PRO IV-A		6,526,694	6,526,694	Probable
PRO IV-B	1,545,433	365,274	1,910,707	Probable
PRO V			-	Probable
PRO VI	299,875	16,348,443	16,648,318	Probable
PRO VII		3,525,986	3,525,986	Probable
PRO VIII			-	Probable
PRO IX			-	Probable
PRO X			-	Probable
PRO XI			-	Probable
PRO XII			-	Probable
CARAGA			-	Probable
BARMM			-	Probable
Grand total	48,150,680	79,818,774	127,969,453	

33.15. COA Disallowances with Notices of Finality of Decision and COA Orders of Execution

Audit Action	Beginning balance 1/1/2024	Issued this period	Settlement this period	Ending balance 12/31/2024
Notice of Finality of Decision	683,930,250			683,930,250
COA Order of Execution	214,406,928		19,734,349	194,672,578
Total	898,337,177	-	19,734,349	878,602,828
Notice of Finality of Decision				
Head Office	475,174,318			475,174,318
NCR & Rizal	128,108,281			128,108,281
Region IV-A	42,986,544			42,986,544
Region IV-B	6,030,731			6,030,731
CARAGA	31,630,376			31,630,376
Total	683,930,250	-	-	683,930,250

COA Order of Execution

Head Office	154,117,115		154,117,115
Region X	49,618	39,000	10,618
Region XII	60,240,194	19,695,349	40,544,845
Total	214,406,928	-	19,734,349
			194,672,578

34. SUPPLEMENTARY TAX INFORMATION UNDER REVENUE REGULATION (RR) NO. 15-2010

The taxes, duties and licenses fees paid and accrued during the taxable year required under RR No. 15-2010 are as follows:

	2024	2023
Real estate tax, license and permit taxes	414,971	547,134
Taxes on compensation and benefits	476,642,285	393,008,075
Government Money Payments	107,196,316	84,079,046
Expanded withholding taxes	1,064,123,197	1,324,495,769
Total	1,648,376,769	1,802,130,024

Other Taxes, Duties and Licenses amounted to P414,971 and P547,134 for CYs 2024 and 2023, respectively, are taxes paid for real property taxes and other fees paid to regulatory entities.



Board of Directors



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Atty. Lora L. Mangasar
Corporate Secretary
Office of the Corporate Secretary



Atty. Mauro Anthony B. Cabading, III
Corporate Legal Counsel
Office of the Corporate Legal Counsel



DR. CARMENCITA D. PADILLA

Member, Indirect Contributors Sector

Dr. Carmencita D. Padilla is an Emeritus Professor of College of Medicine, University of the Philippines (UP) Manila. She received her MD from UP College of Medicine. After her residency training in pediatrics at the Philippine General Hospital (PGH), she completed a fellowship in clinical genetics at the Royal Alexandra Hospital for Children in Sydney, Australia. She also pursued a Master of Arts in Health Policy Studies from the College of Public Health, UP Manila. She is a pioneer in genetics in the Philippines and the Asia Pacific Region having established genetic services at the PGH and help found the Asia Pacific Society of Human Genetics. She initiated and developed a sustainable newborn screening system in the country and has been responsible for the lobby efforts for the passage of the Newborn Screening Act of 2004 (Republic Act No 9288) and the Rare Diseases Act of 2016 (Republic Act No 10747). She is an international consultant in genetics, newborn screening and rare diseases.

Her service to the country has included holding many key positions including Chancellor, University of the Philippines Manila (2014-2023); Founding Director, Institute of Human Genetics (2000-2010) and Newborn Screening Reference Center of the National Institutes of Health (2006-2014), UP Manila; Executive Director, Philippine Genome Center (2011-2016); President of the Asia Pacific Society for Human Genetics (2008-2010); and Vice President/Treasurer of International Society of Neonatal Screening (2013-2016).

Dr. Padilla has received numerous awards and recognitions such as being amongst the 100 Most Influential Filipina Women in the World by the US-based Women's Network, Eric C. Nubia Excellence Award by the Professional Regulation Commission (PRC), Outstanding Professional of the Year Award in the field of Medicine by the PRC, Outstanding Pediatrician by the Philippine Pediatric Society, UPAA Distinguished Alumna Award for Health Education, The Outstanding Filipino Physician (TOFP) by the JCI Senate of the Philippines and the Department of Health (DOH), Lingkod Bayan Award by the Civil Service Commission, The Outstanding Young Men (TOYM) Awardee for Medicine (Medical Genetics) by the Philippine Jaycees, and elected as Academician of the National Academy of Science and Technology. In 2023, President Ferdinand R. Marcos, Jr. honored her with the title of National Scientist.



ATTY. GIOAN FERNAND A. LEGAZPI

Member, Direct Contributors Sector

Atty. Gioan Fernand A. Legazpi is a private practitioner with more than two decades of comprehensive experience providing legal services on health-related concerns to not just health care institutions but also to indigents needing representation on health and healthcare-related disputes.

Atty. Legaspi, with his significant legal background coupled with his academic history as a Juris Doctor from the University of Santo Tomas and a graduate of AB Political Science from the Ateneo De Manila University, firmly set his objective to help craft policies that would aid the government in the combat and elimination of fraud and abuse in the health care industry to ensure the efficient implementation of the Universal Health Care (UHC) Act.

Aside from being a member of the PhilHealth Board of Directors, Atty. Legaspi is a Senior Partner at Legaspi Barcelo & Salamera Law Offices (Lebarsa Law) with extensive litigation experience in Health Care Law, Government Procurement, Commercial and Taxation Law, Customs, Labor, Immigration, Criminal, Anti-graft and Corrupt Practices, Environmental Law, among others. He was a Senior Partner at Benitez Legaspi Barcelo Rafael & Salamera Law Offices (Belarus Law) and was once a lawyer at the Benitez Parlade Africa Herrera Parlade and Panga Law Offices (Pablaw). This two-time Aegean Awardee for Excellence used to work in the 3rd Division of the Sandiganbayan.



BGEN. MARLENE R. PADUA, AFP (RET.)

Board Member - Health Care Providers Sector

Education

Health Service Management, PhD in Nursing-University of the Philippine Baguio, University of the Philippines Manila • Master of Arts in Nursing-University of Asia and the Pacific • Pre-Nursing/ Graduate Nurse/Bachelor of Science in Nursing-St. Louis University College of Arts and Science General Hospital School of Nursing- Philippine Women's University • Secondary Education-St. Louis University High School Baguio City • Basic Education-St. Louis University Elementary School Baguio City

Before her stint as a member of the PhilHealth Board as a representative for the Health Care Provider Sector, BGen. Padua served as Chair of the Advisory Council of the PNP Health Service PATROL Plan of the Philippine National Police Health Service in 2017. BGen. Padua also served as Dean of the College of Nursing at Arellano University in Pasig in 2012. She was a faculty member and research coordinator at Florentino Cayco Memorial School of Graduate Studies in 2009. BGen.

Padua also served as Chief Nurse in the Nurse Corps. of the Armed Forces of the Philippines from 2004-2007; Chief Nurse of the Philippine Army from 2003-2004; Deputy Chief Nurse of the Philippine Navy from 2001-2003; Assistant Chief Nurse for Clinical Service Fort Bonifacio General Hospital in 1999-2000. BGen. Padua also served as Command Nurse of the 4th Infantry Division/Camp Evangelista Station Hospital in Cagayan De Oro from 1998-1999. She also served as Staff Nurse from 1987 to 1998 at the Coney Island Hospital, Brooklyn, New York, USA; Nurse- In-Charge from 1984 to 1986 at the Bonifacio Naval Station Dispensary and Chief Registrar from 1983 to 1984 and Head Nurse from 1973 to 1983 in the AFP Medical Center. With her prolific contributions to Nursing and health care, she handles the role of Philhealth's Health Care Provider Sector with ease and confidence.



ROBERT FRANCIS F. MARONILLA

Member, Employers Group

Robert Francis F. Maronilla is the Head of Legal Services and Corporate Secretary of the Employers Confederation of the Philippines (ECOP) with more than a decade of experience in the legal profession that is dedicated to employers' right, labor management and industrial relations. His academic credentials and experiences as a post-graduate law degree from Lyceum University and undergraduate studies at the University of the Philippines Diliman supported and set his strength in safeguarding and balancing employer rights with labor interests.

Mr. Maronilla is an accomplished and skilled facilitator in high-level external engagements and collaboration with key national and international bodies. As the Head of Legal Services, he works closely with the International Labor Organization (ILO), the Department of Labor and Employment (DOLE) and its attached agencies like the Regional Tripartite and Wages Productivity Board (RTWPB) in the assessment and review of regulations as well as in setting policy directions related to labor, employment and related concerns.

Mr. Maronilla used to serve as a Commissioner for the Tripartite Voluntary Arbitration Advisory Council, advising on making voluntary arbitration the preferred method for resolving labor disputes and establishing accreditation guidelines for arbitrators. He has been affiliated with the Tripartite Executive Committee (TEC) of the National Tripartite Industrial and Peace Council (NTIPC) of the Department of Labor and Employment, Bureau of Labor Relations, and the Association for Overseas Technical Cooperation and Sustainable Partnerships (AOTS) Tokyo, Japan.



DR. THEA ARCELY R. GIMENEZ

Member, Expert Panel

Dr. Thea Arcely R. Gimenez is a physician with a career that was rooted in high-stakes international finance capitalizing on her academic background in Economics and Finance from the United Kingdom: Master in Finance and Economics from the Warwick Business School, University of Warwick and a Bachelor of Science (Hons) in Economics from Queen Mary & Westfield, University of London. She served as a foreign exchange, energy and proprietary trader for major institutions like Deutsche Bank and Asea Brown Boveri (ABB) in London and Frankfurt.

In an unusual and significant career shift, Dr. Gimenez pursued a Bachelor of Medicine, Bachelor of Surgery (BMBS) from the University of Southampton. After earning her medical degree, she gained medical experience as a foundation doctor within the Wessex Deanery National Health Service (NHS) working in multiple hospitals such as Basingstoke Hospital, Southampton General Hospital, Winchester Hospital and Princess Anne Hospital and completing clinical audits in Pediatrics and Surgery. This period demonstrated her commitment to mastering a completely new, rigorous

professional field. Dr. Gimenez has since leveraged her combined expertise to excel in managing several companies under diverse industries like retail, manufacturing, food and beverages, leasing, drug testing, and medical screening—a role she has been doing well since 2013. She has perfectly integrated her background: the operational discipline of management, the analytical rigor of finance, and the attention to detail required in medicine, making her a highly versatile corporate leader.



DR. MARIA GRACIELA M. GARAYBLAS-GONZAGA

Member, Expert Panel

Dr. Maria Graciela M. Garayblas-Gonzaga is an accomplished physician with advanced international training in pharmacology and toxicology, holding degrees from UST and Chulalongkorn University. Her foundation in medicine was cemented through residency at UST Hospital and specialized fellowships in Japan and Belgium. At the University of Santo Tomas (UST), she built a significant academic career, teaching across Pharmacology, Internal Medicine, and Clinical Epidemiology before rising through the ranks to serve as Dean, underscoring her commitment to medical education and research administration.

Her professional influence is broad, demonstrated by her leadership roles in various medical societies. Dr. Gonzaga served as President of the Philippine Society of Experimental and Clinical Pharmacology and the Association of Philippine Medical Colleges, and also chaired the UST Hospital's Institutional Review Board and Pharmacy and Therapeutics Committee. This history showcases her expertise in setting standards for both clinical practice and research ethics within the medical community.

Crucially, Dr. Gonzaga has been repeatedly appointed to significant national policy bodies by the highest office. Presidents Aquino, Duterte, and Marcos, Jr. have all tapped her expertise, appointing her to the Professional Regulation Commission's Board of Medicine and as an expert panel member of the PhilHealth Board of Directors. These ongoing roles confirm her sustained influence on the country's medical practice regulation and health insurance governance. She is also the incumbent chairperson of the National Committee on Ethics of the Philippine College of Physicians.



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Senior Manager Atty. Francis Jay E. Remigio, Physical Resources and Infrastructure Department |
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| **Acting Branch Manager Arlan M. Granali, PRO IIIB**
| **Regional Vice President BGEN. LLeuwelyn R. Binasoy (Ret.), PRO II**



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| Regional Vice President Atty. Jerry F. Ibay, PRO IVB |
Regional Vice President Alberto C. Manduriao, PRO V



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| **Regional Vice President Paolo Johann C. Perez**, PRO IX | **Regional Vice President**
Atty. Harvey L. Carcedo, PRO XI | **Regional Vice President Masiding M. Alonto, Jr.**, PRO BARMM



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| **Regional Vice President Delio A. Aseron, II, PRO X**



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PRO NCR

Regional Office
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Technocenter,
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PRO NCR North - Manila

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PRO NCR Central - Quezon City

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Local Health Insurance Office

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City
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Fairview, Quezon City

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Local Health Insurance Office

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PRO X - Cagayan De Oro

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PRO BARMM - Marawi

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PhilHealth Action Center

New Hotline: (02) 866-225-88

Available 24/7 including weekends and holidays
www.philhealth.gov.ph

Mobile Hotlines: Call and text (24/7)

Smart: 0998-857-2957; 0968-865-4670 Globe:
0917-127-5987; 0917-110-9812

To request for callback via preferred mobile hotline, text:
"PHICallback [space] Mobile Number to be called
[space] details of your concern"

Callback schedule is from 8 am - 8 pm, 12 hours by 7
days, including weekends and holidays. Ask agent for
details to make sure the callback is from PhilHealth.

 actioncenter@philhealth.gov.ph

 PhilHealthofficial

 teamphilhealth

